FIRST AID KIT CHECKLIST

Israel ישראל – Home, Vehicle & Family

MDA מד"א	Police משטרה	Fire כיבוי	Hatzalah הצלה	Poison Center
101	100	102	1221	04-7771900

1. ESSENTIAL – Every Home

Wound Care & Bandaging

- [ ] Sterile gauze pads 10x10 cm **(x10)**
- [ ] Rolled gauze bandage 5 cm **(x3)**
- [ ] Rolled gauze bandage 10 cm **(x2)**
- [ ] Israeli Emergency Bandage 4″ **(x1)**
- [ ] Adhesive bandages / plasters, assorted **(x20)**
- [ ] Butterfly closure strips **(1 pack)**
- [ ] Triangular bandage + safety pins **(x2)**
- [ ] Elastic bandage 6 cm **(x1)**
- [ ] Medical tape / micropore **(1 roll)**
- [ ] Non-adherent wound dressing **(x5)**

Bleeding Control

- [ ] Hemostatic gauze pads (SIDA-CEL / Celox) **(x2)**
- [ ] Hemostatic agent packets **(x3)**

Burns

- [ ] Burn dressings (Burnshield 4″x4″) **(x3)**
- [ ] Burn gel sachets **(x3)**

Tools & Equipment

- [ ] Trauma shears 7.5″ **(x1)**
- [ ] Tweezers **(x1)**
- [ ] Digital thermometer **(x1)**
- [ ] Safety pins **(x6)**
- [ ] Permanent marker **(x1)**
- [ ] Pen torch / flashlight **(x1)**

Infection Prevention

- [ ] Nitrile gloves, pairs **(x5)**
- [ ] Alcohol prep pads **(x10)**
- [ ] Antiseptic – Polydine פוליד‌ין **(1 bottle)**
- [ ] Hand sanitiser **(1 small bottle)**
- [ ] Sterile eye wash ampoules **(x2)**

CPR & Rescue

- [ ] CPR pocket mask with one-way valve **(x1)**
- [ ] Emergency rescue blanket **(x1)**

Other

- [ ] Waterproof / dustproof container or bag
- [ ] Emergency numbers card **(x1)**

2. FOR FAMILIES WITH CHILDREN – Pediatric Additions

Equipment

- [ ] Pediatric CPR mask (infant size) **(x1)**
- [ ] Rectal thermometer (for infants) **(x1)**

Children's Medications

- [ ] Acamol Tsinuk / Acamol Li (paracetamol liquid)
- [ ] Advil / Nurofen for Children (ibuprofen liquid)

- [ ] Infant nasal aspirator **(x1)**
- [ ] Paediatric dosing syringe / cup **(x1)**
- [ ] Child-sized plasters **(x20)**
- [ ] Oral rehydration salts (ORS) sachets **(x5–10)**
- [ ] Saline nasal drops **(x1)**
- [ ] Telfast / Fenistil drops (antihistamine)
- [ ] Saline nasal drops (Physiomer)

EpiPen (prescription required)

- [ ] EpiPen 0.3 mg – adults / children >30 kg
- [ ] EpiPen Junior 0.15 mg – children 15–30 kg

Covered by kupot cholim for diagnosed allergies. Store 15–25 C. Check expiry every 12–18 months.

3. CLIMATE & ENVIRONMENT – Israel-Specific

Heat & Sun

- [ ] Electrolyte sachets (e.g. Slow Sodium, ORS) **(x5)**
- [ ] Sunscreen SPF 50+ **(x1)**
- [ ] Aloe vera gel **(x1)**
- [ ] Cooling towels
- [ ] Extra bottled water (2L / person / day)

Stings & Bites

- [ ] Sting relief pads / antihistamine cream
- [ ] Instant cold packs **(x2)**
- [ ] Antihistamine tablets (Telfast / cetirizine)
- [ ] Elastic bandage (for snakebite immobilisation)
- [ ] Tweezers (for tick removal)

Scorpion stings (deathstalker) and snakebites (Palestine viper): Call MDA 101 immediately. Transport to hospital for antivenom. Home first aid is supportive only. Do NOT apply tourniquet for snakebite.

4. SECURITY / TRAUMA KIT – Civilian Additions (Optional)

- [ ] CAT or SAM-XT tourniquet **(x1–2)**
- [ ] Israeli Emergency Bandage 6″ **(x2)**
- [ ] Chest seal, vented (e.g. HyFin / SAM) **(1 pair)**
- [ ] Hemostatic combat gauze, z-fold **(x2)**
- [ ] Compressed gauze for packing **(x2)**
- [ ] NPA airway **(x1)** – only if trained
- [ ] TCCC card (English or Hebrew) **(x1)**
- [ ] Trauma shears **(x1)**
- [ ] Permanent marker **(x1)**

Source: MDA / United Hatzalah civilian “Stop the Bleed” curriculum. Practice with the specific tourniquet model you own.

VEHICLE FIRST AID KIT

Ministry of Transportation Standard + Recommended Additions

MDA מד"א	Police משטרה	Fire כיבוי	Hatzalah הצלה	Poison Center
101	100	102	1221	04-7771900

Legal note: A vehicle first aid kit is **recommended but not legally mandatory** for private cars in Israel. It **is mandatory** for security vehicles, trucks, and public transport.

Ministry of Transportation Standard (Minimum)

- [ ] Israeli Emergency Bandage (sterile) **(x1)**
- [ ] Triangular bandage **(x1)**
- [ ] Adhesive bandages / plasters **(x10)**
- [ ] Sterile gauze strip **(x1)**
- [ ] Binding bandage 6 cm **(x1)**
- [ ] Bandage scissors **(x1)**

Recommended Additions

- [ ] Nitrile gloves **(2 pairs)**
- [ ] Burn dressing **(x1)**
- [ ] Emergency blanket **(x1)**
- [ ] Emergency light stick, green 6″ **(x1)**
- [ ] CPR mask **(x1)**
- [ ] Reflective safety vest **(legally required)**
- [ ] Bottled water 0.5L **(for wound irrigation)**

Vehicle Kit Storage & Maintenance

- Store in glove compartment or door pocket – **not in the boot/trunk** (must be quickly accessible)
- Sealed bag resistant to heat, moisture, humidity
- Check contents every **3–4 months** (heat degrades items faster)
- General kit validity: **5 years**, but check individual expiry dates
- Israeli summers (40 C+) accelerate degradation of medications and adhesives

OTC MEDICATIONS – Israeli Brand Names

Medication	Israeli Brand	Use	Notes
Paracetamol	**Acamol** (Teva), Dexamol	Pain, fever	500 mg tablets. Most common analgesic in Israel
Dipyrone	**Optalgin** (Teva)	Pain, fever	Widely used in Israel. Tablets, drops, suppositories
Ibuprofen	**Advil**, **Nurofen**	Pain, inflammation	200–400 mg tablets
Diclofenac gel	**Voltaren Gel**	Topical pain	For sprains, muscle pain
Cetirizine / Loratadine	**Telfast**, **Histec**	Allergies	Non-drowsy options preferred
Diphenhydramine	**Benadryl**	Allergic reactions	Causes drowsiness; less common in Israel
Activated charcoal	Available at pharmacies	Poisoning	**Only if directed by Poison Center**

Medication	Israeli Brand	Use	Notes
Oral rehydration salts	Various brands	Dehydration	Dissolve in water
Antacid	**Rennie**, **Tums**	Heartburn	
Anti-diarrhoeal	**Imodium** (loperamide)	Diarrhoea	
Povidone-iodine	**Polydine** פוליד‌ין	Antiseptic	Widely used in Israel

WHERE TO BUY IN ISRAEL

Pharmacies & Chains

Super-Pharm – super-pharm.co.il
Largest chain. First aid kits, medications. Online ordering.

Community pharmacies בית מרקחת
Found in every neighbourhood. Pharmacists can advise on kit contents.

MDA Shop – via IFMDA (ifmda.org.il)
Kits available through training courses.

United Hatzalah Shop – shop.1221.org.il
Home/car first aid kit (60 ILS).

Specialist & Online

Israeli First Aid – israelifirstaid.com
Israeli-made kits with hemostatic products. Basic $40, “tactical” $91.

Ariel Medic – a101.co.il
Comprehensive emergency equipment.

S-Medic – s-medic.co.il
Vehicle kits from 29 ILS.

Kasda – kasda.co.il
Advanced IFAK kits for civilian and security use.

Mimram Medical – mimrame.co.il
Ministry of Transportation standard vehicle kits.

EXPIRY TRACKING

Check twice a year:

- ☐ **Rosh Hashana** ראש השנה (September/October)
- ☐ **Pesach** פסח (March/April)

Shortest shelf lives:

EpiPen: **12–18 months**
Medications: **2–3 years**
Sterile dressings: **3–5 years**
Adhesive bandages: **3–5 years**
Burn gel: **3–4 years**
Hemostatic gauze: **3–5 years**
Antiseptic wipes: **2–3 years**
Nitrile gloves: **3–5 years**

Key reminders:

- Tape an inventory list inside the kit lid or store digitally
- **Replace used items immediately** – do not wait for 6-month check
- Israeli heat (35–45 C) shortens shelf life of adhesives, gels, and medications
- **Vehicle kits:** check every 3–4 months due to heat exposure
- Seal broken = item no longer sterile – replace

– CUT ALONG DASHED LINE – PLACE IN KIT –

EMERGENCY NUMBERS – ISRAEL מספרי חירום

MDA (Ambulance): 101
Police: 100
Fire: 102

United Hatzalah: 1221
Home Front Command: 104
Electric Company: 103

Poison Center: 04-7771900
Mental Health (ERAN): 1201
Domestic Violence: 1-800-220-000

From mobile (international): +972 + number

Sources: Magen David Adom, United Hatzalah, Israeli Ministry of Transportation, Israeli First Aid (israelifirstaid.com)
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Personal reference only – not medical advice. Always call 101 in an emergency.

AED Operation (Automated External Defibrillator) -- Adult

Israel – Adult (age 8+)

CALL 101 (MDA) – CALL IMMEDIATELY IF IN DOUBT

MDA (101 :(מד"א | **Police (100 :(משטרה** | **Fire (102 :(כיבוי** | **Hatzalah (1221 :(הצלה**

WHEN TO APPLY: An adult (age 8 and above, or showing signs of puberty) is found unresponsive, not breathing normally (absent breathing or only gasping/agonal breathing), and suspected to be in cardiac arrest.

Equipment needed:

– AED (Automated External Defibrillator) -- available from MDA smart stands, public locations, or via Eifo-Defi app locator
– Razor (for shaving excessive chest hair -- often included in AED kits)
– Towel or cloth (for drying the chest)
– Scissors (for cutting away clothing -- often included in AED kits)
– Phone (to call 101 and use speakerphone for dispatcher-guided CPR and AED use)

Step 1: Confirm cardiac arrest: check responsiveness by calling out and shaking the person's shoulders. Check breathing for no more than 10 seconds by looking for chest rise, listening for breath sounds, and feeling for air on your cheek.

Gasping (agonal breathing) is NOT normal breathing -- treat it as cardiac arrest. If trained to check a pulse, do so simultaneously with the breathing check, but do not spend more than 10 seconds total.

Is the person unresponsive and not breathing normally?

YES -> Proceed to Step 2 (call 101 and begin emergency response).

NO -> Do not apply AED. If the person is unconscious but breathing normally, place in the recovery position and call 101. If conscious and breathing, provide appropriate first aid.

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Step 2: Call 101 (MDA emergency dispatch) immediately and put the phone on speakerphone.

MDA dispatchers provide real-time CPR and AED coaching over the phone. They can also identify the nearest smart stand AED and remotely unlock it. If another person is present, send them to fetch the nearest AED while you begin CPR.

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Step 3: Begin CPR immediately while waiting for the AED: place the person on a firm, flat surface. Deliver chest compressions at the centre of the chest (lower half of the sternum), at a rate of 100--120 per minute, to a depth of 5--6 cm (2--2.4 inches). Allow full chest recoil between compressions.

If trained and willing: deliver 2 rescue breaths after every 30 compressions (30:2 ratio). If untrained or unwilling: perform compression-only CPR. Do not delay compressions to wait for the AED.

Is another person available to retrieve the AED?

YES -> Send them to retrieve the nearest AED while you continue CPR without interruption.

NO -> If the AED is immediately nearby (within a few seconds' reach), retrieve it quickly and return. Otherwise, continue CPR until help arrives with an AED.

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Step 4: Locate and retrieve the nearest AED.

Use the Eifo-Defi app to find the closest device. Call 101 -- MDA dispatch can locate the nearest smart stand and remotely unlock it (the case opens and a siren and flashing lights activate). Check nearby buildings, shopping centres, train stations, sports facilities, and public phone booth AED stations (bright yellow boxes). By Israeli law, any public space with 500+ daily visitors must have an AED.

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Step 5: Open the AED cover. The device will power on automatically (some models require pressing a power button). The AED will begin providing voice instructions in Hebrew (and possibly other languages: English, Arabic, Russian, Amharic depending on the model).

Follow the AED's voice and visual prompts throughout the process. The device will guide you step by step.

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Step 6: Prepare the patient's chest: expose the bare chest by removing or cutting away clothing, including bra. Ensure the chest is dry -- wipe off sweat, water, or other moisture.

Address special situations before applying pads.

Are there special chest conditions (excessive hair, medication patches, implanted pacemaker/defibrillator, or metal surface)?

YES -> Excessive chest hair: shave the pad areas using the razor often included in AED kits. Medication patches: remove any transdermal patches from the chest area and wipe the skin. Implanted pacemaker/defibrillator (visible lump under the skin): place pads at least 2.5 cm (1 inch) away from the device. Metal surface: move the person off the metal surface if possible; otherwise ensure nobody is in contact with the metal during shock delivery.

NO -> Proceed directly to applying the electrode pads.

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Step 7: Apply the electrode pads to the bare, dry chest following the diagram printed on each pad. Use anterolateral placement for adults: Pad 1 (upper right) below the right collarbone to the right of the sternum; Pad 2 (lower left) on the left side of the chest below the armpit, in line with the nipple.

Connect the pads to the AED if they are not pre-connected (plug the cable into the AED unit). Some AEDs have pre-connected pads.

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Step 8: Allow the AED to analyse the heart rhythm. Stop CPR and ensure nobody is touching the patient during analysis.

WARNING: Do not touch the patient during rhythm analysis. Any contact may interfere with the analysis and produce inaccurate results.

Does the AED advise a shock (shockable rhythm detected)?

YES -> Proceed to Step 9 (deliver shock).

NO -> The rhythm is not shockable. Resume CPR immediately (proceed to Step 10). The AED will not deliver a shock and cannot harm a person who does not need one.

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Step 9: Deliver the shock: ensure nobody is touching the patient. Call out clearly “Stand clear!” (Hebrew: התרחקו!). Press the shock button when instructed by the AED.

Some fully automatic AEDs deliver the shock automatically without requiring a button press. The AED will announce before delivering the shock.

WARNING: Visually confirm that nobody is in contact with the patient before pressing the shock button. Electrocution risk to bystanders.

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Step 10: Resume CPR immediately after the shock is delivered (or after a 'no shock advised' result). Continue CPR for 2 minutes (approximately 5 cycles of 30:2).

Do not wait for the AED to re-analyse before resuming compressions. The AED will automatically prompt for another rhythm analysis after approximately 2 minutes.

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Step 11: After 2 minutes of CPR, the AED will automatically re-analyse the heart rhythm. Stop CPR and ensure nobody is touching the patient during analysis. Follow the AED prompts (shock if advised, then resume CPR).

Continue the cycle of: CPR for 2 minutes -> AED analysis -> shock if advised -> CPR. Each cycle repeats until EMS arrives or the person shows signs of life.

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Step 12: Continue CPR and AED cycles until one of the following occurs: MDA paramedics arrive and take over; the person shows obvious signs of life (moves, breathes normally, opens eyes); or you are physically unable to continue.

WARNING: Do not disconnect the AED pads even if the person regains consciousness. Do not turn off the AED. Leave the pads attached at all times.

Does the person show signs of life (breathing, movement, coughing)?

YES -> Stop CPR. Place the person in the recovery position. Keep AED pads attached and the device on. Monitor continuously until MDA arrives.

NO -> Continue CPR and AED cycles without stopping.

DO NOT:

X Do not use the AED on a person who is conscious and breathing normally -- AEDs are only for cardiac arrest.
X Do not apply AED pads while the person is submerged in water or lying in a puddle -- move them to dry ground first and dry the chest before applying pads.
X Do not use an AED near flammable gases, fumes, or oxygen-enriched atmospheres -- risk of fire or explosion.
X Do not place AED pads directly over an implanted pacemaker or defibrillator -- place pads at least 2.5 cm (1 inch) away from the device.
X Do not place AED pads over transdermal medication patches -- remove patches and wipe the skin before placing pads.
X Do not touch the patient during AED rhythm analysis or shock delivery.
X Do not turn off the AED or disconnect the pads at any point, even if the person regains consciousness.
X Do not delay defibrillation to wait for EMS -- early defibrillation is critical. For every minute defibrillation is delayed, survival decreases by approximately 7--10%.
X Do not interrupt chest compressions for more than 10 seconds for any reason, including AED pad placement.
X Do not use the AED on a person who is breathing and has a pulse, even if unconscious -- place them in the recovery position and monitor.
X Do not allow AED pads to touch or overlap each other on the chest.

Personal reference only – not medical advice. Always call 101 in an emergency.

ANAPHYLAXIS / ALLERGIC REACTION

Israel — Adult

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

Equipment needed: EpiPen (adult 0.3 mg) + Phone to call MDA at 101

Step 1: Ensure scene safety and recognise signs of severe allergic reaction.

Look for: SKIN (rash, hives, swelling of face/lips/tongue/throat) -- RESPIRATORY (breathing difficulty, wheezing, stridor, throat tightness) -- CARDIOVASCULAR (weak/rapid pulse, low BP, pallor, confusion, loss of consciousness) -- GI (nausea, vomiting, abdominal pain). Involvement of breathing, circulation, or consciousness indicates anaphylaxis.

Are signs of anaphylaxis present?
(breathing difficulty, circulatory compromise, or altered consciousness)

YES -> Proceed to Step 2 – call 101 immediately and continue protocol.

NO -> Monitor for worsening. If only mild skin reaction without systemic involvement, observe closely. Call 101 if condition worsens.

Step 2: Call MDA at 101 immediately.

Keep the phone on speaker mode and follow the dispatcher's instructions. Call for help from bystanders. If possible, identify the allergen (food, medication, insect sting, latex).

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Step 3: Remove the offending agent if possible.

Remove a bee stinger by scraping sideways, stop any infusion or medication, move patient away from the allergen source. Do not delay other treatment to do this.

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Step 4: Manage the airway.

If facial or tongue swelling is present, assume airway compromise. Ensure the tongue is not obstructing the airway. Remove any food debris from the mouth. Loosen tight clothing around the neck and chest.

WARNING: Airway swelling can progress rapidly. Continuous monitoring is essential.

Step 5: Position the patient appropriately.

Positioning depends on the patient's condition and primary symptoms.

Is the patient conscious?

YES -> If breathing difficulty is primary: allow patient to sit upright. If hypotension/shock is primary: position supine (lying down) with legs elevated.

NO -> Proceed to Step 6 to assess breathing.

Step 6: If the patient is unconscious, assess breathing.

Is the unconscious patient breathing?

YES -> Place in the recovery position. Continue to monitor breathing and proceed to Step 7 (EpiPen).

NO -> Begin CPR immediately. Continue CPR until EMS arrives or the patient resumes breathing.

Step 7: Administer epinephrine auto-injector (EpiPen) if available.

Adult dose (over 50 kg): 0.3 mg auto-injector. Technique: (1) Remove from plastic case. (2) Remove blue safety cap. (3) Hold with orange tip pointing downward. (4) Position at 90 degrees to middle of outer thigh. (5) Press firmly (swing and jab) until click is heard. (6) Hold in place for 10 seconds. (7) Can be administered through clothing. (8) After removal, massage injection site for 10 seconds. (9) Hand used auto-injector to paramedics.

WARNING: Epinephrine is time-critical. Do not delay. Over 50% of anaphylaxis deaths occur within the first 60 minutes.

Is an EpiPen available?

YES -> Administer immediately using the technique described. Proceed to Step 8.

NO -> Maintain airway, positioning, and monitoring. Inform the 101 dispatcher that no EpiPen is available. Wait for EMS.

Step 8: Reassess the patient after 5 minutes.

Check airway, breathing, circulation, and level of consciousness.

Has the patient's condition improved after 5 minutes?

YES -> Continue monitoring. Epinephrine effects last approx. 20 minutes; symptoms may recur (biphasic reaction). Do not leave patient unattended.

NO -> Administer a second dose of epinephrine if a second auto-injector is available. Continue monitoring.

Step 9: Monitor continuously until EMS arrives.

Continuously monitor airway, breathing, and circulation. Be prepared to begin CPR if the patient loses consciousness and stops breathing. All patients with suspected anaphylaxis require hospital evaluation, even if symptoms fully resolve after epinephrine.

WARNING: Biphasic reactions can occur hours after the initial event. Hospital observation is mandatory.

DO NOT:

X Do not delay calling 101 -- anaphylaxis is a life-threatening emergency.
X Do not delay epinephrine administration if anaphylaxis is suspected. Epinephrine is the only effective first-line treatment.
X Do not substitute antihistamines (e.g., diphenhydramine) for epinephrine. Antihistamines are not effective against anaphylaxis.
X Do not make the patient stand or sit up suddenly if they are hypotensive -- this can cause cardiac arrest.
X Do not leave the patient unattended after epinephrine administration -- symptoms may recur.
X Do not assume the patient is safe after symptoms resolve -- all suspected anaphylaxis requires hospital evaluation.
X Do not waste time removing clothing before injecting the EpiPen -- it can be administered through trousers.
X Do not administer the EpiPen into a vein -- it is an intramuscular injection into the outer thigh only.
X Do not give food or drink to a patient experiencing anaphylaxis.

Source: Magen David Adom (MDA) — MDA First Aid Guidance on Allergies and ALS Protocol (April 2024); Israeli Ministry of Health Anaphylaxis Guidance (2024-04-01)
Last verified: 2026-03-15
Protocol ID: IL-ADULT-ALLERGIC-REACTION-001
Personal reference only — not medical advice. Always call 101 in an emergency.

ASTHMA ATTACK

Adult Protocol

Israel — Adult

CALL 101 (MDA (מד"א)) — CALL IMMEDIATELY IF IN DOUBT

MDA (מד"א): 101	**Police (משטרה):** 100	**Fire (כיבוי):** 102	**Hatzalah (הצלה):** 1221

CALL 101 IF: Symptoms do not improve within minutes of inhaler use – person cannot speak in full sentences – lips or fingertips turn blue – person loses consciousness – no inhaler is available.

SUMMARY: First aid for an asthma attack: position upright, administer rescue inhaler, and call 101 if symptoms do not improve. Signs: shortness of breath, wheezing, persistent coughing, chest tightness, difficulty speaking, or visible breathing effort.

Step 1: Help the person sit upright or lean slightly forward.

This position makes breathing easier by reducing pressure on the airways. Do NOT lay them down.

WARNING: Do not lay the person down -- this worsens breathing difficulty during an asthma attack.

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Step 2: Reassure and calm the person. Stay with them -- do not leave them alone.

Speak calmly. Anxiety worsens bronchospasm and can escalate the attack.

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Step 3: Check if the person has a rescue inhaler (Ventolin / mashaf).

Does the person have their own rescue inhaler available?

YES → Provide the inhaler immediately. Go to Step 4.

NO → Call MDA at 101 immediately. Continue to monitor and reassure while waiting for help.

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Step 4: Administer the rescue inhaler: 2--4 puffs initially.

If the person cannot operate the inhaler themselves, operate it for them. Use a spacer device (merkhav) if available -- it significantly improves drug delivery.

↓

Step 5: Wait several minutes and assess the effect of the inhaler.

Has breathing improved after the initial dose?

YES → Continue to monitor the person. Allow time to recover. If symptoms return, repeat 2–4 puffs.

NO → Repeat 2–4 puffs. If still no improvement, go to Step 6.

↓

Step 6: Call MDA at 101 if symptoms do not improve after repeated inhaler use.

Are any of the following present: cannot speak in full sentences, blue lips or fingertips, loss of consciousness, silent chest?

YES → This is a life-threatening emergency. Call 101 immediately if not already done. Stay with the person and keep them upright.

NO → Continue monitoring. Seek medical evaluation even if symptoms partially improve.

↓

Step 7: Monitor continuously until emergency services arrive.

Watch for worsening signs: skin colour changes (cyanosis), increased breathing effort, decreased responsiveness, silent chest (no wheezing despite obvious distress -- indicates minimal air movement).

Step 8: If the person loses consciousness and stops breathing, begin CPR.

Has the person lost consciousness and stopped breathing?

YES → Begin CPR immediately. Ensure 101 has been called.

NO → Continue to keep them upright and monitor until help arrives.

DO NOT:

X Do NOT lay the person down -- the upright position is essential for breathing.
X Do NOT leave the person alone during an attack.
X Do NOT administer controller medications (inhaled corticosteroids) for acute relief -- these do not provide immediate effect.
X Do NOT delay calling 101 if no inhaler is available.
X Do NOT ignore a 'silent chest' -- absence of wheezing during obvious distress indicates a life-threatening emergency with minimal air movement.

Equipment needed:

— Rescue inhaler (Ventolin / Salbutamol)
— Spacer device (merkhav) if available

Blast / Shrapnel Injuries — Adult Protocol

Israel — Adult

ID: IL-ADULT-BLAST-INJURIES-001

CALL 101 (MDA (מד"א)) — CALL IMMEDIATELY IF IN DOUBT

MDA (מד"א): 101 | **Police (משטרה):** 100 | **Fire (כיבוי):** 102 | **Hatzalah (הצלה):** 1221

When to apply:

Person has been injured by an explosion, blast, or shrapnel. Signs include: visible wounds from shrapnel or debris, active bleeding, burns, signs of shock (rapid pulse, pale/cold skin, confusion), difficulty breathing, hearing loss, visible embedded objects. Blast victims frequently have injuries from multiple mechanisms simultaneously.

Summary:

Ensure scene safety, call 101, control life-threatening hemorrhage using direct pressure, Israeli bandage, wound packing, or tourniquet, stabilise embedded objects, monitor for shock, and treat burns while awaiting MDA.

Step 1: Confirm scene safety before approaching.

Do not approach until security forces confirm the area is safe. Be aware of secondary devices (second bombs), structural collapse, fire, and ongoing threats. Your safety comes first -- you cannot help the victim if you are wounded.

WARNING: Secondary devices (second bombs) are a known tactic in Israel. Maintain distance until the area is declared safe.

Step 2: Call MDA at 101.

Report the location, number of casualties, and type of incident (explosion/blast). If there are multiple casualties, report this immediately so MDA can activate mass-casualty protocols.

Step 3: Assess for life-threatening hemorrhage.

Quickly scan the victim for severe bleeding. Hemorrhage control is the life-saving priority. MDA's approach is 'scoop and run': perform only immediate life-saving treatment, then prepare for rapid evacuation.

Is there life-threatening bleeding from a limb?

YES → Proceed immediately to hemorrhage control (Step 4). Hemorrhage control takes priority over all other interventions.

NO → Check for other injuries (embedded objects, burns, breathing difficulty). Proceed to Step 7.

Step 4: Apply direct pressure to the bleeding site.

Apply your hand or sterile gauze directly to the wound. Maintain continuous pressure to allow clot formation. Transition to an Israeli bandage (emergency bandage) when possible -- its dual-pad design creates direct pressure while wrapping around the wound.

Has the bleeding stopped or slowed significantly with direct pressure?

YES → Maintain pressure or secure with an Israeli bandage. Verify pulse distal to the bandage to confirm no arterial blockage. Proceed to check for other injuries (Step 7).

NO → Escalate to wound packing (Step 5) or tourniquet application (Step 6).

Step 5: Pack large wounds if direct pressure is insufficient.

For large wounds in limbs, shoulders, or groin, fill the wound with gauze to create internal pressure. Secure with bandaging over the top.

Has wound packing controlled the bleeding?

YES → Secure the packing with bandaging and proceed to check for other injuries (Step 7).

NO → Apply a tourniquet (Step 6).

Step 6: Apply a tourniquet for uncontrollable limb hemorrhage.

Apply when direct pressure and wound packing fail, or when there is amputation, severe crushing injury, or uncontrollable hemorrhage. Also apply in multi-casualty situations where you cannot maintain pressure on every wound. Place the tourniquet above the injury site, tighten until bleeding stops, and document the time of application in an obvious location.

WARNING: Do NOT remove the tourniquet once applied -- only medical professionals should make this decision. If bleeding does not stop, apply a second tourniquet above the first one.

Step 7: Check for and stabilise embedded objects.

If shrapnel or debris is embedded in the body, do NOT remove or move it. Apply pressure on either side of the object and stabilise it in place if possible.

WARNING: Never remove embedded objects -- removal may cause uncontrollable bleeding.

Step 8: Look for additional wounds.

Blast victims often have multiple injury sites from different mechanisms (shrapnel, blast displacement, burns). Examine the entire body for wounds beyond the most obvious one.

Are there additional bleeding wounds found?

YES → Apply hemorrhage control to each wound using the same graduated approach (direct pressure, wound packing, tourniquet as needed).

NO → Continue to monitoring and burn assessment.

Step 9: Assess and treat burns.

Burns are present in approximately 33% of ICU-admitted blast patients. Cool burns with clean water if available. Do not remove clothing stuck to burns. Cover with clean, non-adherent dressing.

Step 10: Monitor for shock and reassure the victim.

Watch for signs of shock: rapid pulse, pale or cold skin, confusion, rapid breathing. Speak to the victim and try to reassure them. Do not let them move until MDA arrives. Keep the victim warm.

Is the victim conscious and breathing?

YES → Continue monitoring and reassurance. Keep the victim still and warm until MDA arrives.

NO → If unconscious but breathing, place in recovery position (on their side). If not breathing, begin CPR immediately (30:2 compression-to-ventilation ratio). Report status to MDA dispatch.

DO NOT:

X DO NOT approach the scene until security forces confirm it is safe -- secondary devices (second bombs) are a known tactic.
X DO NOT remove embedded shrapnel or debris from the body -- this may cause uncontrollable bleeding.
X DO NOT remove a tourniquet once it has been applied -- only medical professionals should make this decision.
X DO NOT remove clothing that is stuck to burns.
X DO NOT let the victim move or walk until MDA arrives.
X DO NOT use head-tilt for airway management in blast/trauma victims -- use jaw-thrust manoeuvre to protect the cervical spine.
X DO NOT administer epinephrine in hypovolemic trauma -- blood vessels are already maximally constricted from shock (EMT/paramedic-level note).
X DO NOT assume only one injury site -- blast victims typically have injuries from multiple mechanisms simultaneously.

Equipment needed:

— Phone to call 101 (MDA)
— Sterile gauze or clean cloth for direct pressure
— Israeli bandage (emergency bandage)
— Gauze for wound packing
— Tourniquet (CAT tourniquet preferred; improvised tourniquet as last resort)
— Non-adherent dressings for burns
— Clean water for cooling burns
— Blanket for warmth

Source: Magen David Adom (MDA) — MDA First Aid Guidelines - Blast and Shrapnel Injuries
Imported: 2026-03-16 · Last verified: 2026-03-16
Personal reference only — not medical advice. Always call 101 in an emergency.

BURNS FIRST AID PROTOCOL (ADULT)

Israel — Adult

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

Call 101 immediately for: extensive burns with severe pain; white/grey/black discolouration; multiple blisters; burns on face, mouth, tongue, hands, feet, genitals, or groin; burns larger than the casualty's palm; chemical or electrical burns; inhalation injuries; high-risk patients (pregnant, elderly, chemotherapy).

When to apply: When an adult has sustained a burn from heat, chemicals, electricity, or radiation. Signs include redness, swelling, blisters, white/grey/black discolouration of skin, or pain at the injury site.

Step 1: Ensure scene safety and remove the casualty from the heat source.

Make sure the scene is safe for the rescuer before approaching. Remove the casualty from the source of the burn (fire, hot liquid, chemical, electrical source).

Step 2: Assess the burn severity.

First-degree: red, slightly swollen, painful, no blisters. Second-degree: severe pain, redness, fluid-filled blisters. Third-degree: brown-white, grey, yellowish, or black tissue, stiff and dry, reduced or absent pain due to nerve destruction.

Is the burn severe? (Third-degree, covers more than the palm, affects face/hands/feet/genitals, or is chemical/electrical/inhalation)

YES → Call 101 (MDA) immediately, then continue first aid steps while waiting for emergency services.

NO → Continue first aid steps. Monitor for worsening and seek medical evaluation if concerned.

Step 3: Cool the burn under cool (not cold) running water for at least 10 minutes.

Begin cooling as soon as possible. Use cool or lukewarm water. Continue for 10 to 20 minutes or until pain subsides.

WARNING: Do NOT apply ice or ice-cold water. Extreme cold can cause additional tissue and blood vessel damage.

Step 4: Remove rings, bracelets, watches, and loose clothing from the affected area.

Do this before swelling begins to prevent circulation problems.

Is clothing stuck to the burn?

YES → Cut around the stuck clothing. Do NOT pull it off.

NO → Gently remove the clothing from the burn area.

Step 5: Cover the burn loosely with cling film (food wrap), a clean plastic bag, or a clean non-fluffy dressing.

Do not wrap tightly. The covering protects the wound from contamination while allowing for swelling.

WARNING: Do NOT apply toothpaste, ground coffee, raw egg, yogurt, butter, fat-based creams, or any home remedies. These trap heat and increase infection risk.

Step 6: Position the patient lying down to reduce shock damage.

Keep the patient warm to prevent hypothermia, especially with large burns.

Step 7: Assess for chemical burn.

Was the burn caused by a chemical substance?

YES → Remove contaminated clothing carefully (protect yourself). Rinse with massive amount of cool running water for at least 15–20 minutes. If in eyes, flush for at least 15 minutes. Do NOT attempt chemical neutralisation. Call 101.

NO → Proceed to assess for electrical burn.

Step 8: Assess for electrical burn.

Was the burn caused by electricity?

YES → Ensure power source is disconnected before approaching. Call 101 immediately. Be prepared to perform CPR if patient becomes unresponsive. Electrical burns may cause internal injuries not visible on the surface.

NO → Continue monitoring the patient until emergency services arrive or the patient receives medical evaluation.

Step 9: Monitor for complications while waiting for emergency services.

Watch for signs of infection (redness, warmth, pus, fever), respiratory issues (if inhalation injury suspected), and signs of hypovolemic shock. For mild first-degree burns, ibuprofen or paracetamol may be used for pain relief.

DO NOT:

X Do NOT apply ice or ice-cold water to the burn -- extreme cold causes additional tissue and blood vessel damage.
X Do NOT apply toothpaste, ground coffee, raw egg, yogurt, butter, or other home remedies -- these trap heat and increase infection risk.
X Do NOT rupture or pop blisters -- the fluid inside protects the underlying tissue during healing.
X Do NOT use fat-based creams or ointments on fresh burns.
X Do NOT remove clothing that is stuck to the burn -- cut around it instead.
X Do NOT attempt chemical neutralisation on chemical burns (do not apply acid to a base burn or vice versa).
X Do NOT approach an electrical burn casualty until the power source is confirmed disconnected.

Equipment needed:

- Cool running water source
- Cling film (food wrap) or clean plastic bag
- Clean non-fluffy dressings or bandages
- Scissors (to cut clothing around stuck areas)
- Blanket (to keep patient warm and prevent hypothermia)

Personal reference only – not medical advice. Always call 101 in an emergency.

CHOKING (Foreign Body Airway Obstruction)

Israel — Adult

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

Recognise choking: Grasping neck, unable to speak / cough / breathe, face turning red or blue, high-pitched wheezing, sudden distress while eating.

Step 1: Assess the choking victim. Ask: 'Are you choking? Can you speak?'

Determine whether the obstruction is partial or complete. Look for: grasping at throat, inability to speak or cough, face turning red/blue, high-pitched wheezing, loss of consciousness.

Can the person cough forcefully and speak?

YES → DO NOT intervene. Allow them to cough. Coughing is the body's most effective defence. Monitor closely and be ready to act if the situation worsens.

NO → Airway is completely obstructed. Proceed to Step 2 immediately.

Step 2: Call for help. Instruct a bystander to call 101 (MDA) immediately.

If alone, begin life-saving actions first (Heimlich maneuver), then call 101 as soon as possible. Do not delay treatment to make the call.

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Step 3: Determine if special population requiring modified technique.

Is the person pregnant, very obese, or in a wheelchair?

YES → Use **chest thrusts** instead: place hands on centre of breastbone (sternum) and thrust firmly. For wheelchair users, perform while seated. Proceed to Step 4 using chest thrusts.

NO → Proceed to Step 4 using standard abdominal thrusts (Heimlich maneuver).

Step 4: Perform abdominal thrusts (Heimlich maneuver).

Stand behind the person. Wrap arms around their waist. Make a fist with one hand. Place the thumb side above the navel and below the breastbone. Grasp fist with other hand. Perform rapid, forceful thrusts inward and upward. Repeat until the object is expelled.

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Step 5: After each set of thrusts, reassess the person.

Has the object been expelled and can the person breathe?

YES → Stop thrusts. Monitor the person. **Seek medical evaluation** even after successful removal — internal injuries from thrusts are possible.

NO → Continue thrusts if conscious (repeat Step 4). If the person **loses consciousness**, proceed to Step 6.

Step 6: If person loses consciousness: STOP abdominal thrusts immediately.

Carefully lower the person to the ground onto a firm, flat surface.

WARNING: Do not continue abdominal thrusts on an unconscious person.

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Step 7: Call 101 (MDA) if not already done.

If alone and emergency services have not yet been contacted, call now before beginning CPR.

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Step 8: Begin CPR: 30 chest compressions followed by 2 rescue breaths.

Before each set of rescue breaths, open the airway and look in the mouth for a visible object. If visible, remove it carefully with a finger sweep. If NOT visible, do NOT perform a blind finger sweep. Continue CPR cycles (30:2) until emergency services arrive or the person recovers.

Step 9 — Self-treatment if choking while alone

Lean over a firm object (back of a chair or countertop edge). Press your upper abdomen forcefully against the object to generate an upward thrust. Repeat until the object is expelled. Call 101 as soon as you can speak.

DO NOT:

X Perform abdominal thrusts on a person who can cough forcefully and speak -- you may worsen the obstruction or cause injury.
X Perform back blows (slaps on the back) on an adult. The Israeli/MDA protocol explicitly describes this as incorrect and potentially harmful for adults.
X Perform blind finger sweeps -- only remove an object from the mouth if you can clearly see it.
X Attempt to give water or any liquid to a choking person.
X Continue abdominal thrusts on an unconscious person -- transition to CPR.
X Use abdominal thrusts on pregnant women or very obese individuals -- use chest thrusts instead.

Source: Magen David Adom (MDA) · MDA 101 Public First Aid Guidance Series; corroborated by BLS Guide 2016
Last verified: 2026-03-15 · Protocol ID: IL-ADULT-CHOKING-001
Personal reference only — not medical advice. Always call 101 in an emergency.

CPR (Cardiopulmonary Resuscitation) -- Adult

Israel – Adult (age 12+)

CALL 101 (MDA) – CALL IMMEDIATELY IF IN DOUBT

WHEN TO APPLY: An adult (age 12+) is found unresponsive and is not breathing normally (absent breathing or only gasping/agonal breathing).

Equipment needed: AED (Automated External Defibrillator) | Barrier device or pocket mask (if available) | Phone (for 101 and speakerphone)

Step 1: Check responsiveness: call out to the person, shake their shoulders gently, and ask loudly 'Are you okay?'

In Hebrew: 'Ha'im ata beseder?' Apply gentle stimulation such as a light pinch to determine if they respond.

Is the person responsive?

YES -> Do not perform CPR. Assess their condition and provide appropriate first aid. Call 101 if needed.

NO -> Proceed to Step 2 (check breathing).

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Step 2: Check breathing for no more than 10 seconds: look for chest rise, listen for breath sounds, feel for air on your cheek.

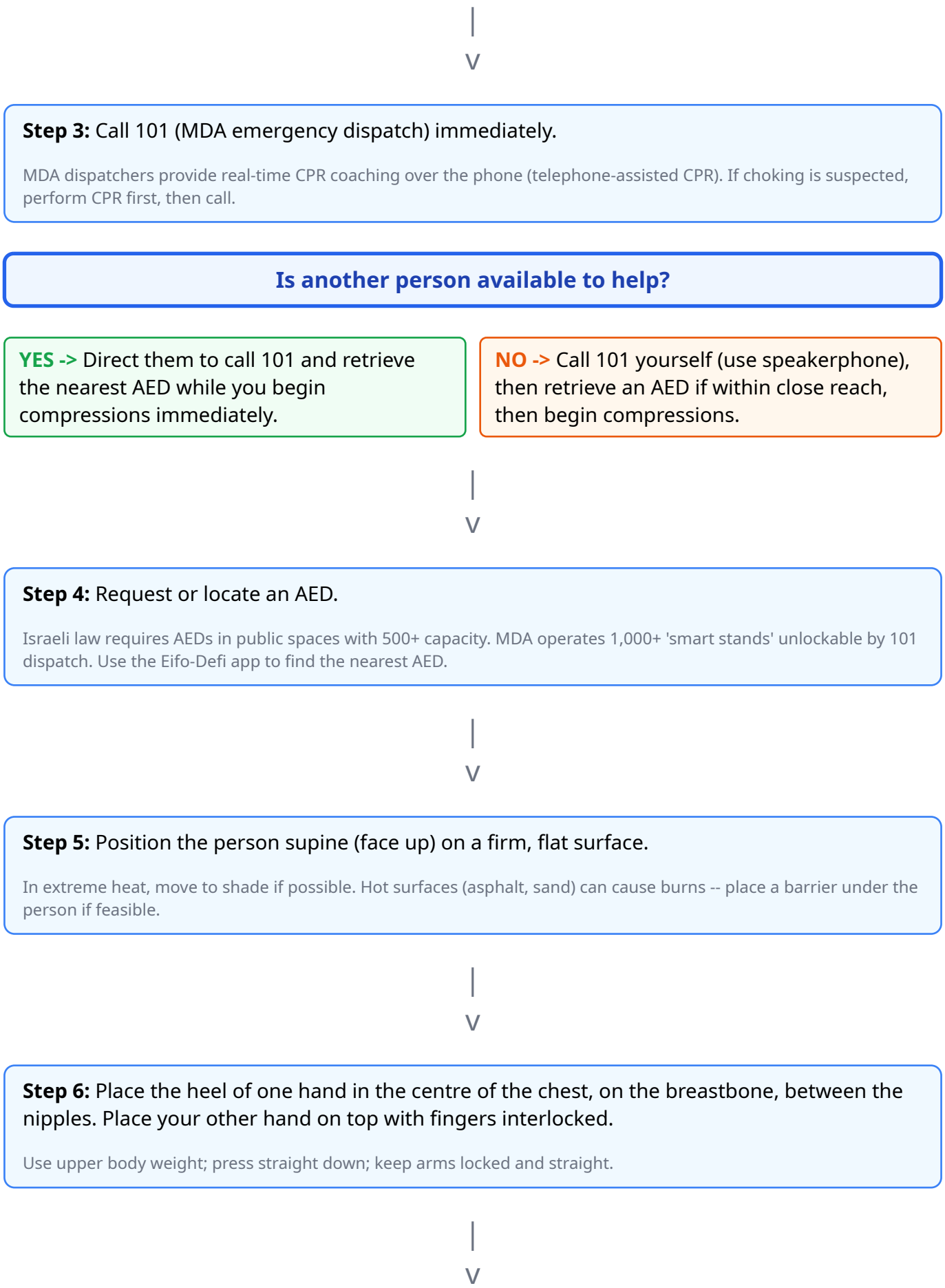
Gasping (agonal breathing) is NOT normal breathing -- treat it as cardiac arrest and proceed with CPR.

WARNING: Do not spend more than 10 seconds checking for breathing. Delays reduce survival.

Is the person breathing normally?

YES -> Place in recovery position (on their side, upper leg bent at 90 degrees). Call 101. Monitor breathing until EMS arrives.

NO -> Proceed to Step 3 (activate emergency response).


Step 3: Call 101 (MDA emergency dispatch) immediately.
MDA dispatchers provide real-time CPR coaching over the phone (telephone-assisted CPR). If choking is suspected, perform CPR first, then call.
Is another person available to help?
YES -> Direct them to call 101 and retrieve the nearest AED while you begin compressions immediately.
NO -> Call 101 yourself (use speakerphone), then retrieve an AED if within close reach, then begin compressions.
Step 4: Request or locate an AED.
Israeli law requires AEDs in public spaces with 500+ capacity. MDA operates 1,000+ 'smart stands' unlockable by 101 dispatch. Use the Eifo-Defi app to find the nearest AED.
Step 5: Position the person supine (face up) on a firm, flat surface.
In extreme heat, move to shade if possible. Hot surfaces (asphalt, sand) can cause burns -- place a barrier under the person if feasible.
Step 6: Place the heel of one hand in the centre of the chest, on the breastbone, between the nipples. Place your other hand on top with fingers interlocked.
Use upper body weight; press straight down; keep arms locked and straight.

KEY PARAMETERS

100–120	5–6 cm	30 : 2
compressions/min	depth (2–2.4 in)	compressions : breaths

Step 7: Begin chest compressions: push hard and fast at 100--120 per minute, to a depth of 5--6 cm (2--2.4 inches).

Allow full chest recoil between compressions. Do not lean on the chest. Minimise interruptions -- keep pauses under 10 seconds.

WARNING: Inadequate depth or rate, incomplete recoil, and excessive interruptions all reduce CPR effectiveness.

Are you trained in CPR and willing to give rescue breaths?

YES -> Perform 30 compressions then 2 rescue breaths (proceed to Step 8).

NO -> Perform continuous chest compressions at 100–120/min without stopping (hands-only CPR). Skip to Step 11.

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Step 8: After 30 compressions, open the airway: head-tilt / chin-lift manoeuvre -- tilt the head back gently and lift the chin.

This lifts the tongue away from the back of the throat to open the airway.

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Step 9: Deliver 2 rescue breaths: pinch nose closed, seal your mouth over theirs, deliver each breath over 1 second with enough volume for visible chest rise.

WARNING: Do not over-ventilate. Excessive ventilation increases gastric inflation and reduces venous return.

Does the chest rise visibly with each breath?

YES -> Resume compressions immediately after the 2 breaths (proceed to Step 10).

NO -> Reposition the head (re-tilt) and reattempt. If chest still does not rise, resume compressions – do not delay for repeated attempts.

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Step 10: Resume compressions immediately after the 2 breaths. Continue cycles of 30 compressions and 2 breaths.

Each cycle of 30:2 takes approximately 2 minutes for 5 cycles.

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Step 11: When the AED arrives, turn it on and apply the electrode pads immediately without delay.

One pad under the right collarbone. The other pad under and to the left of the left breast. Alternative: one pad on the front, one on the back. Follow the diagram on the pads.

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Step 12: Follow the AED's voice prompts. The device will analyse the heart rhythm and advise whether a shock is indicated.

WARNING: Ensure no one is touching the person during AED analysis and shock delivery. Announce 'Stand clear!' before analysis and before any shock.

Does the AED advise a shock?

YES -> Ensure everyone is clear, then press the shock button. Resume CPR immediately after shock.

NO -> Resume CPR immediately. The AED will re-analyse after 2 minutes.

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Step 13: Resume CPR immediately after shock delivery (or if no shock advised). Do not wait for the AED to re-analyse.

The AED will prompt for another rhythm check after approximately 2 minutes (5 cycles of 30:2).

Step 14: Reassess every 2 minutes (approximately 5 cycles of 30:2).

If two or more trained rescuers are present, rotate compressors every 2 minutes to maintain quality. Complete the switch in under 10 seconds.

Step 15: Continue CPR until: EMS (MDA) arrives and takes over; the person shows signs of life (breathing, movement, coughing); an AED prompts you to stop; or you are physically unable to continue.

Has the person begun breathing normally on their own?

YES -> Stop CPR. Place in recovery position (on their side, upper leg bent at 90 degrees). Monitor until EMS arrives.

NO -> Continue CPR cycles and AED use. Do not stop until EMS takes over or you are physically unable to continue.

DO NOT:

X Do not delay calling 101 -- early activation of EMS is critical for survival.
X Do not spend more than 10 seconds checking for breathing.
X Do not interrupt chest compressions for more than 10 seconds for any reason.
X Do not lean on the chest between compressions -- allow full recoil.
X Do not compress deeper than 6 cm (2.4 inches) to avoid internal injury.
X Do not over-ventilate -- deliver each breath over 1 second only, just enough for visible chest rise.
X Do not touch the person while the AED is analysing the rhythm or delivering a shock.
X Do not delay AED use -- apply it as soon as it arrives, even if CPR is in progress.
X Do not wait for the AED to re-analyse before resuming compressions after a shock.
X Do not move the person unnecessarily -- perform CPR where they are unless the scene is unsafe.
X Do not assume gasping (agonal breathing) is normal breathing -- it is a sign of cardiac arrest.

Source: Magen David Adom (MDA) | Edition: AHA 2025 Guidelines | Published: 2025-01-01 | Last verified: 2026-03-15
Protocol ID: IL-ADULT-CPR-001
Personal reference only – not medical advice. Always call 101 in an emergency.

DEHYDRATION AND HEAT-RELATED ILLNESS

Adult Protocol

Israel — Adult

CALL 101 (MDA (מד"א)) — CALL IMMEDIATELY IF IN DOUBT

MDA (מד"א: 101) **Police (משטרה:** 100) **Fire (כיבוי:** 102) **Hatzalah (הצלה:** 1221)

CALL 101 IF: Heat stroke (body temp >40 degrees C, sweating stopped, confusion, loss of consciousness, seizures) – dehydration not improving after 30 min of rest and rehydration – vomiting prevents oral rehydration – signs of shock.

SUMMARY: First aid for dehydration through gradual rehydration with small sips, cooling, and rest. Escalates to emergency treatment for heat stroke. Key distinction: dehydration involves continued sweating; heat stroke involves ABSENT sweating with high body temperature.

Step 1: Assess the severity of the condition.

Does the person show signs of heat stroke (body temp >40 degrees C, hot/dry/red skin with NO sweating, confusion, loss of consciousness)?

YES → This is a medical emergency. Go to Step 8 (heat stroke protocol).

NO → Proceed to Step 2 for dehydration management.

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DEHYDRATION PATHWAY

Step 2: Stop all physical activity. Ensure complete rest.

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Step 3: Move the person to a shaded, cool place -- air-conditioned room if available.

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Step 4: Remove excess clothing and loosen tight garments.

↓

Step 5: Rehydrate gradually with small sips of water or ORS (Oral Rehydration Solution).

ORS is preferred over plain water for moderate dehydration. Use a straw for small sips if needed. ORS can be prepared at home: 1 litre of clean water + 6 teaspoons sugar + 0.5 teaspoon salt. Commercial ORS (Electrolit, Hydralite) available at Israeli pharmacies.

WARNING: Do NOT drink rapidly -- rapid fluid replacement can worsen symptoms due to electrolyte imbalance and may increase nausea and vomiting.

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Step 6: Cool the body with wet cloths, a fan, or cool (not cold) water spray.

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Step 7: Monitor the person's condition.

Have symptoms improved after 30 minutes of rest and rehydration?

YES → Continue gradual rehydration and rest. Monitor for relapse.

NO → Call MDA at 101. The person may need intravenous fluids.

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HEAT STROKE EMERGENCY PATHWAY

Step 8: HEAT STROKE EMERGENCY: Call MDA at 101 immediately.

WARNING: Heat stroke is a life-threatening emergency. Body temperature regulation has failed and internal organs are being damaged.

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Step 9: Move the person to a shaded, ventilated, or air-conditioned location. Remove excess clothing.

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Step 10: Cool the body rapidly using ice, wet towels, fans, and cold water on the torso.

Active cooling must continue until MDA arrives.

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Step 11: Assess consciousness.

WARNING: Do NOT wet the head -- risk of choking if consciousness is altered.

Is the person conscious?

YES → Offer small sips of water only.

NO → Lay on back, elevate legs. If breathing, place in recovery position. Do NOT give fluids.

↓

Step 12: Continue active cooling and monitoring until MDA arrives.

DO NOT:

X Do NOT give large quantities of water quickly -- rapid fluid replacement disrupts electrolyte balance.
X Do NOT give sugary drinks, coffee, or alcohol.
X Do NOT wet the head of a heat stroke patient -- choking risk if consciousness is altered.
X Do NOT ignore the key distinction: dehydration involves continued sweating; heat stroke involves ABSENT sweating with high body temperature.
X Do NOT delay calling 101 for heat stroke -- it is a medical emergency requiring hospital treatment.
X Do NOT give fluids to an unconscious person.

Equipment needed:

— Water or ORS (Oral Rehydration Solution)
— Fan or method of creating air flow
— Wet cloths or towels for cooling
— Ice packs (for heat stroke)
— Straw (for controlled sipping)

Personal reference only – not medical advice. Always call 101 in an emergency.

DIABETIC EMERGENCIES

Adult Protocol

Israel — Adult

CALL 101 (MDA (מד"א)) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :(מד"א **Police (100 :(משטרה)** **Fire (102 :(כיבוי)** **Hatzalah (1221 :(הצלה)**

CALL 101 IF: Patient is unconscious – has seizures – shows signs of diabetic ketoacidosis – does not improve within 10–15 minutes of sugar administration – or if you are unsure of the type of diabetic emergency.

SUMMARY: Assess whether the patient has low blood sugar (hypoglycemia) or high blood sugar (hyperglycemia). When in doubt, treat for hypoglycemia by giving sugar – this is the safer default. Call 101 for all severe or uncertain cases.

WHEN TO APPLY: Person is a known or suspected diabetic showing signs of altered consciousness, confusion, weakness, trembling, sweating, behavioural changes, excessive thirst, fruity breath odour, or rapid deep breathing. Also apply when any person has unexplained altered consciousness (rule out hypoglycemia even in non-diabetic patients).

Step 1: Assess the scene and check for medical identification.

Look for a medical alert bracelet, necklace, or card in the patient's wallet that may confirm a diabetes diagnosis and current medications. Check for insulin pumps, pens, or glucometers nearby.

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Step 2: Check blood glucose level if a glucometer is available.

A reading below 60--70 mg/dL indicates hypoglycemia. A reading above 250 mg/dL indicates hyperglycemia. Glucose testing should be performed on any patient with unexplained altered consciousness, even if they are not a known diabetic.

Is a glucometer available and can you obtain a reading?

YES → Use the reading to determine whether the patient has hypoglycemia (low) or hyperglycemia (high), and proceed to the appropriate steps.

NO → Assess based on signs and symptoms. If uncertain whether blood sugar is high or low, treat for hypoglycemia (give sugar) – this is the safer default.

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Step 3: Determine whether the patient has hypoglycemia or hyperglycemia based on signs and symptoms.

Hypoglycemia (rapid onset): sweating, pallor, trembling, confusion, irritability, hunger, rapid pulse, seizures.
Hyperglycemia (gradual onset over hours to days): excessive thirst, frequent urination, fruity/acetone breath, fast deep breathing (Kussmaul), warm dry skin, nausea, weakness.

Do the signs suggest hypoglycemia (low blood sugar)?

YES → Proceed to Step 4 (hypoglycemia treatment pathway).

NO → If signs suggest hyperglycemia, proceed to Step 8. If unsure, treat for hypoglycemia (give sugar) as the safer default.

HYPOGLYCEMIA PATHWAY (LOW BLOOD SUGAR)

Step 4: HYPOGLYCEMIA PATHWAY: Assess the patient's level of consciousness.

Is the patient conscious and able to swallow safely?

YES → Proceed to Step 5 (give sugar to conscious patient).

NO → Proceed to Step 6 (assess if patient has reduced consciousness or is fully unconscious).

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Step 5: Give sugar immediately to the conscious patient.

Administer rapidly absorbed sugars: sweetened drink (fruit juice, sugary soft drink -- NOT diet), sugar cubes or glucose tablets, glucogel packets (15g glucose -- standard in United Hatzalah volunteer kits), honey or jam. Choose foods and drinks WITHOUT added fat, as fat slows sugar absorption. After symptoms improve, give a complex carbohydrate (bread, crackers) to maintain blood sugar levels. Monitor the patient and do not leave them alone.

Do symptoms improve within 10--15 minutes of giving sugar?

YES → Continue monitoring the patient. Provide complex carbohydrates. Do not leave the patient alone.

NO → Call 101 (MDA) immediately. The patient may need intravenous dextrose from paramedics.

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Step 6: Assess whether the patient has reduced consciousness or is fully unconscious.

Is the patient partially responsive (confused, drowsy, but not fully unconscious)?

YES → If safe to do so, apply a sweet paste (honey, chocolate spread) under the tongue or to the inside of the cheek for absorption through the oral mucosa. Call 101 (MDA) immediately.

NO → Patient is unconscious. Proceed to Step 7.

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Step 7: Treat the unconscious hypoglycaemic patient.

Place patient in the recovery position (on their side) to maintain an open airway. Call 101 (MDA) immediately. If the patient has a glucagon injection kit and you are trained to use it, administer glucagon. If the patient has an insulin pump, disconnect it gently to stop further insulin delivery. Monitor breathing continuously and be prepared to perform CPR if breathing stops. Wait for MDA paramedics, who can administer intravenous dextrose.

WARNING: DO NOT give food or drink to an unconscious patient -- risk of choking and aspiration. DO NOT give insulin -- it will further lower blood sugar and worsen the emergency.

Is the patient breathing?

YES → Maintain recovery position and continue monitoring until MDA paramedics arrive.

NO → Begin CPR immediately and continue until paramedics arrive or the patient resumes breathing.

HYPERGLYCEMIA PATHWAY (HIGH BLOOD SUGAR)

Step 8: HYPERGLYCEMIA PATHWAY: Call 101 (MDA) immediately if diabetic ketoacidosis or severe hyperglycemia is suspected.

Signs of diabetic ketoacidosis include fruity/acetone breath, fast deep breathing (Kussmaul breathing), nausea, vomiting, confusion, and warm dry skin.

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Step 9: Ensure the airway is open and clear.

Loosen tight clothing that may restrict breathing.

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Step 10: Allow the patient to drink water if conscious.

This helps combat dehydration, which worsens hyperglycemia. In Israel's hot climate, dehydration can be a significant exacerbating factor.

Is the patient conscious and able to drink?

YES → Offer water in small sips. Continue monitoring.

NO → Do not give anything by mouth. Place the patient in the recovery position. Monitor breathing and be prepared to perform CPR.

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Step 11: Monitor the patient until MDA paramedics arrive.

Watch for deterioration of consciousness. Be prepared to place the patient in the recovery position if they lose consciousness, and to begin CPR if breathing stops.

WARNING: DO NOT administer insulin -- insulin dosing requires medical expertise and equipment. This is a paramedic/physician intervention only.

DO NOT:

X DO NOT administer insulin as a first aider -- insulin dosing requires medical expertise and clinical assessment. Leave this to MDA paramedics or physicians.
X DO NOT give food or drink to an unconscious patient -- risk of choking and aspiration.
X DO NOT give diet drinks or sugar-free foods when treating hypoglycemia -- they will not raise blood sugar.
X DO NOT give sugary foods high in fat (chocolate bars, ice cream) as first-line treatment for hypoglycemia -- fat slows sugar absorption.
X DO NOT withhold sugar when in doubt about whether blood sugar is high or low -- giving sugar to a hypoglycaemic patient is life-saving, and giving sugar to a hyperglycaemic patient will not cause significant immediate additional harm.
X DO NOT leave a diabetic emergency patient unattended -- their condition can deteriorate rapidly.
X DO NOT delay calling 101 if the patient is unconscious, seizing, or not improving after sugar administration.
X DO NOT disconnect an insulin pump roughly -- disconnect gently to avoid damaging the device or the insertion site.

Equipment needed:

— Phone to call 101 (MDA) or 1221 (United Hatzalah)
— Glucometer and test strips (if available)
— Rapidly absorbed sugar source (glucose tablets, glucogel packets, fruit juice, sugar cubes, honey)
— Complex carbohydrate for follow-up (bread, crackers)
— Glucagon injection kit (if available and trained to use)

Source: Magen David Adom (MDA) – MDA 101 Public First Aid Guidance Series; corroborated by MDA Course 60 training materials and United Hatzalah volunteer training material
Protocol ID: IL-ADULT-DIABETIC-EMERGENCIES-001 · Imported: 2026-03-15 · Last verified: 2026-03-15
Personal reference only – not medical advice. Always call 101 in an emergency.

DROWNING (NEAR-DROWNING)

Adult Protocol

Israel — Adult

CALL 101 (MDA (מד"א)) — CALL IMMEDIATELY IF IN DOUBT

MDA (מד"א): 101	**Police (משטרה):** 100	**Fire (כיבוי):** 102	**Hatzalah (הצלה):** 1221

SUMMARY: Rescue from water, assess breathing, perform CPR (ABC sequence preferred for trained rescuers), and transport to hospital – even if the person recovers.

Step 1: Ensure your own safety before attempting any rescue.

Assess the scene. Do not enter the water unless you are a trained swimmer or lifeguard. Use reaching aids (pole, rope, flotation device) if possible. Alert a lifeguard if one is present.

WARNING: Untrained rescuers who enter the water frequently become drowning victims themselves.

Are you a trained swimmer or lifeguard?

YES → You may enter the water to perform a rescue, using appropriate technique and flotation aids.

NO → Do NOT enter the water. Use reaching aids (pole, rope, flotation device), throw a buoyant object, or call for a trained rescuer. Wait for the person to be brought to shore.

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Step 2: Call 101 (MDA) immediately.

Keep the dispatcher on speakerphone. Provide your exact location (beach name, pool address, GPS coordinates if possible). If another person is present, send them to locate the nearest AED. AEDs are commonly found at lifeguard stations, first aid rooms, and public facilities with 500+ occupants in Israel.

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Step 3: Remove the person from the water as quickly as possible.

Extract the drowning person from the water. Move them to a dry area away from the water's edge. Do not endanger yourself in the process.

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Step 4: Check responsiveness and breathing.

Call to the person and tap their shoulder. Observe the chest: is it rising and falling? Listen for breath sounds. Look for signs of life.

Is the person responsive and breathing normally?

YES → Place in recovery position (on their side). Monitor breathing continuously until MDA arrives. Keep warm – remove wet clothing, cover with dry blankets. Go to Step 8.

NO → The person needs immediate CPR. Proceed to Step 5.

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Step 5: Position the person on their back on a firm, flat surface away from the water.

Ensure the surface is dry. This is essential for effective chest compressions and safe AED use.

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Step 6: Begin CPR.

Drowning is a hypoxic (oxygen-deprivation) emergency, making rescue breaths especially important. The recommended CPR sequence for drowning differs from standard cardiac arrest.

Are you trained in full CPR with rescue breaths?

YES → Use the ABC approach (Airway, Breathing, Compressions): open the airway, give 2 rescue breaths, then begin chest compressions. Continue at 30:2 ratio. Compress at 100/min.

NO → Perform hands-only CPR (chest compressions only) at 100 compressions per minute. Use “Stayin’ Alive” by the Bee Gees as a tempo guide. Continue without stopping until MDA arrives.

Step 7: Use an AED if one is available.

Dry the patient's chest thoroughly and ensure they are on a dry surface before applying AED pads. Follow the AED's voice prompts. Continue CPR between AED analysis/shock cycles as directed by the device.

WARNING: Using an AED on a wet patient or wet surface risks electrical injury to the patient and rescuer. Always dry the chest and move to a dry area first.

Step 8: Continue CPR or monitoring until MDA paramedics arrive.

Do not stop CPR unless the person begins breathing on their own or professional medical help takes over. If the person regains consciousness, place them in the recovery position and monitor continuously.

Step 9: Ensure hospital transport regardless of apparent recovery.

Even if the person regains consciousness and appears stable, mandatory hospital transport is required. Dangerous electrolyte imbalances can develop after drowning. Secondary drowning (delayed pulmonary edema) can cause symptoms 1-24 hours after the incident.

WARNING: Do NOT allow the person to refuse treatment or walk away. A person who appears recovered can deteriorate rapidly hours later due to secondary drowning or electrolyte toxicity.

DO NOT:

X DO NOT enter the water to rescue someone unless you are a trained swimmer or lifeguard -- untrained rescuers frequently become drowning victims.
X DO NOT attempt to remove water from the victim's lungs. MDA states this is impossible, dangerous, and a waste of critical time.
X DO NOT perform the Heimlich maneuver, turn the person upside down, or use any other method to 'drain' water from the lungs.
X DO NOT use an AED on a wet patient or on a wet surface -- dry the chest and move to a dry area first.
X DO NOT allow a near-drowning victim to refuse hospital transport, even if they appear fully recovered.
X DO NOT stop CPR until the person is breathing on their own or professional medical help takes over.
X DO NOT delay calling 101 -- call immediately, even before attempting rescue if you are alone.

Equipment needed:

— AED (defibrillator) -- commonly available at lifeguard stations and public facilities in Israel
— Reaching aids (pole, rope, flotation device) for water rescue
— Dry blankets or clothing to keep the victim warm after extraction

ELECTROCUTION / ELECTRICAL INJURY FIRST AID PROTOCOL (ADULT)

Israel — Adult

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א) **Police** (100 :משטרה) **Fire** (102 :כיבוי) **Hatzalah** (1221 :הצלה)

For high-voltage incidents also call: Fire Services **102** and Israel Electric Corporation (IEC) **103**

When to apply: When an adult has received an electrical shock from any source: household appliances, power lines, industrial equipment, lightning. Signs include burns (entry/exit wounds), loss of consciousness, muscle spasms, breathing difficulty, cardiac arrest, confusion, or the victim being visibly in contact with an electrical source.

Step 1: Ensure scene safety -- DO NOT touch the victim.

Contact with a victim still connected to an electrical source can be fatal to the rescuer. Multiple rescuers have been killed by 'chain electrocution'. Assess whether the victim is still in contact with the electrical source before approaching.

WARNING: DO NOT touch the victim with bare hands or any wet/damp object while they may still be in contact with the electrical source.

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Step 2: Determine whether the incident involves low-voltage or high-voltage electricity.

Low-voltage: under 1,000 volts (household and most workplace). High-voltage: 1,000 volts and above (power lines, substations, industrial equipment). Standard Israeli household supply is 220V AC / 50Hz.

Is this a HIGH-VOLTAGE incident (power lines, substations, industrial equipment)?

YES → DO NOT approach. Maintain at least 10 metres distance. Call MDA (101), Fire Services (102), and Israel Electric Corporation (103). Wait for professional teams to de-energise the source.

NO → Proceed to disconnect the power source (Step 3).

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Step 3: Disconnect the power source (low-voltage incidents only).

Turn off the power at the mains breaker or fuse box, or unplug the appliance from the wall socket.

WARNING: Never use a wet or damp object. Never touch the victim or the electrical source directly.

Can you disconnect the power at the mains or unplug the device?

YES → Switch off the mains breaker or unplug the appliance. Confirm the power is off before approaching the victim.

NO → Use a dry, non-conducting object (dry wooden broom handle, dry rope, dry rubber mat) to push or pull the victim away from the source. Stand on a dry, insulating surface.

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Step 4: Call emergency services.

Call MDA at 101 immediately. For high-voltage incidents, also call Fire Services at 102 and Israel Electric Corporation at 103. Report: location, number of casualties, whether the power source has been disconnected, and the victim's condition.

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Step 5: Assess consciousness and breathing once the scene is confirmed safe.

Call out to the victim and tap their shoulders to check consciousness. Look, listen, and feel for breathing for no more than 10 seconds.

Is the victim responsive?

YES → Keep the victim still. Proceed to Step 8 (burns assessment). Monitor continuously for deterioration.

NO → Check breathing immediately (Step 6).

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Step 6: Check breathing of the unresponsive victim.

Is the victim breathing normally (not just gasping)?

YES → Place in recovery position (on their side). Monitor breathing continuously until EMS arrives. Be prepared to begin CPR if breathing stops.

NO → Begin CPR immediately (Step 7).

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Step 7: Begin CPR if the victim is unresponsive and not breathing (or only gasping).

Perform 30 chest compressions followed by 2 rescue breaths. Compression rate: 100--120 per minute. Compression depth: 5--6 cm. Allow full chest recoil between compressions. If an AED is available, use it -- ensure the victim's chest is dry before applying pads. Do NOT stop resuscitation until professional EMS teams arrive.

WARNING: Electrocution victims, particularly younger patients, have a higher-than-average survival rate from cardiac arrest if CPR is started promptly. Do not give up.

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Step 8: Assess and treat burns.

Look for entry and exit wound burns. External burns may appear small but conceal severe internal tissue damage along the current pathway. Once the scene is confirmed safe, cool visible burns with cool (not cold) running water for at least 10 minutes. Cover burns loosely with cling film or a clean, non-fluffy dressing.

WARNING: Do NOT apply water to burns while the victim is still connected to the electrical source. Do NOT apply creams, ointments, or home remedies to electrical burns.

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Step 9: Assess for spinal injury.

Did the victim fall from height or get thrown by the electrical force?

YES → Suspect spinal injury. Minimise movement of the head, neck, and spine. Wait for professional extraction if possible.

NO → Proceed to treat for shock (Step 10).

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Step 10: Treat for shock.

Keep the victim lying down. Keep them warm by covering with a blanket. Do not give food or drink.

Is spinal injury suspected or is the victim having difficulty breathing?

YES → Do NOT elevate legs. Keep the victim flat and still. Wait for EMS.

NO → Elevate the victim's legs slightly to improve circulation.

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Step 11: Ensure hospital evaluation for ALL electrocution victims.

All electrocution victims must be evaluated at a hospital, even if the injury appears minor. Cardiac arrhythmias can develop hours after the incident. Internal tissue damage may not be immediately apparent. Rhabdomyolysis (muscle breakdown) can cause kidney failure. Neurological symptoms may be delayed.

WARNING: Do NOT assume the injury is minor based on small visible burns -- internal damage may be severe.

DO NOT:

X Do NOT touch the victim while they may still be in contact with the electrical source -- this can kill the rescuer (chain electrocution).
X Do NOT use wet or damp objects to separate the victim from the electrical source.
X Do NOT approach high-voltage incidents (power lines, substations) -- maintain at least 10 metres distance and wait for professional teams (IEC 103).
X Do NOT perform CPR on a conscious victim.
X Do NOT apply water to burns while the victim is still connected to the electrical source.
X Do NOT apply creams, ointments, or home remedies to electrical burns.
X Do NOT assume the injury is minor based on small visible burns -- internal damage along the current pathway may be severe.
X Do NOT give food or drink to an electrocution victim.

Equipment needed:

— Dry non-conducting object for separating victim from source (dry wooden broom handle, dry rope, dry rubber mat)
— Dry insulating surface to stand on (dry wood, dry rubber mat, newspapers)
— AED (automated external defibrillator)
— Cool running water source (for burn cooling after scene is safe)
— Cling film or clean non-fluffy dressings (for burn covering)
— Blanket (for shock prevention)

Personal reference only – not medical advice. Always call 101 in an emergency.

EYE INJURIES

Adult Protocol

Israel — Adult

CALL 101 (MDA (מד"א)) — CALL IMMEDIATELY IF IN DOUBT

MDA (מד"א: 101) **Police (משטרה:** 100) **Fire (כיבוי:** 102) **Hatzalah (הצלה:** 1221)

CALL 101 IF: Penetrating injury – chemical burns (after initiating flushing) – sudden vision loss – blood in the eye – embedded objects – eyeball laceration.

SUMMARY: First aid for eye injuries including foreign bodies, chemical splashes, embedded objects, blunt trauma, and lacerations. Injury type determines the response pathway.

Step 1: Assess the type of eye injury.

Is there a chemical splash in the eye?

YES → Go to Step 2 (chemical splash protocol).

NO → Go to Step 5 to assess other injury types.

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CHEMICAL SPLASH PATHWAY

Step 2: CHEMICAL SPLASH: Flush the affected eye immediately with a stream of running water for 15 to 30 minutes.

Hold the eyelid open during flushing to ensure the chemical is fully washed out. Tilt the head so water runs away from the unaffected eye.

WARNING: Do not delay flushing -- begin immediately, even before calling emergency services.

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Step 3: Remove contact lenses if present, after starting the flush.

If possible, note the chemical product name or bring the container for the medical team.

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Step 4: After flushing, take the casualty to an emergency room. Call MDA at 101 for severe chemical burns or if the person cannot open their eye.

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EMBEDDED / PENETRATING OBJECT PATHWAY

Step 5: Assess whether an object is embedded or penetrating the eye.

Is there an object embedded in or penetrating the eye?

YES → Go to Step 6 (embedded object protocol).

NO → Go to Step 9 to assess other injury types.

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Step 6: EMBEDDED OBJECT: Do NOT attempt to remove the object. Do NOT apply pressure to the eye or the object.

WARNING: Attempting to remove an embedded object can cause severe permanent damage.

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Step 7: Cover BOTH eyes loosely to prevent sympathetic eye movement.

Use a cup, cone, or ring of cloth around the object to protect it from being pushed further in.

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Step 8: Keep the casualty calm and still. Call MDA at 101 immediately -- this requires hospital treatment.

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FOREIGN BODY / BLUNT TRAUMA / LACERATION PATHWAY

Step 9: Assess whether the injury is a non-embedded foreign body, blunt trauma, or laceration.

Is there a non-embedded foreign body (dust, sand, metal shaving)?

YES → Go to Step 10 (foreign body protocol).

NO → Go to Step 13 (blunt trauma / laceration protocol).

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Step 10: FOREIGN BODY: Do NOT rub the eye.

WARNING: Rubbing can embed the object further and cause additional damage.

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Step 11: Rinse the eye with lukewarm running water for approximately 15 minutes. Check every 5 minutes whether the foreign body has been dislodged.

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Step 12: If the object does not come out with flushing, or if symptoms persist after removal (pain, redness, tearing), seek medical attention at an emergency room.

Has the foreign body been successfully removed and symptoms resolved?

YES → Monitor for any delayed symptoms. Seek medical attention if pain, redness, or tearing returns.

NO → Go to emergency room for professional removal.

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Step 13: BLUNT TRAUMA: Apply a cold compress gently. Do NOT press on the eyeball.

Is vision affected, or are there signs of serious injury (double vision, blood in eye, irregular pupil, severe pain)?

YES → Call MDA at 101 immediately. Seek emergency medical evaluation.

NO → Seek medical evaluation even if the eye looks normal – internal damage may not be visible.

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Step 14: LACERATION: If there is a cut to the eyeball or eyelid, do NOT wash the eye. Cover loosely with a sterile pad without applying pressure.

WARNING: Do not wash a lacerated eyeball -- this can worsen the injury.

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Step 15: Call MDA at 101 -- lacerations require emergency room treatment. Keep the casualty lying down if possible.

DO NOT:

X Do NOT rub the eye when a foreign body is present.
X Do NOT attempt to remove an embedded or penetrating object.
X Do NOT apply pressure to an injured eye or embedded object.
X Do NOT wash the eye if there is a laceration to the eyeball.
X Do NOT delay flushing for chemical splashes -- start immediately.
X Do NOT press a cold compress onto the eyeball for blunt trauma.

Equipment needed:

— Clean running water source for flushing
— Sterile eye pads or clean cloth
— Cold compress (for blunt trauma)
— Cup or cone (to protect embedded objects)
— Bandages to loosely cover both eyes if needed

Source: Magen David Adom (MDA) – MDA Training Department Manual – Eye Injury First Aid
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Personal reference only – not medical advice. Always call 101 in an emergency.

FRACTURES AND SPRAINS FIRST AID PROTOCOL (ADULT)

Israel — Adult

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :(מד"א | **Police (100 :(משטרה** | **Fire (102 :(כיבוי** | **Hatzalah (1221 :(הצלה**

Call 101 immediately for: any suspected fracture; open fractures (bone visible through skin); fractures involving the head, neck, spine, pelvis, or hip; loss of sensation or circulation below the injury; signs of shock; inability to immobilise the injury adequately; multiple injuries or high-energy mechanism (fall from height, road traffic accident); or any doubt about the severity of the injury.

When to apply: When an adult has sustained an injury to a bone or joint. Signs include severe pain worsening with movement, swelling and bruising, visible deformity, inability to move the affected limb, possible crepitus, or a popping sensation at the time of injury.

Step 1: Call MDA at 101.

Keep the call on speaker and provide your location. MDA can provide phone guidance on how to manage the injury at the scene.

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Step 2: Assess whether the injury is a fracture or a sprain.

A fracture typically presents with severe pain that worsens with movement, visible deformity, inability to move the limb, and rapid swelling. A sprain presents with joint pain, swelling, bruising, and limited joint movement but no deformity. If in doubt, treat as a fracture.

Does the injury show signs of a fracture (deformity, severe pain, inability to move the limb, crepitus)?

YES → Proceed to Step 3 to assess fracture type.

NO → If the injury appears to be a sprain (joint pain, swelling, no deformity), proceed to Step 9 for RICE protocol.

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Step 3: Determine whether the fracture is open or closed.

An open fracture means the bone has pierced through the skin, creating a wound. A closed fracture means the bone is broken but the skin remains intact.

Is there a wound with bone visible or protruding through the skin (open fracture)?

YES → This is a medical emergency. Proceed to Step 4 for open fracture management.

NO → This is a closed fracture. Proceed to Step 6 for closed fracture management.

OPEN FRACTURE PATH

Step 4: Open fracture: Control bleeding.

Apply direct pressure around (not on) the protruding bone to stop bleeding. If bleeding is severe and cannot be controlled with direct pressure, use a tourniquet.

WARNING: Do NOT push the bone back in. Leave it as found.

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Step 5: Open fracture: Cover the wound and immobilise.

Cover the exposed bone and wound with a sterile dressing or clean cloth. Support the limb in the position found and do not attempt to realign. Monitor for shock -- keep the patient lying down, warm, and reassured. Open fractures carry a high risk of blood loss and shock. Continue to Step 8.

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Continue to Step 8 (cold therapy and monitoring).

CLOSED FRACTURE PATH

Step 6: Closed fracture: Do NOT move the limb unnecessarily.

Allow the injured person to sit or lie in the most comfortable position. Do NOT attempt to straighten the limb, pull it, or return a bone to its place -- such actions may tear arteries and nerves.

WARNING: Do NOT attempt to straighten or realign the limb. This may cause further damage to blood vessels and nerves.

Step 7: Closed fracture: Immobilise the injured area.

The core principle is 'minimum movement, maximum stability'.

Is the fracture in the arm or upper limb?

YES → Support the injured arm with the person's healthy hand. Use an improvised sling (shirt, scarf, triangular bandage) and attach it to the chest. Place the apex of the triangle at the elbow, pass one side under the affected area and one above, and tie at the neck.

NO → For leg or lower limb fractures: ask the person not to move the leg. Place pillows or rolled blankets on both sides of the leg to prevent involuntary movement. For thigh fractures and below, use a triangular cloth above the injury site and tie firmly, securing the knot on the uninjured leg.

BOTH FRACTURE PATHS CONVERGE HERE

Step 8: Apply cold therapy and remove restrictive items.

Apply ice wrapped in a cloth (never directly on skin) to reduce swelling and pain. Apply for 15-minute intervals. Remove rings, bracelets, watches, and jewellery immediately from the affected limb to prevent tourniquet effects as swelling occurs. Monitor for signs of shock (pale skin, rapid breathing, weak pulse) and keep the patient warm and comfortable until MDA arrives.

WARNING: Do NOT apply ice directly to skin -- always wrap in cloth.

SPRAIN PATH (RICE PROTOCOL)

Step 9: Sprain: Apply the RICE protocol -- Rest.

Stop all activity involving the injured joint. Do not bear weight on the injured limb.

Step 10: Sprain: Ice.

Apply an ice pack wrapped in a cloth to the injured area for 15--20 minutes every 2--3 hours. Continue for the first 48--72 hours after injury. Do NOT apply ice directly to skin.

WARNING: Do NOT apply ice directly to skin -- always wrap in cloth.

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Step 11: Sprain: Compression.

Bandage the area firmly but not too tightly around, above, and below the injury using an elastic bandage if available. The bandage should reduce swelling without cutting off circulation.

Do the fingers or toes below the bandage become numb, pale, or blue?

YES → Loosen the bandage immediately – circulation is being compromised.

NO → Bandage tension is appropriate. Continue to elevation.

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Step 12: Sprain: Elevation.

Elevate the injured limb above the level of the heart using pillows or cushions. This helps reduce swelling by promoting fluid drainage.

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Step 13: Determine whether the sprain requires emergency evaluation.

Some sprains may actually be fractures and cannot be distinguished without imaging.

Can the person bear no weight at all on the limb, is there visible deformity, is pain severe and unrelenting, has numbness developed below the injury, or is swelling extreme and not improving with RICE?

YES → Call MDA at 101 or go to an emergency department for further evaluation. Treat as a possible fracture.

NO → Continue RICE at home and seek medical evaluation if symptoms do not improve within 48–72 hours.

DO NOT:

X Do NOT attempt to straighten or realign a broken limb -- this may tear arteries and nerves.
X Do NOT push a protruding bone back in during an open fracture.
X Do NOT apply ice directly to skin -- always wrap in cloth first.
X Do NOT apply heat to a fresh fracture or sprain -- heat increases swelling.
X Do NOT allow the patient to walk on a suspected leg fracture.
X Do NOT remove embedded objects from a wound associated with a fracture.

Equipment needed:

- Triangular bandage or improvised sling (shirt, scarf)
- Ice pack or cooling spray (wrapped in cloth)
- Elastic bandage (for compression of sprains)
- Pillows, blankets, or rolled towels (for leg immobilisation and elevation)
- Sterile dressings or clean cloth (for open fracture wounds)
- Tourniquet (for severe bleeding in open fractures)

Personal reference only – not medical advice. Always call 101 in an emergency.

HEAD AND SPINAL INJURY FIRST AID PROTOCOL (ADULT)

Israel — Adult

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א) **Police** (100 :משטרה) **Fire** (102 :כיבוי) **Hatzalah** (1221 :הצלה)

Call 101 immediately for: any suspected head or spinal injury. Specific triggers: any loss of consciousness (even momentary), confusion or altered mental state after a blow to the head, neck or back pain after trauma, numbness/tingling/weakness in any limb, inability to move arms or legs, visible deformity of skull or spine, blood or clear fluid from nose or ears, unequal pupils, seizures after head trauma, or any mechanism suggesting significant force (road accident, fall from height, blast injury, direct blow to head/neck).

Summary: First aid for suspected head and spinal injuries in adults, emphasising manual in-line stabilisation, jaw thrust airway management, and not moving the patient unless life is in immediate danger.

When to apply: When an adult has sustained trauma to the head, neck, or back. Common scenarios in Israel include road traffic accidents, falls from height (hiking in wadis, cliffs), security incidents (blast, shrapnel), diving into shallow water, sports injuries, falls from ladders or rooftops, and any mechanism involving significant force to the head or spine.

Step 1: Ensure scene safety.

Assess for ongoing danger: traffic, fire, structural instability, active security threat. Do NOT approach if the scene is unsafe -- call 101 and wait for professional responders.

Is there immediate life-threatening danger to the patient (fire, active shooting, imminent structural collapse)?

YES → Move the patient only as a last resort using a log-roll technique keeping the spine aligned. Proceed to Step 2.

NO → Do NOT move the patient. Proceed to Step 2.

Step 2: Call MDA at 101 immediately.

Report: your precise location (GPS coordinates if in a remote area), mechanism of injury (how it happened), whether the patient is conscious and breathing, whether there is visible bleeding, and whether the patient can move their limbs. Keep the call on speaker if possible.

Step 3: Assess mechanism of injury.

Determine the force and nature of the trauma. High-risk mechanisms include: road traffic accidents, security incidents (blast injuries, shrapnel), hiking falls (wadis in Negev, Judean Desert, Golan Heights), falls from height greater than the patient's standing height, diving into shallow water, sports injuries, falls from ladders or rooftops, heavy object striking the head or back, and any axial loading mechanism.

Is this isolated penetrating trauma (stab wound or gunshot) without blunt trauma component and without neurological deficits?

YES → Spinal immobilisation is NOT required for isolated penetrating trauma. Manage wounds and bleeding. Still call 101.

NO → Assume spinal injury. Proceed to Step 4 for manual in-line stabilisation.

Step 4: Perform manual in-line stabilisation (ייצוב ידני).

This is the single most important first aid intervention for suspected spinal injury. Kneel or lie behind the patient's head. Place your hands on both sides of the patient's head, cupping the ears gently. Hold the head in a neutral position aligned with the body. Do NOT twist, flex, or extend the neck. Maintain this stabilisation continuously until professional help arrives. Talk to the patient to keep them calm and still.

WARNING: Do NOT attempt to straighten the head if it is found in a rotated position and the patient resists or reports pain -- stabilise in the position found.

Step 5: Assess airway and breathing.

Is the patient conscious and talking?

YES → The airway is open. Maintain stabilisation and proceed to Step 6.

NO → The patient is unconscious. Use the jaw thrust manoeuvre (משלט לסת) to open the airway. Do NOT use head-tilt chin-lift as this moves the cervical spine. Place your fingers behind the angles of the lower jaw on both sides and push the jaw forward without moving the neck. Proceed to check breathing.

Step 6: If the patient is unconscious, check for breathing and pulse.

Is the patient breathing and does the patient have a pulse?

YES → Maintain stabilisation and jaw thrust if needed. Proceed to Step 7.

NO → Begin CPR with spinal precautions: use jaw thrust for ventilations (ideally two responders – one maintains jaw thrust, second operates ventilation), perform chest compressions as normal. Control any severe haemorrhage before initiating CPR. Proceed to Step 7 between CPR cycles.

Step 7: Control bleeding.

If there is bleeding from the scalp, apply gentle direct pressure with a clean dressing. For associated limb bleeding, apply direct pressure or a tourniquet (CAT tourniquet) as indicated.

WARNING: Do NOT apply pressure if you suspect a skull fracture (depressed bone, visible brain matter) -- cover loosely with a sterile dressing. If blood or clear fluid is draining from the ears, do NOT plug the ears -- allow drainage and cover loosely.

Is there a suspected skull fracture (depressed bone, visible deformity, brain matter visible)?

YES → Do NOT apply direct pressure. Cover the wound loosely with a sterile dressing. Proceed to Step 8.

NO → Apply gentle direct pressure to any scalp wounds with a clean dressing. Proceed to Step 8.

Step 8: Perform neurological assessment.

Check: Can the patient feel sensations in all four limbs? Can the patient move fingers and toes? Compare strength left versus right and note any asymmetry. Check pupil size and reactivity (unequal pupils suggest raised intracranial pressure). If trained, assess Glasgow Coma Scale (GCS) and report to MDA dispatchers. GCS 13--15 = mild, GCS 9--12 = moderate, GCS 3--8 = severe (critical).

Step 9: Apply a cervical collar if available.

A second person must maintain manual stabilisation while the collar is applied. Sizing must be correct -- an improperly sized collar can cause harm.

WARNING: A collar alone is NOT sufficient. Continue to support the head even with a collar applied.

Step 10: Monitor and reassure the patient continuously.

Keep the patient still and calm -- explain what you are doing. Monitor breathing and consciousness continuously. Note any changes in neurological status (can they still move fingers/toes? are they becoming more or less alert?). Watch for signs of deterioration: progressive loss of consciousness, repeated or projectile vomiting, seizures, unequal pupils, weakness on one side. Keep the patient warm -- cover with a blanket or clothing (hypothermia worsens trauma outcomes). In desert or hot conditions, shade the patient from direct sun while maintaining immobilisation.

Step 11: Assess for concussion signs (דעדוע מוח).

Concussion is a mild traumatic brain injury. Signs include: headache (most common), feeling foggy or confused, difficulty concentrating, dizziness, nausea, sensitivity to light and noise, fatigue and drowsiness, emotional changes (irritability, sadness), sleep disturbances, balance problems. A person does NOT need to lose consciousness to have a concussion. Concussion symptoms may be delayed and appear hours after the injury.

Step 12: If the unconscious patient is vomiting, use the log-roll technique with spinal precautions.

One person maintains head and neck alignment while others roll the patient onto their side as a single unit. Continue to support the head in alignment with the body. This requires a minimum of 3 people -- the person at the head commands the roll.

Step 13: If the patient must be moved due to a life-threatening situation, use a log-roll technique.

This applies ONLY when there is immediate danger to life (fire, active threat, imminent structural collapse). Minimum 3 people required: one controlling the head, others supporting the body. The person at the head commands the roll. Roll the patient as a single unit onto their side or onto a firm surface (door, board). Maintain spinal alignment throughout.

WARNING: Moving the patient without proper technique may cause or worsen spinal cord damage, potentially resulting in permanent paralysis.

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Step 14: Do NOT remove a motorcycle or bicycle helmet unless the patient cannot breathe.

Open the face shield first for communication and airway assessment. Full helmet removal requires two trained responders -- one maintains C-spine stabilisation while the other removes the helmet. A single responder cannot safely remove a helmet while maintaining cervical spine control.

Is the patient wearing a helmet and unable to breathe?

YES → Remove the helmet with two trained responders: one maintains C-spine stabilisation while the other carefully removes the helmet.

NO → Leave the helmet in place. Open the face shield if present for communication and airway monitoring.

DO NOT:

X Do NOT move the patient unless there is immediate danger to life (fire, active threat, structural collapse).
X Do NOT twist, flex, or extend the neck.
X Do NOT attempt to sit the patient up.
X Do NOT use head-tilt chin-lift to open the airway -- use jaw thrust only.
X Do NOT remove a helmet without proper training and a second person.
X Do NOT give the patient food or drink.
X Do NOT leave the patient alone.
X Do NOT apply direct pressure to a suspected skull fracture (depressed bone, visible brain matter).
X Do NOT plug the ears or nose if blood or clear fluid is draining.
X Do NOT pull the patient by the arms or legs.
X Do NOT attempt to extract a patient from a vehicle unless the vehicle is on fire or there is another immediate life threat.
X Do NOT give oral fluids even if the patient is dehydrated (risk of vomiting and aspiration).
X Do NOT attempt to straighten the head if the patient resists or reports pain -- stabilise in the position found.

Equipment needed:

- Disposable gloves
- Clean dressings or bandages for wound care
- Sterile dressing for skull fracture coverage
- Cervical collar (if available and properly sized)
- Blanket or thermal protection
- Tourniquet (CAT) for associated limb haemorrhage
- High-visibility vest (if responding at a road accident)

Personal reference only – not medical advice. Always call 101 in an emergency.

Heart Attack / Chest Pain -- Adult

Israel – Adult

CALL 101 (MDA) – CALL IMMEDIATELY IF IN DOUBT

MDA (101 :(מד"א | **Police (100 :(משטרה** | **Fire (102 :(כיבוי** | **Hatzalah (1221 :(הצלה**

WHEN TO APPLY: An adult presents with chest pain or pressure (tightness, squeezing, heaviness, or burning in the centre of the chest) lasting more than 15 minutes, possibly with radiating pain to the arms, neck, jaw, back, or upper abdomen; shortness of breath; cold sweating; nausea; dizziness; or a feeling of impending doom. Atypical presentations (especially in women) may include extreme fatigue, upper back pain, or symptoms resembling heartburn without significant chest pain.

Equipment needed: Aspirin tablets (100 mg, 1–3 tablets) | AED (if available) | Phone (for 101/1221) | Nitroglycerin (only if prescribed to patient)

Step 1: Call 101 (MDA) immediately.

State clearly that you suspect a heart attack. Provide the location and the patient's condition. Stay on the line -- the MDA dispatcher can guide you through first aid steps. If possible, ask a bystander to simultaneously call 1221 (United Hatzalah) and to call a family member.

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Step 2: Stop all physical activity -- the patient must rest completely.

Do not allow the patient to walk, go to the bathroom, or make any physical effort. Even minor exertion can worsen damage to the heart muscle by increasing its oxygen demand.

WARNING: Any physical activity, even minor, can significantly worsen heart muscle damage.

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Step 3: Position the patient in a comfortable half-sitting position (semi-reclined) with their back supported and knees slightly bent.

The half-sitting position eases breathing and reduces the heart's workload. Alternatively, allow them to lie on their side if they prefer. Loosen tight clothing -- unbutton collar, loosen belt, remove tie. Keep the patient calm and speak reassuringly; stress increases the heart's oxygen demand. In Israel's hot climate, keep the patient cool and shaded while waiting.

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Step 4: Ask the patient whether they have a known allergy or sensitivity to aspirin.

Is the patient allergic to aspirin?

YES -> Do NOT give aspirin. Inform the arriving MDA team about the allergy. Skip to Step 6.

NO -> Proceed to Step 5 (administer aspirin).

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Step 5: Administer 100--300 mg of aspirin. The patient must CHEW the aspirin, not swallow it whole.

In Israel, standard aspirin tablets are commonly 100 mg (Micro-Aspirin). Give 1 to 3 tablets for a total of 100--300 mg. Chewing ensures faster absorption into the bloodstream. Aspirin inhibits platelet aggregation (blood clotting), helping to limit the blockage in the coronary artery and reduce damage to the heart muscle.

WARNING: Paracetamol (acetaminophen / Acamol) is NOT a substitute for aspirin. Only aspirin (acetylsalicylic acid) provides the needed antiplatelet effect.

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Step 6: Check whether the patient has a personal prescription for nitroglycerin (spray or sublingual tablets).

Does the patient have their own prescribed nitroglycerin?

YES -> Help the patient take their nitroglycerin as directed by their physician.

NO -> Do NOT administer nitroglycerin. Proceed to Step 7.

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Step 7: Monitor the patient continuously. Stay with them at all times.

Watch for changes in consciousness, breathing, and skin colour. Be prepared to escalate to CPR if the patient loses consciousness.

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Step 8: If an AED is available nearby, send someone to retrieve it.

AEDs are widely deployed in Israeli public spaces (malls, train stations, public buildings). MDA maintains a registry of AED locations. Turn the AED on -- it will provide step-by-step voice instructions in Hebrew. No prior training is required. Apply pads as shown in the diagrams on the device. Do not apply the AED unless the patient becomes unresponsive and stops breathing normally.

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Step 9: If the patient loses consciousness, check for responsiveness and normal breathing.

Tap and shout. Check for normal breathing: look for chest rise, listen for breath sounds, feel for air -- no more than 10 seconds. Gasping (agonal breathing) is NOT normal breathing.

Is the patient unconscious and not breathing normally?

YES -> Begin CPR immediately (proceed to Step 10).

NO -> Continue monitoring. Maintain the half-sitting position and keep the patient calm.

Heart Attack / Chest Pain -- Adult – continued

Israel – Adult

CPR KEY PARAMETERS

100–120	5+ cm	30 : 2
compressions/min	depth (at least)	compressions : breaths

Step 10: Begin CPR: place the heel of one hand on the centre of the chest (sternum), other hand on top. Compress hard and fast.

Compress to a depth of at least 5 cm at a rate of 100--120 compressions per minute. Allow full chest recoil between compressions. If trained and willing, give 30 compressions followed by 2 rescue breaths. If untrained or unwilling, perform hands-only CPR (continuous compressions without breaths).

WARNING: Do not stop CPR until MDA arrives and takes over, an AED is applied, or the patient regains consciousness and starts breathing normally.

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Step 11: If an AED is available and the patient is in cardiac arrest, apply it immediately and follow voice prompts.

Apply pads as shown on the device. Ensure no one is touching the patient during analysis and shock delivery. Resume CPR immediately after shock delivery or if no shock is advised.

Does the AED advise a shock?

YES -> Ensure everyone is clear of the patient, press the shock button, then resume CPR immediately.

NO -> Resume CPR immediately. The AED will re-analyse after approximately 2 minutes.

DO NOT:

X Do not allow the patient to walk, exert themselves, or dismiss their symptoms.
X Do not give the patient food or beverages.
X Do not leave the patient alone (except briefly to call for help).
X Do not give aspirin if the patient is allergic to it.
X Do not give nitroglycerin that was not prescribed to this specific patient.
X Do not wait to see if symptoms resolve on their own -- every minute of delay increases irreversible heart muscle damage.
X Do not drive the patient to hospital yourself -- call 101 and wait for the ambulance. MDA has cardiac monitoring equipment and can begin treatment en route.
X Do not substitute paracetamol (Acamol) for aspirin -- only aspirin provides the needed antiplatelet effect.
X Do not give aspirin as a whole tablet -- it must be chewed for rapid absorption.

Personal reference only – not medical advice. Always call 101 in an emergency.

HEATSTROKE & HEAT EXHAUSTION

Israel — Adult

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

When to use this protocol: Person showing signs of heat-related illness after prolonged heat exposure or physical exertion in hot conditions. Signs include heavy sweating, dizziness, nausea, confusion, hot dry skin, or loss of consciousness.

Step 1: Assess the person's condition to determine whether this is heat exhaustion or heat stroke.

Heat exhaustion: heavy sweating, warm moist skin, dizziness, nausea, headache, weakness, muscle cramps, rapid pulse. Heat stroke: body temperature above 40 C, hot dry skin (sweating may have stopped), confusion, disorientation, staggering gait, seizures, hallucinations, loss of consciousness.

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Does the person show signs of HEAT STROKE?
(confusion, hot dry skin, temperature above 40 C, seizures, loss of consciousness)

YES -> Life-threatening emergency. Go to HEAT STROKE pathway below.

NO -> Treat as heat exhaustion. Go to HEAT EXHAUSTION pathway below.

HEAT STROKE PATHWAY — LIFE-THREATENING EMERGENCY

Step 2: HEAT STROKE: Call MDA immediately at 101.

Heat stroke is a life-threatening emergency. Do not delay calling emergency services.

WARNING: Do not delay calling 101 for suspected heat stroke.

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Step 3: Move the person to shade or an air-conditioned area immediately.

Step 4: Remove unnecessary clothing.

Step 5: Begin aggressive cooling: apply wet towels to the body (especially neck, armpits, groin), spray or pour cool/cold water on the body, fan the person vigorously, and apply cold packs to neck, armpits, and groin if available.

Target the neck, armpits, and groin where major blood vessels are close to the skin surface for maximum cooling effect.

Step 6: Assess whether the person is fully conscious and able to swallow.

WARNING: Do not give fluids to a person who is not fully conscious -- aspiration risk.

Is the person fully conscious and able to swallow?

YES -> Offer small sips of cool water continuously.

NO -> Do NOT give any fluids. Place in recovery position if breathing. Monitor airway.

Step 7: Monitor the person continuously until MDA arrives.

Elevate legs if blood pressure drops (signs of shock). If the person becomes unresponsive and stops breathing, begin CPR.

Has the person become unresponsive and stopped breathing?

YES -> Begin CPR immediately.

NO -> Continue cooling and monitoring. Stay with the patient until ambulance arrives.

HEAT EXHAUSTION PATHWAY

Step 8: HEAT EXHAUSTION: Move the person to a shaded or air-conditioned location.

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Step 9: Have the person lie down. Elevate legs slightly if blood pressure appears low.

|
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Step 10: Remove excess or tight clothing.

|
v

Step 11: Begin cooling: apply wet towels, spray with cool water, fan the person.

|
v

Step 12: Assess whether the person is fully conscious and able to swallow.

Is the person fully conscious and able to swallow?

YES -> Offer small sips of cool water continuously.

NO -> Do NOT give fluids. Call MDA at 101 immediately.

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Step 13: Monitor for deterioration.

Are symptoms improving with first aid?

YES -> Continue cooling and hydration. Keep monitoring.

NO -> Call MDA at 101. Condition may be progressing to heat stroke. Return to Step 1 and reassess.

DO NOT:

X Do not administer fever-reducing medications (paracetamol, ibuprofen) -- antipyretics do not work for environmental hyperthermia and may cause harm.
X Do not give fluids to a person who is not fully conscious or who cannot swallow safely.
X Do not delay calling 101 for suspected heat stroke.
X Do not ignore early signs of heat exhaustion -- it can progress to life-threatening heat stroke.
X Do not leave the person unattended while waiting for emergency services.

Equipment needed:

- Wet towels or cloths
- Cool/cold water (for spraying, pouring, and drinking)
- Cold packs or ice packs (if available)
- Fan or other means of air circulation (if available)

Personal reference only – not medical advice. Always call 101 in an emergency.

HYPOTHERMIA / COLD EXPOSURE

Adult Protocol

Israel — Adult

CALL 101 (MDA (מד"א)) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :(מד"א **Police (100 :(משטרה)** **Fire (102 :(כיבוי)** **Hatzalah (1221 :(הצלה)**

CALL 101 IF: Immediately upon recognising hypothermia – call MDA at 101 as the first step.

SUMMARY: First aid for hypothermia (body temperature below 35 degrees C) through gradual rewarming, starting with the torso. Signs: shivering, confusion, pale/cold skin, fatigue, slurred speech, muscle rigidity, or loss of consciousness after cold exposure.

Step 1: Call MDA immediately -- dial 101.

Hypothermia can deteriorate rapidly. Request ambulance assistance immediately.

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Step 2: Move the person to a warm, dry shelter.

Protect from wind, rain, and further cold exposure. If outdoors, shield from the elements as best as possible.

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Step 3: Remove wet clothing promptly and replace with dry layers.

Wet clothing accelerates heat loss. Replace with dry, lightweight layered clothing that allows some movement, as movement generates heat.

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Step 4: Cover the person with blankets and layers, including the head.

Significant heat is lost through the head. Wrap thoroughly in blankets.

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Step 5: Warm the room to approximately 22 degrees C (72 degrees F) if indoors.

A warm ambient environment supports gradual rewarming.

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Step 6: Assess whether the person is conscious.

Is the person conscious and able to swallow?

YES → Provide warm (NOT hot) beverages. Avoid alcohol and caffeine.

NO → Do not give any fluids. Monitor breathing closely and be prepared to begin CPR.

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Step 7: Apply heat packs to the chest, abdomen, and back ONLY.

Use a cloth barrier between heat packs and skin -- never place directly on bare skin. Warm the torso FIRST, not the hands and feet.

WARNING: Warming extremities first can cause cold blood to rush to the heart, potentially triggering cardiac arrest (afterdrop phenomenon).

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Step 8: Monitor breathing continuously.

Has the person stopped breathing?

YES → Begin CPR immediately.

NO → Continue monitoring and gradual rewarming until MDA arrives.

Step 9: Continue gradual rewarming until emergency services arrive.

Rewarming must be gradual. Rapid rewarming can cause dangerous cardiac arrhythmias.

DO NOT:

X Do NOT rub extremities -- friction can cause tissue damage and cardiac complications.
X Do NOT use direct intense heat (hot water baths, heating pads directly on skin) -- rapid rewarming can cause dangerous cardiac arrhythmias.
X Do NOT give alcohol -- it dilates blood vessels and accelerates heat loss.
X Do NOT give caffeine -- it worsens hypothermia.
X Do NOT warm extremities first -- cold blood rushing to the heart can trigger cardiac arrest (afterdrop).
X Do NOT apply vigorous external rewarming.

Equipment needed:

— Blankets or thermal blankets (space blankets)
— Dry clothing
— Heat packs with cloth barrier
— Warm (not hot) beverages (for conscious patients only)

Personal reference only – not medical advice. Always call 101 in an emergency.

Poisoning — Adult Protocol

Israel — Adult

ID: IL-ADULT-POISONING-001

CALL 101 (MDA (מד"א)) — CALL IMMEDIATELY IF IN DOUBT

MDA (מד"א): 101 | **Police (משטרה):** 100 | **Fire (כיבוי):** 102 | **Hatzalah (הצלה):** 1221

Israel Poison Information Center (24/7): 04-7771900 — Rambam Health Care Campus, Haifa

When to apply:

Suspected poisoning through any route: ingestion of toxic substances (medications, cleaning products, chemicals, plants), inhalation of toxic fumes or gases (carbon monoxide, chlorine gas, chemical fumes), skin or eye contact with hazardous chemicals, or injection of toxins (venomous bites/stings, IV drug use). Signs include: nausea, vomiting, abdominal pain, confusion, drowsiness, breathing difficulty, seizures, chemical burns around mouth or skin, unusual smell on breath, or loss of consciousness.

Summary:

Scene safety assessment, call for help, identify route of exposure, provide route-specific first aid, preserve the substance, and monitor the patient until emergency services arrive.

Step 1: Ensure scene safety before approaching the patient.

Assess the environment for hazards. Protect yourself from contamination. For suspected inhalation poisoning, do not enter a confined space with potential toxic fumes without proper protection -- wait for trained responders with protective equipment.

WARNING: Do NOT enter areas with potential toxic gas exposure. You cannot help the patient if you become a casualty.

Step 2: Call 101 (MDA) immediately. Call the Israel Poison Information Center at 04-7771900 for substance-specific guidance.

Be prepared to provide: the substance involved, estimated amount, time of exposure, patient's age and weight, current symptoms, and the route of exposure (ingestion, inhalation, skin contact, or injection).

Step 3: Preserve the substance container, packaging, or remaining material.

Collect and keep all containers, packaging, pills, or remaining substance. Bring them to the hospital or have them available for emergency responders. This is critical for identification and appropriate treatment. If the patient has vomited, preserve a sample for analysis if possible.

Step 4: Determine the route of exposure.

Was the poison ingested (swallowed)?

YES → Proceed to Step 5 for ingestion-specific treatment.

NO → Proceed to Step 7 to check for inhalation exposure.

Step 5: For ingestion poisoning: determine if a caustic or corrosive substance was swallowed.

WARNING: Do NOT induce vomiting. Vomiting is often ineffective, caustic substances cause additional burns on the way back up, and there is a risk of aspiration.

Was a caustic or corrosive substance ingested (acids, bases, strong detergents)?

YES → NEVER induce vomiting. If the patient is conscious and breathing normally, give approximately 1 cup (250 ml) of water or milk to dilute the substance. Seek immediate medical care. Proceed to Step 10.

NO → Do NOT induce vomiting unless specifically directed by the Poison Information Center or a physician. If poison remains in the mouth, attempt careful removal. Do not give food or drink unless directed by the Poison Information Center. Proceed to Step 10.

Step 6: For ingestion: do NOT give activated charcoal unless directed by medical professionals.

Activated charcoal is effective only within 1-2 hours of exposure and only for certain substances. This is a medical-level intervention, not a first aid action.

Step 7: Check for inhalation exposure.

WARNING: Never mix bleach (Economica) with other cleaning products, especially acidic substances -- this releases toxic chlorine gas.

Was the poison inhaled (toxic fumes, gases, carbon monoxide)?

YES → Move the patient to fresh air immediately – remove from the contaminated area. Do NOT enter the area yourself if there is a risk of toxic gas exposure. Call 101. Proceed to Step 9.

NO → Proceed to Step 8 to check for skin or eye contact.

Step 8: Check for skin or eye contact exposure.

Did the poison contact the skin or eyes?

YES → Remove contaminated clothing immediately (protect yourself while doing so). For skin contact: flush the affected area with large amounts of cool running water for at least 15-20 minutes. For eye contact: rinse and flush eyes thoroughly with running water for at least 15 minutes, holding eyelids open. Do NOT attempt to chemically neutralise the substance. Proceed to Step 10.

NO → The exposure may be through injection (bites, stings, IV drug use). Proceed to Step 10 for general monitoring.

Step 9: For inhalation exposure: assess the patient's breathing after moving to fresh air.

Is the patient breathing?

YES → Monitor breathing and vital signs continuously. Keep the patient calm and warm. Proceed to Step 10.

NO → Begin CPR immediately. Continue until emergency services arrive.

Step 10: Assess the patient's level of consciousness.

Is the patient conscious?

YES → Keep the patient calm and warm. Monitor breathing, pulse, and level of consciousness continuously. Do not leave the patient alone. Wait for emergency services.

NO → Proceed to Step 11.

Step 11: For an unconscious patient: check for breathing.

Is the unconscious patient breathing?

YES → Place in the recovery position (lateral/ side-lying position) to prevent aspiration. Monitor breathing continuously. Do not leave the patient alone.

NO → Begin CPR immediately. Continue until emergency services arrive.

Step 12: Recovery position for unconscious but breathing patients.

Kneel beside the patient. Place the arm nearest to you at a right angle to the body. Bring the far arm across the chest and hold the back of the hand against the near cheek. Pull the far knee up and roll the patient towards you onto their side. Adjust the top leg so the knee is at a right angle. Tilt the head back slightly to keep the airway open. Monitor breathing continuously.

Step 13: Continue monitoring until emergency services arrive.

Monitor breathing and pulse continuously. Be prepared to perform CPR if breathing or heartbeat stops. Keep the patient warm and calm. Do not leave the patient alone. Provide all collected substance information to arriving medical team.

DO NOT:

X Do NOT induce vomiting unless specifically directed by the Poison Information Center (04-7771900) or a physician.
X Do NOT give food or drink unless directed by medical professionals (exception: water or milk for conscious patients who ingested caustic substances, as directed).
X Do NOT attempt to neutralise the poison (e.g., giving acid for base ingestion or vice versa).
X Do NOT use home remedies (salt water, mustard, raw eggs, etc.).
X Do NOT give activated charcoal without medical direction.
X Do NOT enter a confined space with potential toxic fumes without proper protection.
X Do NOT delay calling for help -- even if symptoms appear mild, they can deteriorate rapidly.
X Do NOT administer antidotes -- this is a hospital-level intervention only.
X Do NOT leave the patient alone.

Equipment needed:

— Gloves (to protect against contamination)
— Running water source (for skin or eye decontamination)

Source: Magen David Adom (MDA) — “Poisoning First Aid Guidance (הרעלות)” · MDA 101 Public First Aid Guidance Series; corroborated by Rambam National Poison Information Center guidance
Imported: 2026-03-15 · Last verified: 2026-03-15
Personal reference only — not medical advice. Always call 101 in an emergency.

RECOVERY POSITION / UNCONSCIOUS CASUALTY

Adult Protocol

Israel — Adult

CALL 101 (MDA (מד"א)) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א) **Police (100 :משטרה)** **Fire (102 :כיבוי)** **Hatzalah (1221 :הצלה)**

CALL 101 IF: As soon as the person is confirmed unresponsive but breathing – call MDA at 101 before or immediately after placing in recovery position.

SUMMARY: Place an unconscious but breathing person in the recovery position to maintain an open airway and prevent aspiration. Use when the person is unresponsive but breathing, with no suspected spinal injury and no need for CPR.

Step 1: Check responsiveness: call out "Are you OK?" (Ata beseder?) and tap or gently shake the shoulders.

Pinch the shoulders if needed to assess response.

Does the person respond?

YES → The person is conscious. Assess their condition and provide appropriate first aid. Do not place in recovery position.

NO → The person is unresponsive. Proceed to Step 2.

↓

Step 2: Check the airway: open the person's mouth and look inside. Tilt the body to the side and use a gripping motion to clear any visible obstructions (food, dentures, vomit).

↓

Step 3: Check breathing: look for chest movement, listen for breathing sounds, and feel for breath on your cheek. Take no more than 10 seconds.

Is the person breathing?

YES → Proceed to Step 4 – prepare to place in recovery position.

NO → Begin CPR immediately. Do NOT place in recovery position.

↓

Step 4: Call MDA at 101. Report an unconscious but breathing casualty.

Is there a suspected spinal injury (fall from height, road accident, blow to head/neck, diving accident, or person complains of neck/back pain)?

YES → Do NOT move the person unless they are in immediate danger (fire, drowning). Support the head and neck in line with the body if movement is absolutely necessary.

NO → Proceed to Step 5 to place in recovery position.

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PLACING IN RECOVERY POSITION:

Step 5: Kneel beside the person who is lying on their back.

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Step 6: Extend the arm nearest to you at a right angle to the body, palm facing up.

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Step 7: Take the far arm and fold it across the chest so the back of the hand rests against the cheek closest to you. Hold it in place.

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Step 8: Bend the far knee up to a right angle using your free hand.

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Step 9: Roll the person toward you by pulling on the bent knee. They should roll smoothly onto their side.

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Step 10: Adjust the top leg so both the hip and knee are bent at right angles, stabilising the position.

↓

Step 11: Tilt the head back slightly to keep the airway open. Adjust the hand under the cheek to maintain the head tilt.

The person should now be lying on their side with the higher shoulder angled toward the ground, knees bent for stability, and airway open to allow drainage.

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Step 12: Monitor the person continuously until MDA arrives.

Check breathing regularly. Monitor for vomiting -- the position should allow natural drainage, but clear the mouth if needed. Keep the person warm with a blanket.

Has the person stopped breathing?

YES → Roll them onto their back and begin CPR immediately.

NO → Continue monitoring. If in recovery position for more than 30 minutes, turn to the opposite side to prevent pressure injuries.

DO NOT:

X Do NOT place in recovery position if the person is not breathing -- begin CPR instead.
X Do NOT move a person with suspected spinal injury unless they are in immediate danger.
X Do NOT leave the person unattended -- monitor breathing continuously.
X Do NOT place the person on their back if they are unconscious and breathing -- risk of airway obstruction and aspiration.
X Do NOT forget to call 101 -- an unconscious person always requires professional medical evaluation.

Equipment needed:

— Blanket (to keep the person warm)
— Phone to call MDA at 101

Source: Magen David Adom (MDA) – MDA Emergency Medicine Manual 2024; MDA BLS Guide 2016
Protocol ID: IL-ADULT-RECOVERY-POSITION-001 · Imported: 2026-03-16 · Last verified: 2026-03-16
Personal reference only – not medical advice. Always call 101 in an emergency.

Scorpion Stings — Adult Protocol

Israel — Adult

ID: IL-ADULT-SCORPION-STINGS-001

CALL 101 (MDA (מד"א)) — CALL IMMEDIATELY IF IN DOUBT

MDA (מד"א): 101 | **Police (משטרה):** 100 | **Fire (כיבוי):** 102 | **Hatzalah (הצלה):** 1221

Israel Poison Information Center (24/7): 04-7771900 — Rambam Health Care Campus, Haifa

When to apply:

Person has been stung or suspected stung by a scorpion. Signs include: immediate intense pain at the sting site, mild to moderate swelling and redness, numbness or tingling at the sting site. May progress to systemic symptoms including restlessness, altered consciousness, seizures, difficulty breathing, excessive sweating, tachycardia, hypertension, nausea, or vomiting. Local symptom severity does NOT reliably predict systemic danger.

Summary:

Call 101 immediately, keep the victim calm and still, immobilise the affected limb, remove constrictive items, monitor for systemic reaction, and transport to hospital for mandatory observation (minimum 6 hours).

Step 1: Call MDA immediately at 101.

Request an ambulance. Follow dispatch instructions until the medical team arrives. All scorpion stings require hospital evaluation -- do not wait to see if symptoms develop.

Step 2: Keep the victim calm and still.

Reassure the victim and reduce stress and anxiety. Have the victim lie down. Minimise all movement -- movement and elevated heart rate accelerate venom spread through the bloodstream. Panic and agitation worsen the clinical outcome.

Step 3: Immobilise the affected limb.

Keep the stung limb as still as possible. Position the limb at or below heart level to slow venom distribution.

Step 4: Remove constrictive items from the sting area.

Remove jewellery, watches, rings, and tight clothing near the sting site. The area may swell significantly.

WARNING: Swelling can progress rapidly. Remove all constrictive items as early as possible.

Step 5: Assess for signs of systemic envenomation.

Watch for: restlessness, altered consciousness, confusion, difficulty breathing or swallowing, excessive sweating, salivation, or tearing, rapid or irregular heartbeat, seizures or muscle twitching, nausea, vomiting, abdominal pain, changes in blood pressure.

Is the victim showing any systemic symptoms beyond local pain (altered consciousness, difficulty breathing, seizures, cardiovascular changes)?

YES → Report symptoms immediately to MDA dispatch. This is a severe envenomation requiring urgent hospital transport. Maintain airway (ABC). If the victim stops breathing, begin CPR. Provide oxygen if available and trained to do so.

NO → Continue monitoring while keeping the victim calm and still. All scorpion sting victims still require hospital transport and observation.

Step 6: Attempt to identify the scorpion if it can be done safely.

If possible, photograph the scorpion or note its colour and size. The deathstalker (Leiurus quinquestriatus) is yellow and is the most dangerous species in Israel. However, the common belief that black scorpions are not dangerous is FALSE -- several black species are medically significant.

Can the scorpion be safely identified or photographed without risk of additional stings?

YES → Take a photograph or note the scorpion's colour, size, and any distinguishing features. Share this information with arriving paramedics.

NO → Do not approach the scorpion. Report any details you observed to paramedics when they arrive.

Step 7: Maintain ABC (Airway, Breathing, Circulation) support while awaiting paramedics.

Continuously monitor the victim's airway, breathing, and circulation. Be prepared to intervene if the victim's condition deteriorates.

Is the victim conscious and breathing adequately?

YES → Continue monitoring. Keep the victim lying down with the stung limb immobilised at or below heart level.

NO → If unconscious but breathing, place in recovery position. If not breathing, begin CPR immediately. Report status change to MDA dispatch.

Step 8: Wait for MDA paramedics and prepare for hospital transport.

All scorpion sting victims must be observed in the emergency department for a minimum of 6 hours from the time of sting (Israel Poison Information Center guideline). Scorpion antivenom is available in Israeli hospitals and may be administered in rare, severe cases based on clinical presentation.

Is the victim in a remote location where MDA response will be significantly delayed?

YES → If safe transport is available, begin moving the victim to the nearest hospital while keeping them immobile and the stung limb at or below heart level. Maintain phone contact with MDA dispatch for guidance.

NO → Wait for MDA paramedics to arrive. Do not move the victim unnecessarily.

DO NOT:

X DO NOT apply a tourniquet -- it traps venom locally and causes tissue damage; it does not prevent systemic spread.
X DO NOT suck the venom out of the wound -- this is ineffective and potentially harmful.
X DO NOT cut the sting area -- this increases infection risk and does not remove venom.
X DO NOT apply ice or cool the sting area -- the Israeli Ministry of Health (2026) explicitly advises against cooling. If MDA dispatch specifically instructs cooling, use indirect application only (ice wrapped in cloth) and keep it brief.
X DO NOT give the victim food or drink -- this risks venom dispersal and may complicate hospital treatment.
X DO NOT allow the victim to walk or move the stung limb -- movement accelerates venom circulation.

Equipment needed:

— Phone to call 101 (MDA) and optionally 04-7771900 (Poison Center)
— Phone or camera to photograph the scorpion if safe to do so

Personal reference only — not medical advice. Always call 101 in an emergency.

SEIZURES (EPILEPTIC SEIZURE / CONVULSIONS)

Adult Protocol

Israel — Adult

CALL 101 (MDA (מד"א)) — CALL IMMEDIATELY IF IN DOUBT

MDA (מד"א): 101	**Police (משטרה):** 100	**Fire (כיבוי):** 102	**Hatzalah (הצלה):** 1221

CALL 101 IF: Seizure lasts >5 min – second seizure without regaining consciousness – first-time seizure – difficulty breathing after seizure – person is pregnant or diabetic – injury during seizure.

SUMMARY: Protect the person from injury during the seizure, time the seizure, place in recovery position after it ends, and call 101 if the seizure lasts more than 5 minutes or meets other emergency criteria.

Step 1: Stay calm and note the time the seizure started.

Most seizures end on their own within 1--3 minutes. Timing is critical for determining whether the seizure qualifies as status epilepticus (>5 minutes) and whether emergency services are needed.

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Step 2: Clear the area around the person to prevent injury.

Move sharp objects, furniture, hard items, or anything that could cause injury during convulsions. If the person is standing or seated, gently guide them to the ground.

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Step 3: Place something soft and flat under the person's head.

Use a folded jacket, towel, cushion, or blanket to prevent head injury from repeated impact with the ground.

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Step 4: Loosen tight clothing around the neck.

Remove or loosen ties, scarves, collars, necklaces, and belts to aid breathing.

↓

Step 5: Assess whether the person needs to be turned onto their side during the seizure.

If the person is vomiting or producing excessive saliva, turn them onto their side (recovery position) with the mouth pointing downward to prevent aspiration. Tilt the head slightly to allow fluids to drain.

Is the person vomiting or producing excessive saliva during the seizure?

YES → Turn the person onto their side (recovery position) with the mouth pointing downward to prevent aspiration.

NO → Allow the seizure to run its course. Do not move the person unless they are in immediate danger.

↓

Step 6: Stay with the person throughout the seizure and monitor the duration.

Remain with the person and do not leave them alone. There is a risk of additional seizures.

Has the seizure lasted more than 5 minutes?

YES → Call 101 (MDA) immediately. This is status epilepticus – a medical emergency that can cause irreversible brain damage.

NO → Continue monitoring. Most seizures end within 1–3 minutes.

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Step 7: After the seizure ends, place the person in the recovery position (on their side) if not already done.

This keeps the airway clear and allows any fluids to drain from the mouth.

↓

Step 8: Check that the person is breathing normally.

Is the person breathing normally after the seizure?

YES → Continue to monitor. Stay with the person until they are fully alert and oriented.

NO → Call 101 (MDA) immediately. If the person stops breathing, begin CPR.

↓

Step 9: Speak calmly and reassuringly as the person regains consciousness.

The person will likely be confused and disoriented (postictal state) and will have no memory of the seizure. Tell them calmly what happened.

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Step 10: Assess whether this is a first-time seizure or differs from the person's known pattern.

Is this the person's first seizure, or is it different from their typical seizure pattern (longer, more intense, or different type)?

YES → Call 101 (MDA). A first-time seizure always requires medical evaluation. An unusual seizure in a known epilepsy patient also requires evaluation.

NO → If the person has known epilepsy and this was a typical seizure that resolved within 5 minutes, stay with them until fully recovered. Ensure they have someone to accompany them.

↓

Step 11: Do not offer food, water, or medication by mouth until the person is fully conscious and able to swallow safely.

Wait until you are certain the person is fully alert before offering anything by mouth to avoid choking risk.

WARNING: Offering food or drink before the person is fully conscious can cause choking or aspiration.

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Step 12: If the person has a prescribed rescue medication and a trained caregiver is present, administer it according to the neurologist's instructions.

Rescue medications include Midazolam (Buccolam) given buccally, Diazepam (Stesolid) given rectally, or Clonazepam (Rivotril) given as oral drops. These are given if the seizure lasts more than 3 minutes. Only administer if a valid prescription exists and the caregiver has been trained.

WARNING: Rescue medications are prescription-only. Only administer at the dose prescribed by the treating neurologist. If the medication is given and the seizure does not stop, call 101 immediately.

Does the person have a prescribed rescue medication available, and is a trained caregiver present?

YES → Administer the rescue medication at the prescribed dose if the seizure has lasted more than 3 minutes. If the seizure does not stop after medication, call 101.

NO → Do not administer any medication. Call 101 if the seizure lasts more than 5 minutes.

DO NOT:

X DO NOT restrain the person or try to hold them down. You cannot stop a seizure by force, and restraining can cause fractures, dislocations, or soft tissue injury.
X DO NOT put anything in the person's mouth -- no spoons, wallets, sticks, fingers, or any other object. The person cannot swallow their tongue. Objects in the mouth risk breaking teeth, injuring the jaw, or causing bite injuries.
X DO NOT attempt mouth-to-mouth resuscitation during the seizure. People usually resume breathing on their own once the seizure ends.
X DO NOT pour water on the person or attempt to 'wake them up'.
X DO NOT offer food, drinks, or medication by mouth during or immediately after the seizure until the person is fully alert and able to swallow safely.
X DO NOT crowd around the person -- give them space and privacy. Ask bystanders to step back.
X DO NOT move the person unless they are in immediate danger (e.g., in the middle of a road, near a ledge, next to fire or water).

Equipment needed:

— Soft padding for under the head (jacket, towel, cushion, blanket)
— Watch or phone to time the seizure duration
— Prescribed rescue medication if available (Buccolam, Stesolid, or Rivotril -- prescription only)

Source: Magen David Adom (MDA) – MDA First Aid Provider Course 2026; MDA ALS Professional Manual April 2024; corroborated by Israeli Epilepsy Association
Protocol ID: IL-ADULT-SEIZURES-001 · Imported: 2026-03-15 · Last verified: 2026-03-15
Personal reference only – not medical advice. Always call 101 in an emergency.

SEVERE BLEEDING (HEMORRHAGE CONTROL)

Israel – Adult

CALL 101 (MDA) – CALL IMMEDIATELY IF IN DOUBT

WHEN TO APPLY: Severe or uncontrolled external bleeding from any cause including trauma, amputation, crush injury, or penetrating injury. Signs include rapid spurting or steady flow of blood, pooling blood, blood-soaked clothing, or a severed or partially severed limb.

Step 1: Ensure scene safety and wear disposable gloves if available.

Do not approach the casualty until the scene is safe. Gloves are standard in Israeli first aid kits.

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Step 2: Call 101 (Magen David Adom) immediately.

Provide location, number of casualties, and nature of injuries. If alone and bleeding is immediately life-threatening, begin hemorrhage control first and call as soon as possible.

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Step 3: Check for embedded or impaled objects in the wound.

Is there an embedded or impaled object (knife, shrapnel, glass, etc.) in the wound?

YES -> Do NOT remove the object. Stabilise it in place with padding and bandages around it. Apply pressure around the object, not on it. Skip to Step 11 (monitor for shock).

NO -> Proceed to Step 4 (direct pressure).

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Step 4: Apply strong, direct pressure to the source of bleeding with your gloved hand or sterile gauze.

Press firmly directly on the wound. Use your bare or gloved hand initially, then replace with a sterile bandage, towel, or clean cloth as soon as available. Maintain continuous, firm pressure without lifting to check -- this allows clot formation.

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Step 5: If the first dressing soaks through, add additional layers on top without removing the original dressing.

Use a triangular bandage, sheet, or clean towel if additional material is needed. Never remove a blood-soaked dressing -- only add on top.

Has direct pressure controlled the bleeding?

YES -> Maintain pressure. Elevate the limb above heart level if possible (Step 6). Continue to Step 11 (monitor for shock).

NO -> Proceed to Step 7 (apply Israeli bandage / pressure dressing).

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Step 6: If possible, elevate the injured limb above heart level.

Elevation is an adjunct to direct pressure, not a replacement. It reduces blood flow to the injured area.

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Step 7: Apply an Israeli Emergency Bandage (pressure dressing).

1) Apply the white sterile pad directly over the wound. 2) Wrap the elastic bandage around the injured area, passing through the pressure bar. 3) Reverse direction and continue wrapping to apply circumferential pressure. 4) Secure using the built-in closure bar.

WARNING: The elastic component can occlude arteries. Verify distal pulse after application. The bandage should be tight enough to control bleeding but not so tight as to act as a tourniquet (unless that is the intent).

Has the Israeli bandage controlled the bleeding?

YES -> Maintain the dressing in place. Continue to Step 11 (monitor for shock).

NO -> Assess wound location. If limb, go to Step 8b (tourniquet). If junctional area (groin, neck, axilla, shoulder), go to Step 8a (wound packing).

SEVERE BLEEDING (HEMORRHAGE CONTROL) (continued)

Israel – Adult

CALL 101 (MDA) – CALL IMMEDIATELY IF IN DOUBT

Step 8: Escalate hemorrhage control: wound packing (junctional areas) or tourniquet (limbs).

Choose the appropriate method based on wound location.

Is the wound on a limb (arm or leg)?

YES -> Apply a tourniquet (Step 10).

NO -> Apply wound packing for junctional wounds – groin, neck, axilla, shoulder (Step 9).

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Step 9 (Wound Packing): Wound packing for junctional areas (groin, neck, axilla, shoulder).

1) Insert specialised hemostatic or plain gauze finger-by-finger into the wound. 2) Fill internally in a sunburst pattern to create internal pressure against the bleeding vessel. 3) Apply the attached dressing with direct pressure for 1-2 minutes. 4) Tighten bandage straps to maintain pressure.

WARNING: Do NOT use wound packing for penetrating stab wounds. Do NOT enlarge wound openings to facilitate packing.

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Step 10 (Tourniquet): Apply a tourniquet for uncontrolled limb bleeding.

1) Place the tourniquet 5-10 cm above the source of bleeding (proximal, toward the heart). 2) Tighten the windlass/mechanism until bleeding completely stops. 3) Secure the windlass in place. 4) Record the time of application -- write on the casualty's forehead or another visible location: 'TQ' followed by the time in 4-digit format (e.g., 'TQ 1430'). 5) Leave approximately 15 cm of strap, which can be folded in an R-shape for visibility.

WARNING: A tourniquet completely blocks blood flow distal to the application point, risking tissue death if left too long. A properly applied tourniquet will cause significant pain -- this is expected. Do NOT remove or loosen a tourniquet once applied unless instructed by a paramedic or physician.

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Step 11: Monitor the casualty for signs of hypovolemic shock.

Watch for: rapid pulse, pale/cool/clammy skin, confusion, rapid breathing. Keep the casualty warm and lying down. Reassure the casualty and wait for MDA responders.

Is the casualty showing signs of shock (rapid pulse, pale/cool/clammy skin, confusion, rapid breathing)?

YES -> Keep the casualty lying down with legs elevated if no spinal injury is suspected. Keep them warm with a blanket or coat. Do not give food or drink. Urgently update the 101 dispatcher on the casualty's deterioration.

NO -> Continue monitoring. Maintain hemorrhage control measures. Keep the casualty warm and reassured until MDA arrives.

DO NOT:

X Do not remove an embedded or impaled object from the wound.
X Do not lift or remove a blood-soaked dressing to check the wound -- add layers on top.
X Do not remove or loosen a tourniquet once applied unless instructed by a paramedic or physician.
X Do not give food or drink to a casualty with severe bleeding or suspected shock.
X Do not use wound packing for penetrating stab wounds.
X Do not enlarge wound openings to facilitate wound packing.
X Do not assume a rapid pulse is caused only by anxiety or pain -- suspect significant blood loss.
X Do not delay calling 101 for severe or uncontrolled bleeding.
X Do not apply a tourniquet directly over a joint -- place it 5-10 cm above the wound on the limb.

EQUIPMENT NEEDED:

- Disposable gloves
- Sterile gauze or clean cloth
- Israeli Emergency Bandage (pressure dressing)
- Tourniquet (CAT or equivalent)
- Wound packing material (hemostatic gauze preferred)
- Triangular bandage (supplementary)
- Blanket or coat (for shock management)

Source: Magen David Adom (MDA) (2026-03-15) · Last verified: 2026-03-15 · ID: IL-ADULT-SEVERE-BLEEDING-001
Personal reference only – not medical advice. Always call 101 in an emergency.

SHOCK (HELEM)

Israel – Adult

CALL 101 (MDA) – CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א) **Police (100 :משטרה)** **Fire (102 :כיבוי)** **Hatzalah (1221 :הצלה)**

WHEN TO APPLY: Patient presents with signs of shock: rapid weak pulse, pale/cold/clammy skin, cold perspiration, rapid shallow breathing, decreased consciousness or confusion, falling blood pressure, weakness, dizziness, fainting, nausea, vomiting, or thirst. May result from massive blood loss (hypovolemic), spinal cord injury (neurogenic), heart failure (cardiogenic), severe infection (septic), or severe allergic reaction (anaphylactic).

SUMMARY: Identify shock type, call 101, perform ABC assessment, control bleeding, elevate legs approximately 30 cm, maintain body temperature, give nothing by mouth, and monitor until MDA arrives.

Step 1: Call 101 (MDA) immediately.

Provide the dispatcher with your location, number of casualties, nature of the incident, and the patient's condition. Keep the line open if possible.

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Step 2: Ensure the scene is safe.

Confirm the scene is safe for you and the patient before approaching. In security incidents, be aware of secondary threats.

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Step 3: Perform primary assessment (ABC).

A - Airway: ensure the airway is open and clear; tilt chin up slightly if needed. B - Breathing: check if the person is breathing effectively. C - Circulation: check for pulse and identify any severe bleeding.

Is the patient breathing?

YES -> Continue to Step 4 to assess and treat the cause of shock.

NO -> Begin CPR immediately. Follow the CPR protocol (IL-ADULT-CPR-001).

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Step 4: Identify the likely type of shock if possible.

Hypovolemic: blood loss, dehydration, burns (rapid pulse, pale/cold skin). Neurogenic: spinal injury (normal or slow pulse, warm red skin below injury). Cardiogenic: heart attack, cardiac tamponade (elevated jugular venous pressure, rapid or slow pulse). Septic: infection with high fever (gradual onset). Anaphylactic: allergic reaction with rash, swelling, wheeze, stridor.

Is the shock caused by visible bleeding?

YES -> Proceed to Step 5 for bleeding control before positioning.

NO -> Skip to Step 6 for patient positioning.

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Step 5: Control any severe bleeding.

Apply direct pressure to wounds. Pack wounds if direct pressure is insufficient. Apply a tourniquet to limbs if bleeding cannot be controlled by other means. Note the time of tourniquet application. Use an Israeli bandage (emergency bandage) if available.

WARNING: Tourniquet use is widely taught in Israeli civilian first aid due to security context. Always note the time a tourniquet is applied and report it to the MDA team.

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Step 6: Position the patient appropriately.

If conscious: lay the patient on their back and gently elevate legs approximately 30 cm (12 inches) above the level of the heart to improve blood flow to vital organs.

Is the patient conscious?

YES -> Lay on back with legs elevated approximately 30 cm, unless contraindicated (head/neck/spinal injury, broken hip or leg, causes pain, or patient has difficulty breathing).

NO -> If unconscious but breathing, place in the recovery position (on their side). If not breathing, begin CPR.

SHOCK (HELEM) (continued)

Israel – Adult

CALL 101 (MDA) – CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א) **Police** (100 :משטרה) **Fire** (102 :כיבוי) **Hatzalah** (1221 :הצלה)

Step 7: Handle contraindications for leg elevation.

If the patient has difficulty breathing, keep them semi-upright instead of elevating legs. If head, neck, or spinal injury is suspected, do not elevate legs and do not move the patient unnecessarily. If broken hip or leg bones are suspected, do not elevate legs.

Does the patient have suspected head/neck/spinal injury, broken hip/leg, difficulty breathing, or does leg elevation cause pain?

YES -> Do NOT elevate legs. Keep patient flat (spinal/fracture) or semi-upright (breathing difficulty). Minimise movement.

NO -> Elevate legs approximately 30 cm above heart level.

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Step 8: Maintain body temperature.

Cover the patient with a blanket, coat, or any available covering to prevent heat loss. Place insulation underneath the patient to protect from cold ground. Shock patients lose body heat even in hot weather.

WARNING: In Israeli summer conditions, shade the patient from direct sun but still keep them warm. Do not overheat the patient.

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Step 9: Do NOT give food or drink.

Do not give the patient anything to eat or drink. If the patient is thirsty, moisten their lips with water only.

WARNING: Giving food or drink to a shock patient may cause vomiting and aspiration, and may complicate potential surgery.

Step 10: Administer epinephrine for anaphylactic shock if available.

If anaphylactic shock is suspected (allergic reaction with rash, swelling, wheeze, stridor) and the patient has a prescribed EpiPen, assist with administration.

Is anaphylactic shock suspected and is an EpiPen available?

YES -> Assist the patient with their EpiPen. Follow the anaphylaxis protocol (IL-ADULT-ALLERGIC-REACTION-001).

NO -> Continue monitoring and supportive care. Keep the patient positioned and warm.

Step 11: Monitor and reassure the patient continuously.

Monitor breathing and pulse continuously. Reassure and comfort the patient; keep them calm. Stay with the patient until MDA emergency services arrive.

Does the patient vomit?

YES -> If no spinal injury is suspected, turn the patient on their side to prevent choking. Clear the airway.

NO -> Continue monitoring in current position.

Step 12: Document and report to the arriving MDA team.

Note the time of injury or onset of symptoms. Note any treatments given, including tourniquet application time, medications administered, and changes in patient condition. Report all information to the arriving MDA paramedics.

DO NOT:

X Do not give the patient anything to eat or drink -- moisten lips only if thirsty.
X Do not elevate legs if head, neck, or spinal injury is suspected.
X Do not elevate legs if the patient has a broken hip or leg.
X Do not elevate legs if it causes the patient pain or further injury.
X Do not elevate legs if the patient has difficulty breathing -- keep them semi-upright instead.
X Do not overheat the patient when maintaining body temperature.
X Do not move a patient with suspected spinal injury unnecessarily.
X Do not delay calling 101 -- shock is a life-threatening emergency.
X Do not remove a tourniquet once applied -- leave removal to MDA paramedics.
X Do not leave the patient unattended.

EQUIPMENT NEEDED:

- Tourniquet -- widely available in Israeli first aid kits due to security context
- Israeli bandage (emergency bandage / amtza'sh) -- for wound packing and pressure
- Blanket or coat -- to maintain body temperature
- Ground insulation -- to protect from cold ground
- EpiPen (epinephrine auto-injector) -- for anaphylactic shock if patient has a prescription

Personal reference only – not medical advice. Always call 101 in an emergency.

Smoke / Tear Gas Inhalation — Adult Protocol

Israel — Adult

ID: IL-ADULT-SMOKE-INHALATION-001

CALL 101 (MDA (מד"א)) — CALL IMMEDIATELY IF IN DOUBT

MDA (מד"א): 101 | **Police (משטרה):** 100 | **Fire (כיבוי):** 102 | **Hatzalah (הצלה):** 1221

When to apply:

Person has inhaled smoke, fumes, or tear gas. Signs include: coughing, wheezing, difficulty breathing, hoarse voice, soot around mouth or nose, singed facial hair, burns to face or mouth, confusion or altered consciousness, eye irritation and tearing (tear gas).

Summary:

Ensure scene safety, remove the person to fresh air, call 101, position upright for easier breathing, monitor for respiratory distress, perform CPR if needed, assess for burns, and keep warm. For tear gas: remove from exposure, flush eyes with water for 15 minutes, decontaminate skin and clothing.

Also call Fire and Rescue (כיבוי אש): 102 — if fire is ongoing

Step 1: Ensure scene safety.

Do NOT re-enter a smoke-filled area to rescue someone unless you are trained and equipped (firefighter with breathing apparatus). If indoors, stay low where air is cleaner (smoke rises). Ensure the area is safe before approaching.

WARNING: Never re-enter a burning or smoke-filled structure without proper protective equipment. Wait for Fire and Rescue (102).

Step 2: Determine the type of exposure.

Is this smoke inhalation (fire, wildfire, fumes) or tear gas exposure?

YES → For smoke inhalation: proceed to Step 3 (move to fresh air and call 101 immediately).

NO → For tear gas exposure: proceed to Step 9 (tear gas protocol).

SMOKE INHALATION BRANCH (Steps 3–8)

Step 3: Move the person to fresh air immediately.

Remove the person from the smoke-filled environment. Move them to an area with clean, fresh air.

Can you safely move the person out of the smoke-filled area?

YES → Remove them immediately to fresh air. Proceed to Step 4.

NO → Wait for Fire and Rescue (102) or MDA (101). Do not risk your own safety.

Step 4: Call MDA at 101.

Report the location, number of casualties, and whether the fire is still active. Also call Fire and Rescue at 102 if fire is ongoing. Every second counts in smoke inhalation.

Step 5: Position the patient for easier breathing.

Is the person conscious?

YES → Sit them in an upright position – this helps with easier breathing.

NO → If unconscious but breathing, place in the recovery position (on their side). If unconscious and not breathing, lay flat for CPR (proceed to Step 7).

Step 6: Monitor breathing and look for warning signs.

Watch for signs of respiratory distress: coughing, wheezing, difficulty breathing, hoarse voice. Look for soot around mouth/nose, singed facial hair, or burns to the face/mouth.

WARNING: Symptoms may worsen over hours as airway swelling develops. A person who appears stable may deteriorate.

Step 7: Begin CPR if the person stops breathing.

If the person stops breathing, begin CPR immediately (if trained). Use standard 30:2 compression-to-ventilation ratio. Continue until the person starts breathing or MDA arrives.

Is the person breathing?

YES → Continue monitoring. Maintain upright position (if conscious) or recovery position (if unconscious).

NO → Begin CPR immediately with 30:2 compression-to-ventilation ratio. Continue until breathing resumes or MDA arrives.

Step 8: Check for burns and keep the person warm.

Check for burns, especially to the face, neck, and hands. Cool burns with clean water. Do not remove clothing stuck to burns. Cover with clean, non-adherent dressing. Cover the person with a blanket -- smoke inhalation patients may become hypothermic.

TEAR GAS BRANCH (Steps 9–12)

Step 9: Tear gas exposure: Remove from exposure area.

Move the person away from the tear gas source to fresh air. Move upwind if possible. Effects are normally self-limiting once exposure ends.

Step 10: Tear gas exposure: Flush eyes with water.

Flush eyes with large amounts of clean, tepid water for at least 15 minutes. Water or saline are the safest options. Do NOT rub the eyes -- this worsens the irritation.

WARNING: Do NOT use milk or other substances to flush eyes due to infection risk. Use only clean water or saline.

Step 11: Tear gas exposure: Decontaminate skin and clothing.

Remove contaminated clothing. Wash affected skin with soap and water. Blow the nose to clear chemical residue from nasal passages.

Step 12: Tear gas exposure: Monitor for deterioration.

Symptoms should begin to subside once exposure is eliminated. If symptoms do not improve or continue to worsen after 30 minutes in fresh air, call MDA at 101.

Are symptoms improving after removal from tear gas exposure?

YES → Continue monitoring. Seek medical attention if the person has pre-existing respiratory conditions (asthma, COPD), is a child, elderly, or pregnant.

NO → Call MDA at 101 immediately. Seek medical attention for: persistent breathing difficulty, chest pain, severe eye pain, or vomiting.

DO NOT:

X DO NOT re-enter a smoke-filled or burning structure without proper firefighting equipment and training.
X DO NOT rub eyes after tear gas exposure -- this worsens irritation and spreads the chemical.
X DO NOT use milk or other substances to flush eyes -- use only clean water or saline due to infection risk.
X DO NOT remove clothing stuck to burns.
X DO NOT assume a smoke inhalation patient is stable -- symptoms may worsen over hours as airway swelling develops.
X DO NOT leave a smoke inhalation patient alone -- continuous monitoring is required.
X DO NOT ignore soot around the mouth/nose or singed facial hair -- these are indicators of potentially serious airway injury requiring hospital evaluation.

Equipment needed:

— Phone to call 101 (MDA) and 102 (Fire and Rescue)
— Clean water for eye flushing (tear gas) and burn cooling
— Clean cloth or non-adherent dressings for burns
— Blanket for warmth
— Soap for skin decontamination (tear gas)

Source: Magen David Adom (MDA) — MDA First Aid Guidelines - Smoke and Tear Gas Inhalation
Imported: 2026-03-16 · Last verified: 2026-03-16
Personal reference only — not medical advice. Always call 101 in an emergency.

Snakebite — Adult Protocol

Israel — Adult

ID: IL-ADULT-SNAKEBITE-001

CALL 101 (MDA (מד"א)) — CALL IMMEDIATELY IF IN DOUBT

MDA (מד"א): 101 | **Police (משטרה):** 100 | **Fire (כיבוי):** 102 | **Hatzalah (הצלה):** 1221

Israel Poison Information Center (24/7): 04-7771900 — Rambam Health Care Campus, Haifa

When to apply:

Person has been bitten or suspected bitten by a snake. Signs include: puncture marks on skin, pain and swelling at bite site, redness or bruising around the wound. May progress to nausea, vomiting, abdominal pain, tachycardia, hypotension, difficulty breathing, or altered consciousness.

Summary:

Immobilise the victim and bitten limb, call 101 immediately, clean wound with soap and water only, monitor for systemic reaction, and transport to hospital for evaluation and possible antivenom.

Step 1: Call MDA immediately at 101.

MDA dispatch will provide real-time telephone guidance until paramedics arrive. Remain calm when calling -- do not create panic, as stress increases the victim's heart rate and accelerates venom spread.

Step 2: Keep the victim calm and completely still.

Reassure the victim and reduce their stress and anxiety. Have them lie down. Immobility and a low heart rate are critical to slowing the spread of venom through the body. Panic increases heart rate and accelerates venom circulation.

Step 3: Position and immobilise the bitten limb.

Position the bitten limb below heart level. The limb should not move at all. Immobilise (splint) the affected limb against the body to prevent any movement.

Step 4: Remove constrictive items from the bite area.

Remove jewellery, watches, rings, and tight clothing from the bitten area and the entire affected limb. The bite area will swell, and constrictive items can cause additional tissue damage.

WARNING: Swelling can progress rapidly. Remove all constrictive items as early as possible.

Step 5: Clean the bite wound with soap and water only.

Gently wash the bite area with soap and water. Do NOT use alcohol-based disinfectants or any other cleaning agents.

WARNING: Do NOT use alcohol-based disinfectants on the bite wound.

Step 6: Attempt to identify the snake if it can be done safely.

If possible, photograph the snake from at least 2 metres away while remaining safe. If the snake is dead, keep it to show medical staff. Identification helps determine whether antivenom is needed and which type.

WARNING: Do NOT attempt to catch or kill the snake. All snakes in Israel are protected under the Wildlife Protection Law. Approaching a snake risks additional bites.

Can the snake be safely photographed from at least 2 metres away?

YES → Take a clear photograph of the snake and share it with arriving paramedics. Note the snake's colour, pattern, and approximate size.

NO → Do not approach the snake. Note any details you observed (colour, size, head shape) and report them to paramedics.

Step 7: Monitor the victim for signs of systemic reaction or anaphylaxis.

Continuously watch for: difficulty breathing or swallowing, swelling spreading beyond the bite area, rash or itching, abdominal pain, nausea, vomiting, changes in consciousness, drop in blood pressure, rapid heart rate.

Is the victim showing signs of severe systemic reaction (difficulty breathing, altered consciousness, severe swelling, hypotension)?

YES → Report symptoms immediately to MDA dispatch on the phone. Prepare to manage airway if victim loses consciousness. If the victim stops breathing, begin CPR. Keep the victim lying down with legs elevated if blood pressure drops.

NO → Continue monitoring while keeping the victim calm and still. Await paramedic arrival.

Step 8: Wait for MDA paramedics and prepare for hospital transport.

The victim must be transported to a hospital for evaluation and possible antivenom administration. Antivenom is available for Daboia palaestinae and Echis coloratus bites (polyvalent equine-derived antiserum manufactured by Kamada Ltd.). Note: there is no antivenom available for Atractaspis engaddensis (Israeli mole viper) bites.

Is the victim in a remote location where MDA response will be significantly delayed?

YES → If safe transport is available, begin moving the victim to the nearest hospital while keeping them immobile and the bitten limb below heart level. Maintain phone contact with MDA dispatch for guidance.

NO → Wait for MDA paramedics to arrive. Do not move the victim unnecessarily.

DO NOT:

X DO NOT apply a tourniquet -- it traps venom locally and causes severe tissue damage.
X DO NOT suck the venom out of the wound -- this is ineffective and dangerous.
X DO NOT cut or incise the bite area -- this increases infection risk and does not help remove venom.
X DO NOT cool or apply ice to the bite area -- this can worsen tissue damage.
X DO NOT give the victim food or drink -- this may complicate hospital treatment.
X DO NOT induce vomiting.
X DO NOT give aspirin, ibuprofen, or naproxen -- these can worsen bleeding because viper venom is often anticoagulant.
X DO NOT use alcohol-based disinfectants on the wound.
X DO NOT attempt to catch or kill the snake -- snakes are protected under Israeli law and approaching them risks additional bites.
X DO NOT allow the victim to walk or move the bitten limb -- movement accelerates venom circulation.

Equipment needed:

— Splint or improvised immobilisation material
— Soap and clean water for wound cleaning
— Phone or camera to photograph the snake

Source: Magen David Adom (MDA) — “How to Treat a Snake Bite (כיצד לטפל בהכשת נחש)” · MDA 101 Public First Aid Guidance Series
Imported: 2026-03-15 · Last verified: 2026-03-15
Personal reference only — not medical advice. Always call 101 in an emergency.

Spider Bites — Adult Protocol

Israel — Adult

ID: IL-ADULT-SPIDER-BITES-001

CALL 101 (MDA (מד"א)) — CALL IMMEDIATELY IF IN DOUBT

MDA (מד"א): 101 | **Police (משטרה):** 100 | **Fire (כיבוי):** 102 | **Hatzalah (הצלה):** 1221

Israel Poison Information Center (24/7): 04-7771900 — Rambam Health Care Campus, Haifa

When to apply:

Person has been bitten or suspected bitten by a spider. Signs vary by species: local pain, swelling, redness at bite site; for recluse bites – initially painless "pricking sensation" progressing to burning, swelling, blistering, and blue-white-red concentric pattern; for black widow bites – local pain (onset 15 minutes to 1 hour), two fang marks, spreading pain, muscle cramps/spasms (especially abdominal), sweating.

Summary:

Stay calm, clean the bite with soap and water, apply cool compress, elevate the bitten area, do not apply tourniquet or cut the wound, attempt spider identification if safe, call MDA at 101 or the Poison Center at 04-7771900 if symptoms are severe or involve a suspected black widow or recluse bite.

Step 1: Stay calm and move away from the spider.

Reassure the victim that most spider bites in Israel are not life-threatening. Do not attempt to catch the spider, but if safely possible, note its appearance or take a photo for identification.

Step 2: Clean the bite wound with mild soap and water.

Gently wash the bite area to reduce infection risk.

Step 3: Apply a cool compress to the bite site.

Use a clean cloth dampened with cold water or a wrapped ice pack. Apply for 15 minutes per hour to reduce pain and swelling. Do not apply ice directly to skin.

Step 4: Keep the bitten area elevated and still.

Elevate the bitten limb above heart level if possible. Minimise movement of the affected area.

Step 5: Attempt to identify the spider if it can be done safely.

Photograph the spider or note its colour, size, and distinguishing features. Do not risk another bite. Two species of medical significance in Israel: Mediterranean recluse (Loxosceles rufescens) -- small brown 'violin spider,' very common in citrus groves and buildings; Mediterranean black widow (Latrodectus tredecimguttatus) -- dark with markings, less common but more systemically dangerous.

Can the spider be safely identified or photographed without risk of additional bites?

YES → Take a photograph or note distinguishing features. Share with medical responders.

NO → Do not approach the spider. Report any details you observed to paramedics when they arrive.

Step 6: Assess whether the bite requires emergency medical attention.

Is the victim showing any of the following: severe pain, muscle cramps/spasms (especially abdominal), difficulty breathing, nausea/vomiting/fever, spreading redness or discolouration, or any systemic symptoms beyond the bite site?

YES → Call MDA at 101 immediately. Also call the Poison Center at 04-7771900. This requires urgent medical evaluation. Proceed to Step 7.

NO → Monitor the bite. Call the Poison Center at 04-7771900 for guidance. Seek medical attention if symptoms worsen or the victim is a child, elderly, or has pre-existing conditions.

Step 7: Determine suspected spider type for appropriate monitoring.

Is a black widow (Latrodectus) bite suspected? Signs: local pain onset within an hour, muscle cramps/spasms, abdominal pain, sweating, spreading pain.

YES → Call MDA at 101 immediately – black widow bites require hospital evaluation and possible antivenom. Monitor breathing closely as respiratory compromise is possible in severe cases. Keep the victim calm and still.

NO → If recluse (Loxosceles) suspected: initially painless bite progressing to burning, blistering, blue-white-red pattern. Call Poison Center (04-7771900) for guidance. Seek medical attention – do not wait for symptoms to worsen. Do NOT attempt to cut out or excise the bite wound.

Step 8: Monitor the victim while awaiting medical help.

Watch for worsening local symptoms (spreading redness, necrosis, increasing swelling) and systemic symptoms (muscle cramps, breathing difficulty, nausea, fever, confusion). Be prepared to report all observations to arriving paramedics.

Is the victim conscious and breathing adequately?

YES → Continue monitoring. Keep the bitten area elevated, cool compress applied, and the victim calm.

NO → If unconscious but breathing, place in recovery position. If not breathing, begin CPR immediately. Call MDA at 101 if not already done.

DO NOT:

X DO NOT apply a tourniquet -- spider venom is not stopped by tourniquets and a tourniquet will cause tissue damage.
X DO NOT cut, suck, or apply suction to the bite wound -- this is ineffective and increases infection risk.
X DO NOT apply heat to the bite -- use only cool compresses.
X DO NOT attempt to catch the spider at risk of additional bites -- photograph from a safe distance if possible.
X DO NOT attempt to cut out or excise a recluse spider bite wound -- early surgical excision is contraindicated and worsens outcomes.
X DO NOT apply ice directly to skin -- wrap in cloth first.
X DO NOT ignore initially mild symptoms -- recluse bites are often painless at first but progress to serious tissue necrosis over hours to days.
X DO NOT assume a bite is from a spider without confirmation -- 83.5% of cases presenting as 'spider bites' are actually other conditions (bacterial infections, allergic reactions, other arthropod bites).

Equipment needed:

— Phone to call 101 (MDA) and 04-7771900 (Israel Poison Information Center)
— Phone or camera to photograph the spider if safe to do so
— Mild soap and clean water for wound cleaning
— Clean cloth or wrapped ice pack for cool compress

Source: Magen David Adom (MDA) — First Aid for Spider Bites - Mediterranean Recluse and Black Widow
Imported: 2026-03-16 · Last verified: 2026-03-16
Personal reference only — not medical advice. Always call 101 in an emergency.

STROKE (Cerebrovascular Event)

Israel — Adult

אירוע מוחי / שבץ מוחי

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א) **Police (100 :משטרה)** **Fire (102 :כיבוי)** **Hatzalah (1221 :הצלה)**

Recognise stroke – use FAST:

F (Face): Facial drooping or asymmetry
S (Speech): Slurred speech, difficulty forming words
A (Arm): Arm weakness, one arm drifts down
T (Time): Note the exact time symptoms began

Also watch for: sudden severe headache, sudden vision loss, sudden loss of balance/coordination, sudden change in consciousness, seizures in adults without prior history. Even a single FAST sign warrants immediate emergency evaluation.

Step 1: Assess for stroke using the FAST method.

F (Face): Ask the person to smile -- look for one-sided drooping. A (Arm): Ask them to extend both arms forward -- look for one arm drifting downward. S (Speech): Ask them to repeat a simple sentence -- listen for slurred or garbled speech. T (Time): Note the exact time symptoms first appeared. Even a single positive sign warrants calling 101.

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Step 2: Call 101 (MDA) immediately.

Do not wait to see if symptoms improve. Do not drive the person to hospital yourself unless absolutely unavoidable -- the ambulance crew will pre-notify the hospital stroke team and can begin assessment en route. Treatment window is approximately 6 hours from onset; target hospital arrival within 60 minutes.

WARNING: Every minute of delay results in approximately two million neurons dying in the brain (per MDA). Time is critical.

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Step 3: Record and report the exact time of symptom onset.

This information is critical for hospital treatment decisions, particularly eligibility for thrombolytic (clot-dissolving) treatment. If the person went to sleep with normal function and woke with symptoms, record onset time as when sleep began.

WARNING: Without an accurate onset time, doctors may not be able to administer time-sensitive treatments.

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Step 4: Assess whether the person is conscious and breathing.

Is the person conscious?

YES → Lay the person down with their head slightly raised and supported. Proceed to Step 5.

NO → Check for breathing. If breathing: place in recovery position (on their side) to protect airway. If **not breathing**: begin CPR and follow cardiac arrest protocol.

Step 5: Position the person appropriately.

If conscious: lay them down with head slightly raised and supported. If vomiting or at risk of vomiting: place on their side (recovery position) to protect airway from aspiration.

Is the person vomiting or at risk of vomiting?

YES → Place the person on their side in the recovery position to protect the airway.

NO → Keep the person lying down with head slightly raised and supported.

Step 6: Loosen restrictive clothing.

Remove or loosen scarves, ties, tight collars, and any other clothing that may restrict breathing.

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Step 7: Stay with the person and monitor continuously.

Watch for changes in consciousness level, breathing difficulties, or worsening symptoms. Any change in consciousness level serves as a critical diagnostic indicator. Be prepared to report condition to arriving paramedics.

Does the person lose consciousness or stop breathing while waiting?

YES → If breathing stops: begin CPR immediately. Update MDA dispatch by phone. If unconscious but breathing: place in recovery position.

NO → Continue monitoring and providing reassurance until paramedics arrive.

Step 8: Provide information to paramedics on arrival.

Report: the exact time symptoms began, what symptoms you observed (which FAST signs), whether symptoms changed or progressed, the person's current medications (if known), their medical history (if known), any changes in consciousness level.

DO NOT:

X Give food or drink -- the person's swallowing reflex may be impaired (dysphagia is common in stroke), creating aspiration risk.
X Give any medications, especially aspirin or blood thinners -- approximately 15% of strokes are hemorrhagic (caused by bleeding), and aspirin or blood thinners can worsen hemorrhagic stroke. Only medical professionals should decide on medication.
X Delay calling 101 -- do not drive the person to hospital yourself unless absolutely unavoidable. The ambulance crew will pre-notify the hospital and begin assessment en route.
X Ignore transient symptoms -- even if symptoms resolve (possible TIA / Transient Ischemic Attack), the person still needs emergency evaluation. Approximately 40% of TIA patients experience a full stroke within three months.
X Allow the person to eat, drink, or take medications by mouth until a medical professional has assessed their swallowing function.

Equipment needed:

— Phone to call 101 and record symptom onset time
— Pillow or support to slightly elevate the head

Source: Magen David Adom (MDA) · MDA 101 Public First Aid Guidance Series; supplemented by Dr. Shfir Botner (MDA Paramedic School) and United Hatzalah educational material
Last verified: 2026-03-15 · Protocol ID: IL-ADULT-STROKE-001
Personal reference only — not medical advice. Always call 101 in an emergency.

Tourniquet Application — Adult Protocol

Israel — Adult

ID: IL-ADULT-TOURNIQUET-001

CALL 101 (MDA (מד"א)) — CALL IMMEDIATELY IF IN DOUBT

MDA (מד"א): 101 | **Police (משטרה):** 100 | **Fire (כיבוי):** 102 | **Hatzalah (הצלה):** 1221

When to apply:

Life-threatening bleeding from a limb that cannot be controlled by direct pressure. Specific indications: direct pressure fails to control bleeding, partial or complete amputation, severe crushing injury to a limb, uncontrollable hemorrhage from a limb wound, multi-casualty situations where sustained direct pressure on each wound is not feasible, circumstances preventing sustained direct pressure (need to move casualty, treat others, or environment is too dangerous).

Summary:

Apply a tourniquet for life-threatening limb hemorrhage when direct pressure fails, for amputations, severe crushing injuries, or multi-casualty situations. Place above the injury, tighten until bleeding stops, document the time, and never remove once applied.

Step 1: Attempt direct pressure first (unless circumstances demand immediate tourniquet).

MDA teaches a progressive approach: direct pressure with hand or gauze, then Israeli bandage, then wound packing, then pressure points, then tourniquet. In multi-casualty or active-threat situations, skip directly to tourniquet.

Is direct pressure controlling the bleeding, or is the situation stable enough for graduated hemorrhage control?

YES → Continue with direct pressure. Apply an Israeli bandage to maintain sustained pressure. Verify pulse distal to the bandage. Monitor closely.

NO → Proceed immediately to tourniquet application (Step 2). Apply a tourniquet when: direct pressure fails, there is amputation or crushing, you are in a multi-casualty situation, or you cannot maintain sustained pressure.

Step 2: Expose the wound.

Cut or remove clothing to identify the bleeding source and determine correct tourniquet placement.

Step 3: Position the tourniquet above the injury site.

Place the tourniquet between the wound and the heart, on the limb above the bleeding site. Position over a single bone if possible -- upper arm or upper thigh is preferred for most effective compression.

WARNING: Tourniquets are NOT suitable for junctional zones (groin, armpit, neck), head wounds, or abdominal wounds. For these locations, use direct pressure, wound packing, or pressure points instead.

Step 4: Tighten the tourniquet strap and secure it.

Pull the strap tight and fasten it. If using a CAT tourniquet, proceed to twist the windlass.

Are you using a commercial tourniquet (CAT or similar)?

YES → Twist the windlass rod (Step 5) until bleeding stops completely.

NO → If improvising (shirt, towel, belt, etc.), apply as tightly as possible until bleeding stops. Improvised tourniquets are less reliable but can be effective as a last resort.

Step 5: Twist the windlass until bleeding stops.

On a CAT tourniquet, turn the windlass rod until bleeding stops completely. Then secure the windlass by locking the rod in the clip or holder so it cannot unwind.

Step 6: Verify that bleeding has stopped.

Check that there is no further blood flow from the wound.

Has the bleeding stopped completely?

YES → Proceed to document the time (Step 7).

NO → Apply a second tourniquet above the first one. Tighten and verify again.

Step 7: Document the time of tourniquet application.

Write the time on the tourniquet's white tab, on the patient's skin, or in any obvious visible location. This is critical -- hospital teams need this information to assess whether the limb can be saved.

WARNING: Time documentation is critical for limb viability assessment. If you forget, estimate the time as accurately as possible.

Step 8: Call MDA at 101.

Always mention that a tourniquet has been applied when calling emergency services. Report the location, the injury, and the time the tourniquet was applied.

Step 9: Monitor the victim while awaiting MDA.

Watch for signs of shock (rapid pulse, pale/cold skin, confusion). Keep the victim calm and still. Do not allow them to walk or move the affected limb.

Is the victim conscious and breathing?

YES → Continue monitoring and reassurance until MDA arrives. Keep the victim warm.

NO → If unconscious but breathing, place in recovery position. If not breathing, begin CPR. Report status change to MDA dispatch.

DO NOT:

X DO NOT remove the tourniquet once applied -- only medical professionals should decide whether to release or convert a tourniquet.
X DO NOT apply a tourniquet to junctional zones (groin, armpit, neck), head, or abdomen -- use direct pressure, wound packing, or pressure points for these areas.
X DO NOT apply the tourniquet over a joint -- place it on the limb above the wound, preferably over a single bone (upper arm or upper thigh).
X DO NOT loosen or adjust the tourniquet to 'check if bleeding has stopped' -- this can restart hemorrhage.
X DO NOT delay tourniquet application in life-threatening hemorrhage due to fear of limb loss -- loss of a limb is preferable to loss of life.
X DO NOT allow the victim to walk or move the affected limb after tourniquet application.
X DO NOT attempt tourniquet conversion (replacing tourniquet with direct pressure) -- this is a professional-level decision under medical direction only.

Equipment needed:

— CAT tourniquet (Combat Application Tourniquet) -- preferred
— Improvised tourniquet material (shirt, towel, belt) as last resort
— Scissors or Leatherman Raptor for cutting clothing to expose the wound
— Pen or marker to document tourniquet application time
— Phone to call 101 (MDA)

Source: Magen David Adom (MDA) — MDA First Aid Guidelines - Tourniquet Application and Hemorrhage Control
Imported: 2026-03-16 · Last verified: 2026-03-16
Personal reference only — not medical advice. Always call 101 in an emergency.

AED Operation — Child and Infant

Israel – Child / Infant

Child: 1–8 years | Infant: under 1 year | Age 8+ or signs of puberty: use adult AED protocol

CALL 101 (MDA) – CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א) **Police (100 :משטרה)** **Fire (102 :כיבוי)** **Hatzalah (1221 :הצלה)**

When to use an AED: Child or infant who is **unresponsive**, **not breathing normally** (absent or gasping), and in suspected **cardiac arrest**.
Lone rescuer: Perform **2 minutes of CPR first** (cardiac arrest in children is more often respiratory in origin), then call 101 and retrieve AED.
Locate AED: Use the **Eifo-Defi** app, or call 101 – MDA dispatch can locate and remotely unlock the nearest smart stand.

Step 1: Recognise cardiac arrest in the child or infant.

Check responsiveness: call out and tap/shake the child's shoulders (for infants, flick the sole of the foot). Check breathing for no more than 10 seconds -- look, listen, and feel. If unresponsive and not breathing normally (absent breathing or only gasping), suspect cardiac arrest.

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Step 2: Call for help and send for an AED.

If a bystander is present, instruct them to call 101 (MDA) immediately and retrieve the nearest AED. Use the Eifo-Defi app to locate the nearest device, or call 101 -- MDA dispatch can locate the nearest smart stand and remotely unlock it.

Is a bystander available to help?

YES -> Send bystander to call 101 and fetch AED. Begin CPR immediately (Step 3).

NO -> If alone, perform **2 minutes of CPR first** (approx. 5 cycles of 30:2), then call 101 and retrieve AED.

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Step 3: Begin CPR immediately while waiting for the AED.

Place the child on a firm, flat surface. Rate: 100--120 compressions per minute. Give 2 rescue breaths after every 30 compressions (solo) or 15 compressions (two trained rescuers).

Is the victim an infant (under 1 year)?

YES -> Two-finger technique (solo) or two-thumb encircling (two rescuers). Compress to **4 cm / 1.5 in**. Slight head tilt only for rescue breaths.

NO -> One-hand technique on centre of chest. Compress to **5 cm / 2 in**. Standard head-tilt chin-lift for rescue breaths.

AED Operation – Child and Infant – Steps 4–7

Power On – Paediatric Mode – Prepare Chest – Apply Pads

MDA (101 :מד"א) **Police (100 :משטרה)** **Fire (102 :כיבוי)** **Hatzalah (1221 :הצלה)**

Step 4: When the AED arrives, open and power on the device.

Most devices power on automatically when opened (some require pressing a power button). The AED will provide voice instructions in Hebrew. Put MDA dispatch on speakerphone if still on the call -- they will guide you.

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Step 5: Activate paediatric mode on the AED.

Many AEDs in Israel have a paediatric mode switch (button or key) that delivers a lower energy dose for children under 8 years or under 25 kg. Activate this before applying pads.

WARNING: Do NOT delay defibrillation to search for paediatric pads or a paediatric mode. If unavailable, use adult pads immediately.

Does the AED have a paediatric mode switch or paediatric pads?

YES -> Activate paediatric mode and/or use paediatric pads (reduced energy dose). Proceed to Step 6.

NO -> Use **adult pads** – do NOT delay defibrillation. The AED can still be used safely. Proceed to Step 6.

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Step 6: Expose the child's bare chest and prepare the skin.

Remove or cut away clothing. Ensure the chest is dry -- wipe off sweat, water, or moisture. Remove any medication patches and wipe skin. If implanted pacemaker/defibrillator visible (lump under skin), place pads at least 2.5 cm (1 inch) away from it.

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Step 7: Apply electrode pads using correct placement for the child's size.

Follow the diagram printed on the pads. For small children and infants, anteroposterior (front-and-back) placement is preferred because the chest may be too small for both pads on the front without touching.

WARNING: Pads must NEVER touch or overlap each other. If the child's chest is too small for anterolateral placement, always use anteroposterior placement.

Is the chest large enough for both pads on the front (anterolateral) with at least 2.5 cm between them?

YES -> Anterolateral: Pad 1 below right collarbone, right of sternum. Pad 2 left side below armpit, in line with nipple.

NO -> Anteroposterior (front-and-back): Front pad on centre of chest (sternum) at heart level. Back pad between shoulder blades, directly behind front pad.

AED Operation – Child and Infant – Steps 8–12

Connect – Analyse – Shock – Resume CPR – Continue

MDA (101 :מד"א) **Police (100 :משטרה)** **Fire (102 :כיבוי)** **Hatzalah (1221 :הצלה)**

Step 8: Connect the pads to the AED if not pre-connected.

Plug the pad cable into the AED connector. Ensure pads are firmly adhered to the skin with no air gaps.

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Step 9: Allow the AED to analyse the heart rhythm.

The AED will instruct: 'Do not touch the patient' / 'Analysing heart rhythm.' Stop CPR and ensure nobody is touching the child during analysis.

WARNING: Everyone must stop touching the child during rhythm analysis. Any movement or contact can interfere with the analysis.

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Step 10: Follow the AED's shock decision.

WARNING: Ensure all bystanders are clear of the child before delivering a shock. Visually verify nobody is touching the child.

Does the AED advise a shock ('Shock advised')?

YES -> Ensure nobody is touching the child. Call out: **'Stand clear!'** (Hebrew: התרחקו!). Press the shock button when instructed. After the shock, **immediately resume CPR** (Step 11).

NO -> AED says 'No shock advised.' **Resume CPR immediately** (Step 11). The device cannot harm the child – it will not deliver a shock if the rhythm is not shockable.

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Step 11: Immediately resume CPR after the shock or 'no shock' advisory.

Continue age-appropriate CPR for 2 minutes (approx. 5 cycles of 30:2 solo, or 15:2 two rescuers). Use the correct compression technique for the child's age. The AED will automatically re-analyse after 2 minutes.

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Step 12: Continue the cycle of CPR and AED analysis until help arrives.

Repeat: CPR for 2 minutes, then AED analysis, then shock if advised, then CPR again. Do NOT turn off the AED. Leave pads attached at all times, even if the child shows signs of recovery.

Has the child shown obvious signs of life (moves, breathes normally, opens eyes) OR have MDA paramedics arrived?

YES -> If paramedics arrive, hand over. If signs of life, stop compressions but **leave AED pads attached and device on**. Place in recovery position if breathing normally.

NO -> **Continue CPR and AED cycle.** Do not stop unless MDA takes over, child shows signs of life, or you are physically unable to continue.

AED Operation – Child and Infant – Warnings

Critical Do-Not List and Equipment

MDA (101 :מד"א)	**Police (100 :משטרה)**	**Fire (102 :כיבוי)**	**Hatzalah (1221 :הצלה)**

DO NOT:

X DO NOT use an AED on a child who is conscious and breathing normally -- AEDs are only for cardiac arrest.
X DO NOT use an AED while the child is submerged in water or lying in a puddle -- move them to dry ground first before applying pads.
X DO NOT use an AED near flammable gases or oxygen-enriched atmospheres.
X DO NOT delay defibrillation because paediatric pads are unavailable -- use adult pads with anteroposterior placement if needed.
X DO NOT allow pads to touch or overlap each other -- use anteroposterior placement if the chest is too small for anterolateral placement.
X DO NOT touch the child during AED rhythm analysis or shock delivery.
X DO NOT turn off the AED or remove the pads once applied, even if the child regains consciousness.
X DO NOT use an AED on a child who is breathing and has a pulse -- place in recovery position and monitor instead.
X DO NOT perform adult-depth compressions on a child or infant -- compress to one-third of chest depth only.
X DO NOT use adult pads with standard anterolateral placement on an infant or small child if the pads would touch or overlap -- switch to anteroposterior placement.

Equipment needed:

- AED (Automated External Defibrillator)
- Paediatric AED pads (preferred for children under 8 / under 25 kg)
- Adult AED pads (if paediatric pads unavailable)
- Scissors or shears (to cut away clothing)
- Towel or cloth (to dry the chest)
- Razor (included in many AED kits, for removing excessive chest hair if needed)

ALLERGIC REACTION / ANAPHYLAXIS IN CHILDREN

Israel — Child

Source: Magen David Adom (MDA) | Published: 2024-04-01 | Verified: 2026-03-15

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

When to apply: Child showing signs of severe allergic reaction – any combination of: skin symptoms (hives, swelling of face/lips/tongue), respiratory symptoms (wheezing, stridor, throat tightness, difficulty breathing), cardiovascular symptoms (weak/rapid pulse, pallor, cyanosis, confusion, loss of consciousness), GI symptoms (vomiting, diarrhoea, abdominal pain), or behavioural changes (sudden irritability, inconsolable crying, clinging). **Any involvement of breathing, circulation, or consciousness = anaphylaxis – do not wait.**

Step 1: Ensure scene safety and recognise anaphylaxis.

Look for behavioural cues in young children: sudden distress, inconsolable crying, drooling, difficulty swallowing. Skin: hives, facial swelling, pallor. Respiratory: wheezing, stridor, throat tightness, hoarse voice or change in cry. Any involvement of breathing, circulation, or consciousness indicates anaphylaxis.

Are signs of anaphylaxis present?
(breathing difficulty, facial/throat swelling, cardiovascular compromise, or multi-system involvement)

YES -> Proceed immediately to Step 2. Do not wait for symptoms to worsen.

NO -> Monitor closely. If only mild skin symptoms with no breathing or circulation involvement, observe and be prepared to escalate.

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Step 2: Call MDA at 101 immediately.

Keep phone on speaker. State child's age and approximate weight. Mention if the child has a known allergy and whether an EpiPen is available. United Hatzalah can also be reached at 1221.

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Step 3: Remove the triggering allergen if possible.

Remove food from child's mouth. Remove bee stinger by scraping sideways (do not squeeze). Move child away from allergen source if environmental. Do not induce vomiting.

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Step 4: Manage the airway.

If facial or tongue swelling is present, assume airway compromise. Ensure the tongue is not obstructing the airway. Remove any food debris from the mouth. Loosen tight clothing around the neck and chest.

WARNING: In infants: tilt head only slightly back to a neutral position. Do not over-extend the neck as this can collapse the infant's airway.

Is the child an infant (under 1 year)?

YES -> Use neutral head position only – slight head tilt. Do not over-extend the neck.

NO -> Use standard head-tilt chin-lift if needed to open the airway.

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Step 5: Position the child appropriately.

Is the child conscious?

YES -> If breathing difficulty: allow upright position of comfort (do not force to lie down). If no breathing difficulty: lie flat on back with legs elevated.

NO -> If breathing: place in recovery position (on side). If NOT breathing: begin paediatric CPR immediately.

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Step 6: Administer epinephrine auto-injector (EpiPen) immediately.

Epinephrine is the primary and most critical treatment. Do not delay. Dosing: children 15-30 kg use EpiPen Junior 0.15 mg (green label); children over 30 kg use EpiPen 0.3 mg (yellow label). Some Israeli allergists prescribe 0.3 mg from 20 kg -- follow the child's individual prescription. For children under 15 kg, the 0.15 mg EpiPen Junior is still recommended over withholding treatment.

WARNING: Do not substitute antihistamines (Benadryl/diphenhydramine) for epinephrine. Antihistamines will NOT reverse anaphylaxis.

Is an epinephrine auto-injector available?

YES -> Administer immediately using Step 7 technique. Do not delay.

NO -> Wait for EMS. Monitor airway, breathing, and circulation. Be prepared to begin CPR if needed.

Step 7: Administer the EpiPen using correct technique.

1) Remove EpiPen from plastic case. 2) Remove blue safety cap ('blue to the sky'). 3) Hold with orange tip pointing down ('orange to the thigh'). 4) Position at 90 degrees to middle of outer thigh. 5) For small/thin children, pinch the thigh muscle. 6) Press firmly until click is heard. 7) Hold 10 seconds. 8) Can inject through clothing. 9) After removal, massage injection site for 10 seconds. 10) Dispose safely (hand to paramedics).

WARNING: Do not touch the orange tip. Do not inject into fingers, hands, feet, or buttocks. Outer thigh only. For small/thin children, always pinch the thigh before injection.

Step 8: Reassess the child after 5 minutes.

Epinephrine effects last approximately 20 minutes. Symptoms may recur (biphasic reaction). Biphasic reactions can occur up to 72 hours after the initial reaction.

Has the child's condition improved after 5 minutes?

YES -> Continue monitoring. Maintain position. Wait for EMS. Hospital evaluation is still required.

NO -> If a second auto-injector is available, administer a second dose. Continue monitoring airway, breathing, and circulation.

Step 9: Monitor continuously until EMS arrives.

Continuously monitor airway, breathing, and circulation. Keep the child calm and reassured. Be prepared to begin paediatric CPR if the child loses consciousness and stops breathing. All suspected anaphylaxis in children requires hospital evaluation, even if symptoms fully resolve. Hospital monitoring: 4-6 hours minimum.

WARNING: Never allow a child who has had anaphylaxis to leave without hospital evaluation. Biphasic reactions can be fatal.

Equipment Needed:

- EpiPen Junior 0.15 mg (green label) – for children 15-30 kg
- EpiPen 0.3 mg (yellow label) – for children over 30 kg
- Phone to call MDA at 101

DO NOT:

X Do not delay epinephrine administration -- it is the only effective treatment for anaphylaxis
X Do not substitute antihistamines (Benadryl/diphenhydramine) for epinephrine in anaphylaxis
X Do not force the child to lie down if they are having breathing difficulty -- allow upright position of comfort
X Do not over-extend an infant's neck when managing the airway -- use neutral position only
X Do not inject epinephrine into fingers, hands, feet, or buttocks -- outer thigh only
X Do not touch the orange tip of the EpiPen
X Do not induce vomiting if the allergen was ingested
X Do not allow a child who has had anaphylaxis to leave without hospital evaluation, even if symptoms resolve
X Do not store EpiPen in a refrigerator, in a car, or in direct sunlight -- Israeli climate makes heat degradation a particular risk
X Do not give oral medications (including antihistamines) to a child with throat swelling or difficulty swallowing

Personal reference only – not medical advice. Always call 101 in an emergency.

ASTHMA ATTACK — CHILD

Israel — Child

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :(מד"א | **Police (100 :(משטרה** | **Fire (102 :(כיבוי** | **Hatzalah (1221 :(הצלה**

When to call: Call MDA at 101 if the child does not improve after using rescue inhaler, if the child cannot speak in full sentences, if lips or fingernails turn blue, if breathing becomes very rapid or laboured, if the child becomes drowsy (dangerous sign of CO_2 buildup), or if this is a first-time asthma attack without a prescribed inhaler. Alternatively, call United Hatzalah at 1221.

Step 1: Help the child sit upright to maximise lung expansion.

An upright or slightly forward-leaning position opens the airways and makes breathing easier. Do not lay the child down. Allow the child to find their most comfortable breathing position. Allow the child to sit with a parent for comfort.

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Step 2: Calm and reassure the child.

Speak directly to the child (not only to parents) in a calm, reassuring voice. Anxiety worsens bronchospasm. Introduce yourself by name and ask the child their name. Use age-appropriate explanations. Never lie about procedures.

WARNING: Stress and anxiety are common asthma triggers and worsen an active attack. Keeping the child calm is a therapeutic intervention.

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Step 3: Assist the child with their prescribed rescue inhaler (Ventolin/salbutamol).

Help the child use their rescue inhaler. Typically 2-4 puffs, one at a time, with 30-60 seconds between puffs. For children under 6 years: use the inhaler with a spacer device (Volumatic or similar) for more effective medication delivery to small airways. In gan (kindergarten) or school settings, ask staff for the child's asthma action plan.

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Does the child have a prescribed rescue inhaler available?

YES: Assist with inhaler use. For children under 6, use with a spacer device if available. Administer 2-4 puffs as per the child's action plan.

NO: Keep the child calm and upright. Call 101 immediately. Do not administer someone else's inhaler without medical direction.

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Step 4: Administer high-concentration oxygen if available.

If supplemental oxygen is available, administer at 10 litres per minute via face mask. This is typically only available to trained first responders with equipment.

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Step 5: Obtain the child's medical history.

Ask the parent or child: Is the child a known asthmatic? What medications do they take? Have they been hospitalised for asthma before? What triggered this attack? When did symptoms start? Common triggers include allergens, air pollution, weather changes, physical exertion, cold air, and stress.

ASTHMA ATTACK — CHILD (continued)

Monitoring, Severe Attack & Transport

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :(מד"א | **Police (100 :(משטרה** | **Fire (102 :(כיבוי** | **Hatzalah (1221 :(הצלה**

Step 6: Monitor for improvement or worsening.

Reassess breathing after 5-10 minutes. Monitor for signs of severe attack: inability to complete sentences, SpO2 below 92% (if pulse oximeter available), rapid breathing, central cyanosis. Drowsiness is a late and dangerous sign indicating CO2 buildup.

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Has the child's breathing improved after inhaler use?

YES: Continue monitoring. A second dose of inhaler may be given after 10-15 minutes if needed. Advise parents to follow up with the child's doctor.

NO: Call 101 (MDA) immediately if not already done. The child needs advanced medical care. Continue to keep the child upright and calm while waiting for emergency services.

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Step 7: Assess for life-threatening attack (status asthmaticus).

A life-threatening attack is characterised by severe hypoxia, inability to speak, exhaustion, silent chest (no wheeze despite distress -- airways too constricted for air movement), drowsiness or confusion, and cyanosis. This requires immediate advanced life support.

WARNING: A 'silent chest' (no wheezing despite obvious distress) is a dangerous sign -- it means airways are too constricted for air movement. This is worse than audible wheezing.

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Is the child showing signs of a life-threatening attack (silent chest, drowsiness, exhaustion, inability to speak)?

YES: This is a life-threatening emergency. Call 101 immediately if not already done. Be prepared to perform CPR if the child stops breathing. Continue rescue inhaler every 5-10 minutes until help arrives.

NO: Continue monitoring and supportive care. Transport to medical facility if symptoms persist.

Step 8: Transport to medical facility if symptoms do not resolve.

Keep the child upright during transport. Bring the child's inhaler and any medication documentation. Report the number of inhaler puffs given and the times administered.

DO NOT:

X Lay the child down -- upright position is essential for breathing.
X Give the child cold drinks during an attack -- cold can worsen bronchospasm.
X Leave the child alone during an attack.
X Administer someone else's prescription inhaler without medical direction.
X Delay calling 101 if the child is not improving.
X Ignore drowsiness during an asthma attack -- this is a dangerous sign of CO2 buildup requiring immediate intervention.
X Assume a quiet chest means improvement -- a silent chest during distress is a life-threatening sign.

Equipment needed:

— Rescue inhaler (Ventolin/salbutamol) -- child's own prescribed inhaler
— Spacer device (for children under 6)
— Supplemental oxygen and face mask (if available -- typically first responder equipment)
— Pulse oximeter (if available)
— Phone (to call 101)

Source: Magen David Adom (MDA) (2024-04-01) · Last verified: 2026-03-16
Personal reference only — not medical advice. Always call 101 in an emergency.
Protocol ID: IL-CHILD-ASTHMA-001

BLAST / SHRAPNEL INJURIES — CHILD

Israel — Child

PHTLS / SABCDE Assessment

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א) **Police (100 :משטרה)** **Fire (102 :כיבוי)** **Hatzalah (1221 :הצלה)**

When to call: Call MDA at 101 immediately for any blast-injured child. Report the nature of the blast, number of casualties, child's condition, and your location. Alternatively, call United Hatzalah at 1221.

Step 1: Ensure scene safety before approaching.

Assess for secondary explosive device risk, structural collapse, fire, and ongoing danger. Do not enter an unsafe scene. Wait for security forces to clear the area if there is any doubt.

WARNING: Risk of secondary explosive devices is significant in terror-related blast events. Do NOT rush in without confirming scene safety. Structural collapse can occur minutes after the initial blast.

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Step 2: Call 101 (MDA) immediately.

Report the blast event, your location, number of casualties, and the involvement of children. MDA dispatch will coordinate ambulances and psychotrauma teams. If multiple casualties, triage may be directed by dispatch.

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Step 3: A -- Assess and maintain the child's airway.

Check airway patency. Look for blood, debris, vomit, or foreign objects in the mouth. In children, airways are smaller and more prone to obstruction. The tongue is proportionally larger and can block the airway. For infants, maintain nasal patency (obligate nasal breathers). Use jaw thrust manoeuvre if spinal injury is suspected.

WARNING: Check for stridor (noisy breathing) which may indicate thermal airway injury from blast. Stridor in children indicates significant airway compromise.

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Is the child's airway clear and open?

YES: Proceed to breathing assessment (Step 4).

NO: Clear the airway manually. Remove visible debris. Position the head to open the airway (head-tilt/chin-lift unless spinal injury suspected, then jaw thrust only).

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Step 4: B -- Assess breathing quality and symmetry.

Look for chest rise and fall. Assess symmetry -- asymmetric chest rise indicates thoracic trauma (possible pneumothorax). Monitor respiratory rate and effort. Look for accessory muscle use, nasal flaring, and intercostal retractions. Blast lung is the most immediately life-threatening primary blast injury.

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Is the child breathing adequately with symmetric chest rise?

YES: Proceed to circulation assessment (Step 5).

NO: If not breathing: begin rescue breathing/ CPR. If asymmetric chest rise: suspect pneumothorax – position child with injured side down if possible. Administer oxygen if available.

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Step 5: C -- Control bleeding and assess circulation.

Evaluate for massive haemorrhage. Children have less blood volume than adults and can decompensate rapidly from blood loss. Check radial and carotid pulses. Assess capillary refill (press fingernail bed -- refill longer than 4-5 seconds indicates shock).

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Is there uncontrollable limb haemorrhage?

YES: Apply a tourniquet proximal to the wound. If no tourniquet available, apply maximum direct pressure with cloth or hands. Record time of tourniquet application. See tourniquet protocol for child-specific guidance.

NO: Apply direct pressure to any bleeding wounds using sterile gauze or clean cloth. Elevate bleeding limbs if no fracture suspected.

Personal reference only — not medical advice. Always call 101 in an emergency.

BLAST INJURIES — CHILD (continued)

Disability, Exposure, Wounds & Transport

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א) **Police** (100 :משטרה) **Fire** (102 :כיבוי) **Hatzalah** (1221 :הצלה)

Step 6: D -- Assess neurological status (Disability).

Use AVPU scale: Alert, responds to Voice, responds to Pain, Unresponsive. Check pupil size and reactivity. A non-communicative child after blast does NOT necessarily mean absence of serious injury -- children may be in psychological shock.

WARNING: A silent, non-communicative child after a blast event may have serious injuries. Do not assume the child is uninjured because they are quiet.

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Step 7: E -- Expose and examine for additional injuries, then prevent hypothermia.

Carefully expose the child's body to check for additional wounds, burns, embedded objects, and fractures. Children's smaller bodies mean organs are closer together, increasing risk of multi-organ injury. After examination, cover the child immediately to prevent hypothermia -- children lose heat rapidly due to higher surface-area-to-mass ratio.

WARNING: Do NOT remove embedded objects (shrapnel, fragments). Stabilise them in place. Removing them may worsen bleeding.

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Step 8: Cover wounds and stabilise embedded objects.

Cover open wounds with sterile dressings. If objects are embedded (shrapnel, glass, debris), do NOT remove them. Build padding around embedded objects to stabilise them. Use bulky dressings to protect wound sites.

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Step 9: Keep the child with their parent or caregiver when possible.

Separation from caregivers compounds psychological trauma in children. If the parent is also injured, assign another trusted adult to stay with the child. Speak calmly and honestly to the child. MDA 101 dispatch can coordinate psychological first responders (psychotrauma teams).

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Step 10: Prepare for transport and monitor continuously.

Keep the child warm and covered. Monitor breathing, pulse, and consciousness level continuously. Be prepared to perform CPR if the child deteriorates. If multiple casualties, follow triage instructions from MDA dispatch. Report all interventions and times to arriving emergency services.

DO NOT:

X Enter an unsafe blast scene -- risk of secondary devices and structural collapse.
X Remove embedded objects (shrapnel, fragments) -- stabilise them in place.
X Assume a quiet child is uninjured -- children may be in psychological shock.
X Remove a tourniquet once applied -- hospital personnel will manage removal.
X Forget to prevent hypothermia after examination -- children lose heat rapidly.
X Separate the child from their parent or caregiver unnecessarily.
X Ignore asymmetric chest rise -- this indicates thoracic trauma requiring urgent attention.
X Give the child food or drink if significant injuries are present -- they may need anaesthesia at hospital.

Equipment needed:

— Tourniquet (CAT or equivalent -- see tourniquet protocol for child-specific sizing)
— Sterile gauze and bandages
— Blankets (for hypothermia prevention)
— Rigid splints (for fracture immobilisation)
— Phone (to call 101)
— Supplemental oxygen and mask (if available)

BURNS — FIRST AID (CHILD)

Israel — Child

ID: IL-CHILD-BURNS-001

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

Call 101 immediately for:

Any burn in an infant under 1 year; burns on face, eyes, mouth, tongue, hands, feet, genitals, groin, or inner thigh; circumferential burns encircling a limb; burns appearing white, grey, or black (third-degree); multiple large blisters; extensive burns; burns to neck, face, or airways; electrical burns; chemical burns; any burn worse than minor first-degree; burns in immunocompromised children.

Summary: First aid for thermal, chemical, and electrical burns in children. Key: cool with lukewarm water for minimum 20 minutes, prevent hypothermia, call emergency services early. Approx. 84% of pediatric burns in Israel are scalds from hot liquids.

Step 1: Ensure scene safety and remove the child from the heat source immediately.

Make sure the scene is safe for the rescuer before approaching. Remove the child from the source of the burn.

What type of burn source?

YES -> **Fire/Electrical/Chemical:** Fire – roll child on floor, extinguish flames. Electrical – use non-conductive object to separate child from source. Chemical – remove contaminated clothing carefully.

NO -> **Thermal/Scald:** Simply move the child away from the heat source.

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Step 2: Remove saturated clothing from the burn area immediately.

Hot liquid in fabric continues damaging skin. Gently remove affected clothing; use scissors if needed. Remove constrictive items (shoes, watches, jewellery, rings) before swelling occurs.

WARNING: If clothing adheres to the skin, LEAVE IT ON. Do NOT force removal -- cut around it instead.

Is clothing stuck to the burn?

YES -> Leave stuck clothing in place. Cut around it with scissors. Do NOT pull it off.

NO -> Gently remove the clothing from the burn area. Remove constrictive items (shoes, watches, jewellery, rings).

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Step 3: Cool the burn under cool (lukewarm, NOT ice-cold) running water for a MINIMUM of 20 MINUTES.

Begin cooling as soon as possible. Use cool or lukewarm running water (mim poshrim). The 20-minute cooling period is critical -- this is the single most important first aid intervention.

WARNING: Do NOT use ice or ice-cold water. Extreme cold can cause additional tissue and blood vessel damage, worsening the injury.

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Step 4: Monitor the child for hypothermia during cooling.

Children are more susceptible to hypothermia from prolonged cooling. Cool the BURN, not the whole child -- avoid immersing the whole child in cold water. Keep the rest of the body warm.

WARNING: Watch for shivering, which indicates the child is getting too cold. Infants and small children are at highest risk.

Is the child showing signs of hypothermia (shivering, cold to touch)?

YES -> Switch to cold-soaked cloths/ bandages applied to the burn area, changing every few minutes. Keep the rest of the body covered and warm.

NO -> Continue cooling the burn under lukewarm running water until the full 20 minutes have elapsed.

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Step 5: Assess burn severity.

First-degree: redness, mild swelling, pain (like sunburn), heals 1-2 weeks. Second-degree: blisters, bright red-pink, severe pain; deeper may appear white. Third-degree: white, grey, yellow-brown, or black with stiff dry texture, tissue may be exposed.

Is the burn severe? (2nd-degree with large blisters, 3rd-degree, large area, sensitive areas, or child under 1 year)

YES -> **Call 101 (MDA) immediately.** Continue first aid steps while waiting for emergency services.

NO -> Continue first aid steps. Monitor for worsening. Seek medical evaluation if concerned.

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Step 6: Cover the burn with sterile, loosely-applied bandages or a clean, dry sheet.

After the 20-minute cooling period, cover the burn loosely. Use material that does NOT stick to skin. If no sterile dressing available, wrap bandage in water-soaked cloth. Continue moistening until arrival at hospital.

WARNING: Do NOT use gauze pads or cotton wool directly on burns -- fibres adhere to damaged skin and cause further injury.

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Step 7: Administer age-appropriate pain relief medication.

Give ibuprofen or paracetamol (acetaminophen) dosed by weight for children. Do not delay pain relief.

Is the child old enough for oral medication and conscious?

YES -> Administer ibuprofen or paracetamol dosed by weight. Offer water in small sips to prevent dehydration.

NO -> Do not give oral medication. Keep child calm and comfortable. Ensure emergency services have been called.

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Step 8: Offer water in small sips to prevent dehydration.

Especially important for children with larger or multiple burns. Dehydration risk is higher in children due to smaller body mass.

Step 9: Call 101 (MDA) for guidance.

Call MDA immediately for serious burns. MDA dispatchers can guide through first aid steps while dispatching an ambulance.

Is the child under 1 year, or does the burn affect sensitive areas, or is the burn worse than a minor first-degree burn?

YES -> **Call 101 immediately.** All burns in infants under 1 year require immediate medical attention, regardless of apparent severity.

NO -> For mild first-degree burns only: apply relief creams after cooling (trolamine/Biafine, bepanthen, or similar). Monitor for infection (redness, swelling, fever, pus). Seek medical attention if symptoms worsen.

Step 10: Assess for chemical burn.

Was the burn caused by a chemical substance?

YES -> Remove contaminated clothing carefully, protecting yourself. Rinse with massive amount of cool running water for at least 20 minutes. If chemical in eyes, flush eyes for at least 15 minutes. Do NOT attempt chemical neutralisation. **Call 101 immediately.**

NO -> Assess for electrical burn (next step).

Step 11: Assess for electrical burn.

Was the burn caused by electricity?

YES -> Ensure power source is disconnected before approaching. **Call 101 immediately.** Electrical burns always require medical evaluation – internal damage may not be visible. Be prepared to perform CPR if child becomes unresponsive.

NO -> Continue monitoring the child until emergency services arrive or the child receives medical evaluation.

Step 12: Monitor for complications while waiting for emergency services.

Watch for signs of infection (redness, warmth, pus, fever), respiratory issues (if face/neck/airway burn is suspected), signs of shock, and continued hypothermia risk. Keep the child warm, calm, and reassured.

DO NOT:

X Do NOT apply ice or ice-cold water to the burn -- extreme cold causes additional tissue and blood vessel damage.
X Do NOT apply toothpaste, coffee grounds, raw eggs, yogurt, or butter -- these trap heat and increase infection risk.
X Do NOT puncture or pop blisters -- the fluid inside protects tissue and prevents infection.
X Do NOT apply ointments, oils, powders, or similar substances to serious burns before medical evaluation.
X Do NOT use gauze pads or cotton wool directly on burns -- fibres adhere to damaged skin.
X Do NOT leave the child unattended if unconscious -- call 101 immediately.
X Do NOT remove clothing that is stuck to the burn -- cut around it instead.
X Do NOT immerse the whole child in cold water -- cool the burn, not the child. High hypothermia risk.
X Do NOT attempt chemical neutralisation on chemical burns.

Equipment Needed:

- Cool (lukewarm) running water source
- Sterile non-adherent dressings or clean dry sheet
- Scissors (to cut clothing around stuck areas)
- Blanket or towel (to keep child warm during cooling and prevent hypothermia)
- Age-appropriate pain relief medication (ibuprofen or paracetamol, dosed by weight)
- Drinking water (for hydration)

Source: Magen David Adom (MDA) (2026-01-01) · Last verified: 2026-03-15
Personal reference only – not medical advice. Always call 101 in an emergency.

CHOKING — CHILD & INFANT

Israel — Child (including Infants)

Foreign Body Airway Obstruction (FBAO)

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

When to call: Have a bystander call 101 immediately upon identifying complete obstruction. If alone, perform 1–2 minutes of intervention first (approx. 5 cycles of CPR if unresponsive), then call 101.

Step 1: Assess the choking victim

Look for: inability to speak, cry, or make sounds; weak or absent cough; difficulty breathing with chest retractions; cyanosis (bluish skin); loss of consciousness. Young children may NOT grasp their throats.

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Can the child cough forcefully, cry, or breathe adequately?

YES: DO NOT intervene. Encourage coughing – it is the body's most effective mechanism. Monitor closely and be ready to act if it worsens.

NO: Airway is severely/completely obstructed. Proceed to Step 2.

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Step 2: Call for help. Instruct a bystander to call 101 (MDA) immediately.

If alone, perform 1-2 minutes of intervention first, then call 101. Do not delay life-saving actions to make the call.

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Step 3: Determine the victim's age category to select the correct technique.

WARNING: Physical size should guide technique selection, not age alone. A very small 1-year-old may need infant techniques.

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Is the victim an infant (under 1 year of age)?

YES: Use INFANT protocol: back blows + chest thrusts. Go to Step 4 (next page).

NO: Use CHILD protocol: abdominal thrusts (Heimlich). Go to Step 7 (next page).

Source: Magen David Adom (MDA) | MDA 101 Public First Aid Guidance Series | https://www.mdais.org/101/asphyxia
Imported: 2026-03-15 · Last verified: 2026-03-15
Personal reference only – not medical advice. Always call 101 in an emergency.

CHOKING — INFANT PROTOCOL

Under 1 year of age

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

Step 4: INFANT: Deliver 5 back blows

Place the infant face-down on your forearm, supporting the jaw and head with your hand. Rest your forearm on your thigh. Tilt the infant's head downward -- head must be lower than the body. Deliver 5 quick, forceful strikes between the shoulder blades using the heel of your other hand.

WARNING: DO NOT use abdominal thrusts (Heimlich) on infants under 1 year. Risk of organ damage.

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Step 5: INFANT: If not dislodged, deliver 5 chest thrusts

Turn infant face-up, supporting body and head. Locate an imaginary horizontal line between the nipples. Place two fingers just below this line at the centre of the sternum. Perform 5 chest compressions, approx. 4 cm deep. Firm but controlled -- avoid excessive force.

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Has the object been expelled?

YES: Stop intervention. Monitor the infant closely and seek medical evaluation.

NO: Continue alternating 5 back blows and 5 chest thrusts. Return to Step 4.

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Step 6: INFANT: Continue alternating 5 back blows and 5 chest thrusts

Check after each cycle whether the object has been dislodged.

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Is the infant still conscious and responsive?

YES: Continue alternating back blows (Step 4) and chest thrusts (Step 5).

NO: STOP back blows and chest thrusts. Proceed to Step 9 (CPR – next page).

Source: Magen David Adom (MDA) | MDA 101 Public First Aid Guidance Series
Imported: 2026-03-15 · Last verified: 2026-03-15
Personal reference only – not medical advice. Always call 101 in an emergency.

CHOKING — CHILD PROTOCOL

Age 1–8 years

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

Step 7: CHILD (1-8 years): Perform abdominal thrusts (Heimlich manoeuvre)

Stand or kneel behind the child (adjust to height). Wrap arms around waist. Make a fist and place it above the navel and below the rib cage. Grasp fist with other hand. Perform quick, forceful thrusts inward and upward. For smaller children, position the child against your leg with fist placed precisely below the ribcage.

WARNING: Do NOT use excessive force that would lift the child off the ground.

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Step 8: CHILD: Continue abdominal thrusts until object is expelled or child becomes unresponsive.

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Has the object been expelled and can the child breathe?

YES: Stop thrusts. Monitor the child and seek medical evaluation even after successful removal.

NO: If still conscious, continue thrusts (repeat Step 7). If child loses consciousness, proceed to Step 9 below.

Step 12 – Special case: If the child is pregnant or obese, use **chest thrusts** instead of abdominal thrusts. Position fist on the middle of the breastbone (sternum) and thrust firmly.

Source: Magen David Adom (MDA) | MDA 101 Public First Aid Guidance Series
Imported: 2026-03-15 · Last verified: 2026-03-15
Personal reference only – not medical advice. Always call 101 in an emergency.

CHOKING — UNRESPONSIVE: CPR

If the child or infant loses consciousness

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

Step 9: Victim loses consciousness: STOP all choking manoeuvres immediately.

Carefully place the victim on a firm, flat surface. Call 101 (MDA) if not already done. If alone, perform 5 cycles of CPR (approx. 2 minutes) before calling 101.

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Step 10: Begin CPR appropriate to the victim's age.

Solo rescuer: 30 compressions to 2 rescue breaths. Two rescuers: 15 compressions to 2 rescue breaths. Compression depth: approx. one-third of chest depth. Before each set of rescue breaths, open the airway and look in the mouth -- remove any visible object. Do NOT perform blind finger sweeps.

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Is the victim an infant (under 1 year)?

YES: Two-finger chest compression (solo) or thumb-encircling method (two rescuers). Open airway with slight head tilt only ("look to sky") – full extension blocks infant airway. Use mouth-to-mouth-and-nose ventilation.

NO: One-hand chest compression. Standard head-tilt chin-lift. Standard mouth-to-mouth ventilation.

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Step 11: Continue CPR until emergency services arrive, the victim recovers, or an AED is available.

If an AED is available and paediatric pads overlap on a small child, apply one pad front-centre and one between the shoulder blades. Continue CPR cycles between AED analysis and shocks.

DO NOT:

X Perform abdominal thrusts (Heimlich) on infants under 1 year -- use only back blows and chest thrusts. Risk of organ damage.
X Insert fingers blindly into the mouth (blind finger sweeps) -- only remove an object if you can clearly see it.
X Force anything into the mouth -- you risk pushing the object deeper.
X Attempt mouth manipulation if teeth are clenched.
X Give food or fluids to a choking victim.
X Intervene if the child is coughing effectively or breathing normally -- allow the body to clear the obstruction naturally.
X Use this protocol if choking is caused by asthma, infection/swelling (e.g. epiglottitis, croup), or head injury -- seek immediate medical care.
X Use excessive force during abdominal thrusts that would lift the child off the ground.
X Fully extend an infant's head when opening the airway -- use the 'look to sky' slight tilt only. Full head tilt-back blocks the infant airway.

Equipment: AED with paediatric pads (if available, for CPR transition)

Personal reference only – not medical advice. Always call 101 in an emergency.

CPR for Children and Infants

Israel – Child / Infant

Child: 1 year to puberty | Infant: 1 month to 1 year

CALL 101 (MDA) – CALL IMMEDIATELY IF IN DOUBT

Key principle: In children, cardiac arrest is almost always caused by *respiratory failure*. **Rescue breaths are essential** – compression-only CPR is NOT adequate.
Sequence: C-A-B (Compressions – Airway – Breathing)
Lone rescuer: Perform **2 minutes of CPR first**, then call 101.

Step 1: Ensure scene safety.

Check the area is safe before approaching the child or infant.

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Step 2: Check responsiveness.

Infant: tap sole of foot. Child: tap and shout, or gently squeeze the trapezius. Call the child's name.

Is the child/infant responsive?

YES -> Do NOT perform CPR. Monitor, keep comfortable, call 101 if concerned.

NO -> Proceed to Step 3 – check breathing.

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Step 3: Check for breathing. Look, listen, and feel for no more than 10 seconds.

Is the child/infant breathing normally?

YES -> Place in recovery position on their side. Monitor breathing. Call 101.

NO -> Child/infant is in cardiac arrest. Proceed to Step 4.

Step 4: Call for help.

If alone: perform 2 minutes of CPR first (approx. 5 cycles of 30:2), then call 101. If someone else is present: send them to call 101 and fetch an AED immediately.

Are you alone with the child/infant?

YES -> Begin CPR now. After 2 minutes (5 cycles of 30:2), call 101 and get AED.

NO -> Send other person to call 101 and fetch AED. Begin CPR now.

CPR for Children and Infants – Steps 5-7

Compressions – Airway – Breathing

Step 5: Begin chest compressions (C). Place on a firm, flat surface.

Compress the lower third of the sternum, on a line between the nipples. Rate: 100-120 compressions per minute. Allow full chest recoil. Minimise interruptions.

Is the patient an infant (under 1 year)?

YES -> **Single rescuer:** Two-finger technique – 2 straight fingers at 90 degrees on centre of chest.
Two rescuers: Two-thumb encircling hands technique.
Depth: approx. **4 cm** (1/3 of chest depth).

NO -> Use heel of **one hand** or **two hands** depending on child's size.
Depth: approx. **5 cm** (1/3 of chest depth).

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Step 6: Open the airway (A).

Infant: neutral/sniffing position with gentle chin lift. Child: head-tilt chin-lift with mild extension.

WARNING: Do NOT hyperextend an infant's neck. Infants in their first months are obligate nasal breathers.

Is the patient an infant (under 1 year)?

YES -> Keep head in **neutral/sniffing position**. Tilt head back only gently and lift chin.

NO -> Perform **head-tilt chin-lift** with mild extension.

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Step 7: Deliver 2 rescue breaths (B).

Each breath over approx. 1 second, just enough to see chest rise. Avoid excessive volume or pressure.

WARNING: Rescue breaths are ESSENTIAL in paediatric CPR. Compression-only CPR is NOT adequate for children and infants.

Is the patient an infant (under 1 year)?

YES -> Seal mouth over infant's **mouth AND nose**. Deliver **gentle puffs** from cheeks, not deep lung breaths.

NO -> Pinch child's nose closed, seal mouth over child's mouth. Deliver breaths until chest rises.

CPR for Children and Infants – Steps 8-12

Continue CPR – AED – When to Stop

Step 8: If chest does not rise with rescue breaths, reposition the airway and try again.

Re-tilt head and re-lift chin, then reattempt. If still unsuccessful, consider foreign body obstruction.

Does the chest rise with rescue breaths?

YES -> Continue CPR cycles (Step 9).

NO -> Reposition head/airway and reattempt. If still no rise, suspect foreign body obstruction.

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Step 9: Continue CPR cycles using the correct compression-to-breath ratio.

Single rescuer: 30:2. Two rescuers: 15:2. Continue without interruption.

Are there two or more rescuers?

YES -> Use **15:2** ratio (15 compressions to 2 breaths). Infants: use two-thumb encircling hands technique.

NO -> Use **30:2** ratio (30 compressions to 2 breaths).

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Step 10: Use an AED as soon as one becomes available.

Use paediatric pads/dose attenuator if available. If only adult pads: ensure pads do not touch each other. For small chests: use anteroposterior placement (one pad front, one pad back). Follow AED voice prompts.

WARNING: If using adult pads on a small child or infant, ensure the pads do not touch or overlap each other.

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Step 11: After any AED shock, immediately resume CPR starting with chest compressions.

Resume 5 cycles of compressions and breaths before reassessing rhythm.

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Step 12: Continue CPR until one of the following occurs.

(1) Professional help arrives (MDA ambulance). (2) Child starts breathing and responding. (3) You are physically unable to continue.

Has the child started breathing and responding?

YES -> **Stop CPR.** Place in recovery position on side. Monitor breathing until MDA arrives.

NO -> **Continue CPR** without interruption until help arrives or you are unable to continue.

DO NOT:

X Do NOT hyperextend an infant's neck -- use neutral/sniffing position only.
X Do NOT insert fingers into the infant's or child's mouth (risk of pushing objects deeper).
X Do NOT deliver deep lung breaths to infants -- use gentle puffs from your cheeks.
X Do NOT use excessive volume or pressure when delivering rescue breaths (risk of gastric distension).
X Do NOT skip rescue breaths in paediatric CPR -- compression-only CPR is NOT adequate.
X Do NOT delay CPR to call for help when alone -- perform 2 minutes of CPR first, then call 101.
X Do NOT allow AED pads to touch or overlap each other on a small child or infant.
X Do NOT stop CPR to check for pulse unless there are clear signs of recovery (breathing, movement).
X Do NOT compress over the xiphoid process (the very bottom tip of the sternum).

Equipment (if available):

- AED with paediatric pads or paediatric dose attenuator
- Barrier device or pocket mask (for rescue breaths)
- Bag-valve-mask: infant size (250-350 cc) or paediatric size

DEHYDRATION — CHILD

Israel — Child (including Infants)

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :(מד"א | **Police (100 :(משטרה** | **Fire (102 :(כיבוי** | **Hatzalah (1221 :(הצלה**

When to call: Call MDA at 101 if the child refuses to drink, has persistent vomiting, has blood in stool, has fever above 40C, shows signs of severe dehydration (lethargy, sunken fontanelle, no tears, rapid weak pulse, cool/mottled extremities, altered consciousness), or is an infant under 3 months with any signs of dehydration. Alternatively, call United Hatzalah at 1221.

Step 1: Move the child to a cool, shaded environment.

Remove the child from heat exposure. Move indoors to an air-conditioned space if possible, or to the nearest shade. Remove excess clothing.

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Step 2: Assess the severity of dehydration.

Mild: dry mouth, decreased urine output, slightly sunken eyes, thirst. Moderate: no tears when crying, sunken fontanelle in infants under 18 months, decreased skin turgor (skin pinch stays tented), lethargy. Severe: very sunken fontanelle, very dry mucous membranes, rapid weak pulse, cool or mottled extremities, altered consciousness.

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Does the child show signs of severe dehydration (altered consciousness, rapid weak pulse, very sunken fontanelle, cool/mottled extremities)?

YES: Call 101 immediately. This is a medical emergency. The child likely needs IV fluids. Continue with supportive care while waiting for emergency services.

NO: Begin oral rehydration. Proceed to Step 3.

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Step 3: Check the fontanelle in infants under 18 months.

The anterior fontanelle (soft spot on top of the head) is open until approximately 18 months of age. Gently palpate it. A sunken fontanelle is a key indicator of dehydration in infants. A bulging fontanelle may indicate other conditions (increased intracranial pressure) -- seek immediate medical care.

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Step 4: Begin oral rehydration with electrolyte-containing fluids.

Use ORS (Oral Rehydration Solution) packets available at Israeli pharmacies (beit mirkachat). Mix as directed on the packet. For infants: give small frequent sips using a spoon or oral syringe. For older children: encourage frequent small sips. Continue breastfeeding if the infant is breastfed.

WARNING: Do NOT give cola, fruit juice, plain water alone, or tea -- these do not adequately replace lost electrolytes and can worsen dehydration.

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Is ORS available?

YES: Prepare ORS as directed on the packet and give small frequent sips.

NO: Use electrolyte-containing sports drinks diluted with equal parts water as a temporary alternative. Do NOT use homemade salt/sugar solutions unless no other option exists.

DEHYDRATION — CHILD (continued)

Monitoring, Feeding & Transport

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :(מד"א | **Police (100 :(משטרה** | **Fire (102 :(כיבוי** | **Hatzalah (1221 :(הצלה**

Step 5: Assess whether the child can drink.

Monitor the child's ability to tolerate oral fluids. Vomiting is common with gastroenteritis but small frequent sips are often still effective.

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Can the child keep fluids down (not vomiting everything)?

YES: Continue oral rehydration with small frequent sips. Give 5-10 ml every 2-5 minutes for young children.

NO: Call 101. The child may need IV fluids. Do not force oral intake if the child is vomiting persistently.

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Step 6: Monitor vital signs and urine output.

Check pulse rate, skin colour, alertness, and whether the child produces urine. Improvement signs include: resumption of urination, moist mouth, return of tears, improved alertness. Continue offering fluids until these signs appear.

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Step 7: Provide moderate salt-containing food if the child is alert and able to eat.

Once the child is tolerating fluids, offer salty crackers or pretzels to help replace sodium. Do not force food. Priority is fluids first.

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Step 8: Transport to hospital if dehydration is moderate or severe, or if the child does not improve.

All infants under 3 months with any signs of dehydration should be taken to hospital. Children with moderate dehydration who are not improving with oral rehydration need medical evaluation. Children with fever above 40C, blood in stool, or yellow/green vomit require emergency evaluation.

DO NOT:

X Give cola, fruit juice, plain water alone, or tea -- these do not replace electrolytes adequately and can worsen dehydration.
X Use homemade salt/sugar solutions unless ORS is completely unavailable.
X Force large volumes of fluid at once -- small frequent sips are more effective and better tolerated.
X Delay calling 101 for an infant under 3 months with any signs of dehydration.
X Ignore a sunken fontanelle in an infant -- this is a key dehydration indicator.
X Give anti-diarrhoea medications to children without medical direction.
X Stop breastfeeding -- continue breastfeeding alongside ORS.

Equipment needed:

— ORS (Oral Rehydration Solution) packets
— Spoon or oral syringe (for infant fluid administration)
— Phone (to call 101)
— Thermometer (to check for fever)

DIABETIC EMERGENCIES IN CHILDREN

Israel — Child

CALL 101 (MDA (מד"א)) — CALL IMMEDIATELY IF IN DOUBT

MDA (מד"א): 101 | **Police (משטרה):** 100 | **Fire (כיבוי):** 102 | **Hatzalah (הצלה):** 1221

Call 101 immediately if:

- Loss of consciousness / decreased responsiveness
- Cannot swallow safely
- Two+ mild hypos in one week
- Any uncertainty about child's condition
- Seizures
- Glucose below 54 mg/dL after 30 min of treatment
- Signs of DKA (vomiting, rapid breathing, fruity breath)
- Glucagon has been administered

Summary: Assess consciousness and ability to swallow, administer fast-acting sugar using the Rule of 15 (10g for children under 6), and call 101 for severe hypoglycemia, seizures, loss of consciousness, or signs of DKA. **When in doubt, give sugar.**

Step 1: Assess the child's level of consciousness and ability to swallow safely.

Look for signs of hypoglycemia: paleness, sweating, trembling, rapid heartbeat, irritability, behavioural changes, confusion, decreased consciousness. In young children (especially in gan or school), sudden irritability, mood swings, or behavioural changes in a child known to have diabetes should prompt immediate assessment. Check for a medical alert bracelet or necklace.

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Is the child conscious and able to swallow safely?

YES -> Proceed to Step 2 (mild/moderate hypoglycemia treatment).

NO -> Skip to Step 7 (severe hypoglycemia – unconscious child).

CONSCIOUS CHILD -- MILD/MODERATE HYPOGLYCEMIA

Step 2: Determine the child's age and administer the appropriate dose of fast-acting sugar (Rule of 15).

Suitable sources: glucose gel syringe (preferred), 125 mL (half a cup) of sweetened juice (not diet), glucose tablets (4 tablets = approx. 15g), 2-3 teaspoons of sugar dissolved in water, a sugar cube, a teaspoon of honey, or 250 mL (one cup) of milk.

WARNING: Avoid chocolate as the initial sugar source -- fat and protein content slows sugar absorption, making it less effective for rapid correction.

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Is the child under 6 years old?

YES -> Give **10 grams** of fast-acting sugar.

NO -> Give **15 grams** of fast-acting sugar.

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Step 3: Wait 15 minutes, then reassess the child.

Monitor the child during this period. If the child has a glucometer available, recheck blood glucose after 15 minutes.

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Has blood glucose risen above target (70 mg/dL for 6+, 80 mg/dL for under 6) and are symptoms improving?

YES -> Proceed to Step 4 (provide complex carbohydrates to sustain glucose).

NO -> Repeat sugar dose (Step 2). If glucose remains below 54 mg/dL after 30 minutes, call 101 immediately.

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Step 4: Once blood glucose is above target, provide complex carbohydrates to prevent recurrence.

Give the child a carbohydrate-rich snack: a slice of bread with cheese, a pastry or cracker, fruit, or any carbohydrate-containing food. This sustains glucose levels and prevents a repeat drop.

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Step 5: Check whether the child has an insulin pump connected.

If the child wears an insulin pump (usually attached to the abdomen or thigh) and is experiencing hypoglycemia, gently disconnect it to prevent further insulin delivery from worsening the low blood sugar.

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Does the child have an insulin pump connected?

YES -> Gently disconnect the insulin pump without damaging the device or tubing.

NO -> No action needed regarding insulin delivery. Continue monitoring.

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Step 6: For moderate hypoglycemia where the child needs assistance but can still swallow: administer 20-30 grams of simple sugar and verify glucose rises above 80 mg/dL within 10 minutes.

If the child is too impaired to self-treat but can still swallow, another person must assist. If swallowing is becoming difficult, apply honey or glucose gel to the inside of the cheek or under the tongue (oral mucosa). Follow with complex carbohydrates once glucose stabilises.

WARNING: If swallowing becomes unsafe at any point, stop oral administration immediately and proceed to Step 7 (severe hypoglycemia protocol).

DIABETIC EMERGENCIES IN CHILDREN

MDA (101 :מד"א) **Police (100 :משטרה)** **Fire (102 :כיבוי)** **Hatzalah (1221 :הצלה)**

SEVERE HYPOGLYCEMIA -- UNCONSCIOUS OR SEIZING CHILD

Step 7: Severe hypoglycemia: the child is unconscious or having seizures. Do NOT give any food or drink by mouth.

This is a life-threatening emergency. Oral administration poses a choking risk when the child cannot protect their airway.

WARNING: Never attempt to give food, drink, or oral medication to an unconscious or seizing child. This is a choking hazard.

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Step 8: Disconnect the insulin pump if the child has one.

Gently disconnect the pump without damaging the device. It is usually attached to the abdomen or thigh.

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Step 9: Assess whether a glucagon emergency kit is available and whether a trained person is present.

Glucagon kits (Hypo-Kit) may be stored at the child's school or gan. Only trained and authorised caregivers (parents, school nurses, designated staff trained by the child's medical team) may administer glucagon.

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Is a glucagon kit available AND is there a trained/authorised person to administer it?

YES -> Proceed to Step 10 (glucagon administration).

NO -> Call 101 (MDA) immediately. Place the child in the recovery position (on their side) and monitor breathing until help arrives.

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Step 10: Administer glucagon by intramuscular or subcutaneous injection using the correct child dose.

To prepare: draw the liquid from the syringe into the powder vial, shake until clear, draw the mixed solution back into the syringe, and inject into a large muscle or under the skin (buttock, abdomen, upper arm, or thigh).

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Is the child under 6 years old or under 25 kg?

YES -> Administer **0.5 mL** (half the vial).

NO -> Administer **1.0 mL** (full vial).

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Step 11: Place the child in the recovery position (on their side) to protect the airway.

This prevents choking if the child vomits. Monitor breathing continuously.

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Step 12: Call MDA at 101 immediately.

Always call 101 after administering glucagon -- the child requires follow-up medical care. Glucagon effect begins approximately 10-15 minutes after injection and lasts approximately 30 minutes.

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Step 13: Once the child regains consciousness and can swallow safely, provide complex carbohydrates.

Use the 10-15 minute window after glucagon takes effect to give the child carbohydrate-rich food (bread with cheese, crackers, fruit). This replenishes liver glycogen stores depleted by the glucagon.

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Has the child regained consciousness and can swallow safely?

YES -> Provide complex carbohydrates and continue monitoring until MDA arrives.

NO -> Maintain recovery position, monitor breathing, and wait for MDA.

HYPERGLYCEMIA / DIABETIC KETOACIDOSIS (DKA)

Step 14: Assess for signs of hyperglycemia or diabetic ketoacidosis (DKA).

If the emergency is suspected hyperglycemia rather than hypoglycemia, look for: excessive thirst, frequent urination, nausea/vomiting, abdominal pain, deep rapid breathing (Kussmaul breathing), fruity/acetone-smelling breath, weakness, and lethargy progressing to decreased consciousness. Hyperglycemia is not managed by first aiders.

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Are signs of DKA present (vomiting, abdominal pain, rapid deep breathing, fruity breath, decreased consciousness)?

YES -> Call 101 (MDA) immediately. This is a medical emergency requiring hospital treatment. Do NOT administer insulin.

NO -> The child should still be evaluated at a medical facility. Arrange transport to hospital.

WHEN IN DOUBT

Step 15: If uncertain whether the child has hypoglycemia or hyperglycemia: treat for hypoglycemia by giving sugar.

Giving sugar to a hypoglycemic child is life-saving. Giving sugar to a hyperglycemic child will not cause immediate harm. The reverse is not true -- withholding sugar from a hypoglycemic child can be fatal. This is the 'when in doubt, give sugar' principle.

WARNING: This rule applies only when the child is conscious and can swallow safely. Never give anything by mouth to an unconscious child.

DO NOT:

X DO NOT administer insulin -- this is strictly a medical intervention and can be fatal if given inappropriately. First aiders should never give insulin.
X DO NOT give food, drink, or oral medication to an unconscious or seizing child -- this is a choking hazard.
X DO NOT use chocolate as the initial sugar source -- fat and protein slow absorption and delay correction.
X DO NOT administer glucagon unless you are trained and authorised to do so.
X DO NOT delay calling 101 (MDA) for severe hypoglycemia, seizures, or loss of consciousness.
X DO NOT attempt to manage hyperglycemia or DKA with first aid measures -- this requires medical intervention.
X DO NOT use diet drinks or sugar-free products as a sugar source for hypoglycemia treatment.

Equipment needed:

— Glucose gel syringe (15g glucose)
— Fast-acting sugar source (juice, glucose tablets, sugar cubes, honey)
— Complex carbohydrate snack (bread, crackers, fruit)
— Glucagon emergency kit (Hypo-Kit) if available and authorised to use
— Blood glucose meter (glucometer) if available

Personal reference only — not medical advice. Always call 101 in an emergency.

DROWNING — FIRST AID (CHILD)

Israel — Child (1 year to puberty)

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :(מד"א | **Police (100 :(משטרה** | **Fire (102 :(כיבוי** | **Hatzalah (1221 :(הצלה**

Key principle: Drowning is a **hypoxic emergency** — rescue breaths are CRITICAL. Compression-only CPR is **insufficient** for drowning victims.
Lone rescuer: Perform **2 minutes of CPR first** (5 cycles of 30:2), then call 101.
Signs: Unconsciousness, absence of breathing, cyanosis (blue skin), limp body, water in/around the airway.

Step 1: Ensure scene safety. Do NOT enter the water unless you are a trained water rescuer.

Use reaching aids (pole, rope, towel) or throwing aids (flotation device) to bring the child to safety. If you must enter water, bring a flotation device.

WARNING: Do NOT enter the water without a flotation device or proper training. You risk becoming a victim.

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Step 2: Remove the child from the water as quickly as possible without risking your own life.

Support the head and neck during removal if spinal injury is suspected (diving accident, fall from height, or unknown mechanism).

Is spinal injury suspected (diving accident, fall, visible head/neck injury)?

YES -> Immobilise head and neck in neutral position during extraction. Maintain spinal alignment. Avoid twisting or bending the neck.

NO -> Remove the child from the water as quickly as possible and place on a firm, flat surface.

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Step 3: Assess responsiveness. Tap the child's shoulder firmly and call out loudly: 'Are you okay?'

Is the child responsive (moving, coughing, breathing)?

YES -> Place in recovery position (Step 9). Call 101. Monitor closely — **always transport to hospital** due to secondary drowning risk.

NO -> Child is unresponsive. Place on back on a firm, flat surface away from the water's edge. Proceed to Step 4.

Drowning — Child — Steps 4-8

Call for Help — Airway — Rescue Breaths — CPR

Step 4: Call for help. Instruct a bystander to call 101 (MDA) immediately.

If a second person is present, have them call 101 while you begin resuscitation. If alone, perform 2 minutes of CPR first (5 cycles of 30:2), then call 101.

Is a second person available to call 101?

YES -> Direct them to call 101 immediately while you proceed to Step 5.

NO -> Perform 2 minutes of CPR first (5 cycles of 30:2), then call 101. Return to CPR immediately after the call.

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Step 5: Open the airway. Gently tilt the child's head back and lift the chin (head-tilt chin-lift).

Look, listen, and feel for breathing for 5-10 seconds: observe chest rise, listen for breath sounds, feel for air on your cheek.

Is the child breathing normally?

YES -> Place in recovery position (Step 9). Continue monitoring until MDA arrives. **Always transport to hospital.**

NO -> The child is not breathing. Begin CPR immediately (Step 6).

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Step 6: Give 2 initial rescue breaths. Drowning is a hypoxic emergency — rescue breaths are CRITICAL.

Pinch the child's nose closed. Seal your mouth over the child's mouth. Give 2 gentle rescue breaths, each lasting approximately 1 second. Watch for visible chest rise with each breath.

WARNING: Compression-only CPR is insufficient for drowning victims. Rescue breaths are essential because drowning is primarily an oxygen-deprivation emergency.

Does the chest rise visibly with each rescue breath?

YES -> Breaths are effective. Proceed to chest compressions (Step 7).

NO -> Reposition the head (re-tilt and re-lift chin) and try again. If chest still does not rise, proceed to compressions — compressions may help dislodge water or foreign material.

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Step 7: Begin chest compressions.

Place the heel of one hand on the centre of the chest, between the nipples, on the lower third of the sternum. Compress to a depth of 1/3 to 1/2 the depth of the chest. Rate: 100-120 compressions per minute. Allow full chest recoil. Count compressions aloud.

WARNING: Avoid compressing over the xiphoid process (bottom tip of sternum). Allow full chest recoil — do not lean on the chest between compressions.

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Step 8: Continue CPR cycles using the correct compression-to-breath ratio.

Solo rescuer: 30 compressions + 2 rescue breaths. Two rescuers: 15 compressions + 2 rescue breaths. Perform cycles continuously. Switch rescuers every 2 minutes (5 cycles) if two rescuers are available. Minimise interruptions — avoid pauses longer than 10 seconds.

Does the child show signs of life (breathing, coughing, movement)?

YES -> Stop compressions. Place in recovery position (Step 9). Continue monitoring breathing until MDA arrives.

NO -> **Continue CPR** without interruption until MDA arrives, an AED becomes available, or you are physically unable to continue.

Drowning — Child — Steps 9-10

Recovery Position — Hospital Transport — Do Not List

Step 9: Recovery position: if the child resumes spontaneous breathing, place them on their side.

Place the child on their side with the upper knee bent forward to stabilise the position. This maintains airway patency and allows water or vomit to drain from the mouth. Continue monitoring breathing until MDA arrives. Maintain warmth — children are particularly vulnerable to hypothermia after water immersion.

WARNING: Even if the child appears to recover fully, ALWAYS transport to hospital. Secondary drowning (delayed pulmonary oedema) and dangerous electrolyte imbalances can develop hours after the event.

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Step 10: Hospital transport is mandatory. All drowning victims must be evaluated at a hospital.

Secondary drowning (delayed pulmonary oedema) can develop hours after a drowning event, even if the child initially appears to recover. Electrolyte imbalances can also occur. Do not assume the child is fine based on initial recovery.

DO NOT:

X Do NOT attempt to remove water from the lungs — this is impossible and can endanger the victim.
X Do NOT perform abdominal thrusts (Heimlich manoeuvre) to expel water — this is not effective for drowning and risks aspiration of stomach contents.
X Do NOT delay CPR to attempt water removal.
X Do NOT use compression-only CPR for drowning — rescue breaths are essential because drowning is a hypoxic (oxygen-deprivation) emergency.
X Do NOT assume the child is fine if they regain consciousness — always transport to hospital due to risk of secondary drowning.
X Do NOT enter the water to rescue the child unless you are a trained water rescuer with a flotation device.
X Do NOT use excessive ventilation force or volume — this can cause gastric distention (stomach inflation) in children.

Equipment needed (if available):

- Reaching aid (pole, rope, towel) or throwing aid (flotation device) for water rescue
- Firm, flat surface for CPR
- Blanket or covering to maintain warmth (hypothermia prevention)

ELECTROCUTION — FIRST AID (CHILD)

Israel — Child

ID: IL-CHILD-ELECTROCUTION-001

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א) **Police (100 :משטרה)** **Fire (102 :כיבוי)** **Hatzalah (1221 :הצלה)**

When to apply:

When a child has sustained an electric shock from any source: household outlet (220V in Israel), electrical cord (especially oral contact/biting in toddlers), faulty appliance, exposed wiring, or high-voltage source. Signs include burns at contact points, loss of consciousness, muscle rigidity or spasms, difficulty breathing, cardiac arrest, or the child being found unresponsive near an electrical source.

Summary: First aid for electrical shock injuries in children, emphasising scene safety (disconnect power before touching the child), CPR readiness, burn care, and mandatory hospital evaluation for all cases.

Step 1: Ensure your own safety -- do NOT touch the child while they are in contact with the electrical source.

Touching a person who is still connected to an electrical source will cause chain electrocution. Assess the scene before approaching. Stand on a dry, insulating surface if possible.

WARNING: You will become an additional victim if you touch the child while they are still in contact with the electrical source. Scene safety is the absolute first priority.

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Step 2: Disconnect the electrical source.

The safest action is to turn off the main circuit breaker (#heb[מפסק ראשי]) or unplug the appliance at the wall socket. If you cannot reach the power source, use a long, dry, non-conductive object (wooden broom handle, dry wooden stick, plastic chair, dry rolled newspaper) to separate the child from the electrical source.

WARNING: Never use metal objects or anything wet or damp to separate the child from the source.

Is this a high-voltage source (power lines, transformer box, industrial equipment)?

YES -> **Do NOT approach.** Call 101 (MDA), 102 (Fire Services), and 103 (Israel Electric Corporation) immediately. Maintain a safe distance and keep bystanders away until emergency services arrive.

NO -> Turn off the main circuit breaker or unplug the appliance. If that is not possible, use a dry non-conductive object to separate the child from the source.

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Step 3: Call 101 (MDA) immediately once the child is separated from the electrical source.

Provide clearly: your phone number, exact address or location, your name, the child's approximate age, what happened (electrocution, type of source), and the child's current condition (conscious or unconscious, breathing or not breathing). MDA dispatchers can guide you through first aid steps while the ambulance is en route.

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Step 4: Assess the child's responsiveness, breathing, and pulse.

Call the child's name and tap gently to check responsiveness. Look for chest rise and listen for breath sounds to check breathing. Check for a pulse.

Is the child responsive?

YES -> Keep the child still and calm. Proceed to step 6 (burn assessment) and step 7 (treat for shock).

NO -> Check breathing and pulse immediately. If the child is not breathing or has no pulse, proceed to step 5 (CPR).

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Step 5: Begin CPR immediately if the child is not breathing and/or has no pulse.

Child CPR (age 1--8): 30 compressions to 2 breaths; compress to a depth of approximately 5 cm using one or two hands on the lower half of the sternum. Infant CPR (under age 1): 30 compressions to 2 breaths; compress to a depth of approximately 4 cm using two fingers on the sternum, just below the nipple line. Continue CPR without stopping until MDA paramedics arrive.

WARNING: Do not stop CPR based on apparent lack of vital signs. Electrocution victims can sometimes be resuscitated after prolonged efforts.

Is an AED available?

YES -> Apply the AED as soon as possible. Use pediatric pads if available; if only adult pads are available, use them. Follow AED voice prompts. Continue CPR between shocks.

NO -> Continue manual CPR (30 compressions to 2 breaths) without interruption until MDA paramedics arrive.

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Step 6: Assess and treat burns at current entry and exit points.

Look for burns where the current entered and exited the body. Cool visible burns with cool (not ice-cold) running water. Cover with sterile, non-adherent dressing. Remember: electrical burns are often internal and far more severe than what is visible on the skin surface.

WARNING: Do NOT apply creams, ointments, toothpaste, or home remedies to electrical burns. Do NOT apply ice.

Is the burn an oral electrical burn (from biting or sucking on an electrical cord)?

YES -> The burn may appear as a localised white or grey area on the lip, tongue, or inside the mouth. Cool the area if possible. Do NOT apply pressure to the wound. **Warn parents about delayed bleeding risk:** bleeding from the labial artery can occur hours or even days after the initial injury. If bleeding occurs later, apply firm pressure and call 101 immediately. Requires immediate medical evaluation.

NO -> Cool visible burns with cool running water. Cover with sterile, non-adherent dressing. Do NOT apply creams or ointments.

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Step 7: Treat the child for shock (#heb[הלם]).

Lay the child down if conscious and no spinal injury is suspected. Keep the child warm with a blanket. Elevate legs slightly if no spinal injury is suspected. Do not give food or drink.

Is spinal injury suspected (child was thrown or fell during electrocution)?

YES -> Do NOT move the child. Keep the child completely still and wait for MDA paramedics. Support the head and neck in the position found.

NO -> Lay the child down, keep warm with a blanket, and elevate legs slightly.

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Step 8: Minimise movement of the child.

Electrocution can cause involuntary muscle contractions that throw the child, potentially causing spinal or other traumatic injuries. Avoid unnecessary movement. If spinal injury is suspected, keep the child completely still and wait for MDA.

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Step 9: Ensure the child is transported to hospital for evaluation -- this is MANDATORY.

Every child who experiences an electric shock, even if it appears minor, MUST be evaluated at a hospital. Internal injuries, cardiac arrhythmias, and organ damage (particularly kidneys from rhabdomyolysis) may not be immediately apparent. Delayed complications can occur hours after the incident. ECG monitoring is typically required to check for cardiac rhythm abnormalities. For oral electrical burns, warn parents about delayed bleeding from the labial artery which can occur hours or even days later.

WARNING: Do NOT assume the child is fine because external injuries appear minor. Internal damage may be hidden. A seemingly small and superficial skin injury may conceal deep internal damage.

DO NOT:

X Do NOT touch the child while they are in contact with the electrical source -- you will become an additional victim (chain electrocution).
X Do NOT use metal objects or wet/damp materials to separate the child from the electrical source.
X Do NOT apply ice to electrical burns.
X Do NOT apply creams, ointments, toothpaste, or home remedies to electrical burns.
X Do NOT assume the child is fine because external injuries appear minor -- internal damage from electrical current may be hidden and far more severe than surface injuries suggest.
X Do NOT delay calling 101 -- every electrocution in a child requires professional medical evaluation and hospital assessment.
X Do NOT move the child unnecessarily -- spinal and traumatic injuries from being thrown or muscle contractions may be present.
X Do NOT approach high-voltage sources (power lines, transformer boxes) -- call 101, 102, and 103 and maintain a safe distance.
X Do NOT stop CPR based on apparent lack of vital signs -- electrocution victims can sometimes be resuscitated after prolonged efforts.
X Do NOT give food or drink to a child in shock after electrocution.

Equipment Needed:

- Non-conductive object for separating victim from source (wooden broom handle, dry stick, plastic chair)
- AED / defibrillator (with pediatric pads if available)
- Cool running water source (for burn cooling)
- Sterile non-adherent dressings (for burn coverage)
- Blanket (for warmth and shock treatment)

EYE INJURIES — CHILD

Israel — Child

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :(מד"א	**Police (100 :(משטרה**	**Fire (102 :(כיבוי**	**Hatzalah (1221 :(הצלה**

When to call: Call MDA at 101 for any penetrating eye injury, chemical burn to the eye, significant blunt trauma with vision changes, or any eye injury where the child is in severe distress. Alternatively, call United Hatzalah at 1221.

Step 1: Assess the type of eye injury.

Determine if the injury is from a chemical substance, foreign object, penetrating object, blunt trauma, or thermal/UV burn. The treatment approach differs significantly based on the type of injury.

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Is this a chemical burn to the eye?

YES: Proceed immediately to Step 2 (chemical burn irrigation). Time is critical – begin irrigation within seconds.

NO: Proceed to Step 3 to assess for other injury types.

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Step 2: For chemical burns: irrigate the eye immediately with clean flowing water for 15 to 30 minutes.

Tilt the child's head so water runs from the inner corner of the affected eye outward, away from the unaffected eye. Use sterile water if available, otherwise tap water. Hold eyelids open during irrigation. For young children who resist, two adults may be needed -- one to hold the child securely, one to irrigate. Continue irrigation for a full 15-30 minutes even if the child protests. After irrigation, transport urgently to hospital.

WARNING: Do NOT delay irrigation to call emergency services. Begin irrigation immediately and have someone else call 101. Identify the chemical substance and bring the container or its MSDS to hospital if possible.

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Step 3: For foreign objects: determine if the object is embedded or superficial.

Ask the child (if old enough) what happened. Look for visible objects on the surface of the eye or under the eyelid.

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Is the foreign object embedded in the eye or visibly penetrating the surface?

YES: Do NOT attempt removal. Cover both eyes with sterile dressings without applying pressure. Transport to hospital immediately. Proceed to Step 4.

NO: Attempt gentle flushing with clean lukewarm water for approximately 15 minutes to dislodge the object. Even after successful removal, seek ophthalmological evaluation.

EYE INJURIES — CHILD (continued)

Penetrating Injuries, Blunt Trauma & Transport

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א) | **Police** (100 :משטרה) | **Fire** (102 :כיבוי) | **Hatzalah** (1221 :הצלה)

Step 4: For penetrating eye injuries: protect the eye without pressure.

Do NOT apply any pressure to the eye. Do NOT apply bandages. Place a rigid shield (such as a paper cup or plastic eye shield) over the injured eye without the shield touching the eye. Position the child supine (lying on back) for transport. Cover the uninjured eye as well to reduce sympathetic eye movement.

WARNING: Do NOT apply pressure or bandage to a penetrating eye injury -- this worsens the wound and can cause further damage. Do NOT attempt to remove any embedded object.

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Step 5: For blunt trauma: apply a cold compress gently.

Apply a cold compress (ice wrapped in cloth) gently over the closed eyelid for 10-15 minutes. Do not press firmly. Monitor for vision changes, increasing pain, or swelling. Seek medical evaluation.

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Does the child report vision changes, double vision, or severe pain?

YES: Call 101 immediately. This may indicate internal eye bleeding or retinal damage requiring urgent hospital evaluation.

NO: Continue cold compress and seek routine ophthalmological evaluation within 24 hours.

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Step 6: Calm and reassure the child throughout treatment.

Approach calmly. Explain procedures honestly using age-appropriate language. Allow the child to sit with a parent during assessment when possible. Children may not report vision changes accurately -- observe behaviour for signs of visual difficulty (bumping into objects, squinting, covering one eye).

WARNING: Do NOT rub the affected eye. Instruct the child not to rub their eye.

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Step 7: Transport to hospital with on-call ophthalmologist.

All significant eye injuries in children require hospital evaluation. For penetrating injuries, transport supine. Bring the chemical container or its MSDS if applicable. Note the time of injury and substances involved for medical staff.

DO NOT:

X Rub the injured eye -- worsens damage from foreign objects and chemical burns.
X Apply pressure or bandages to a penetrating eye injury.
X Attempt to remove an embedded object from the eye.
X Delay irrigation for chemical burns -- begin immediately.
X Use any eye drops or ointments without medical direction.
X Allow the child to rub or touch the injured eye.
X Apply antiseptic or alcohol to the eye.

Equipment needed:

— Clean water source (tap water or sterile water for irrigation)
— Rigid eye shield or paper cup
— Sterile gauze or clean cloth
— Cold compress or ice wrapped in cloth
— Phone (to call 101)

FRACTURES AND SPRAINS — FIRST AID (CHILD)

Israel — Child

ID: IL-CHILD-FRACTURES-001

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א) **Police (100 :משטרה)** **Fire (102 :כיבוי)** **Hatzalah (1221 :הצלה)**

Call 101 immediately for:

Open fractures (bone visible through skin); suspected spinal or neck injury; suspected hip or pelvis fracture; loss of sensation or circulation below the injury; severe limb deformity; heavy bleeding; child is unconscious or having difficulty breathing; multiple injuries; any fracture in an infant or very young child.

Summary: Immobilise the injury without realigning, apply cold in 15-minute intervals, manage bleeding for open fractures, keep the child calm, and call 101 (MDA) for serious injuries.

When to apply: Suspected fracture or sprain in a child: severe pain worsening with movement; visible swelling and bruising; limb deformity; inability to move or bear weight; tenderness when touched; audible snap or crack; child refuses to use the limb. In young children: sudden refusal to use an arm or leg after a fall, inconsolable crying, or unexplained irritability.

Step 1: Stay calm and reassure the child.

Emotional management is critical. A frightened child in pain will tense muscles, increasing pain. Speak calmly at the child's eye level. Explain simply what you are doing. Do not display panic -- children take cues from adults. Allow a parent or trusted adult to stay close. Use distraction techniques (talking, counting, singing).

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Step 2: Assess the injury and determine severity.

Look for: severe pain, swelling, bruising, limb deformity, inability to move the limb, tenderness, and whether bone is visible through the skin. In young children, watch for refusal to use a limb, inconsolable crying, and irritability. Note: what appears to be a sprain in a child may actually be a growth plate fracture.

Is bone visible through the skin (open/compound fracture)?

YES -> **Medical emergency.** Call 101 (MDA) immediately. Proceed to Step 8 for open fracture management, then continue with immobilisation.

NO -> Proceed to Step 3.

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V

Step 3: Call MDA at 101 if the injury meets emergency criteria.

Put the phone on speaker so you can follow dispatcher instructions while providing care. Describe: the child's age, location and nature of the injury, whether bone is visible, level of consciousness, and any bleeding.

Is there suspected spinal or neck injury (e.g., fall from height, diving accident, traffic accident)?

YES -> **Do NOT move the child at all.** Call 101 immediately. Hold the child's head steady to prevent movement. This is a life-threatening emergency. Do not proceed with other steps – wait for professional help.

NO -> Proceed to Step 4.

|
V

Step 4: Do NOT move or realign the injury.

NEVER attempt to straighten, pull, or push a bone back into place. MDA principle: 'Never try to straighten the limb, pull it, or return a bone to its place' (#heb[לעולם אל תנסו ליישר את הגפה, למשוך אותה או להחזיר עצם למקומה]). If the limb is in an unnatural position, leave it as found.

WARNING: Attempting to realign a bone can damage arteries, nerves, and growth plates, potentially causing permanent harm to the child's future bone development.

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V

Step 5: Immobilise the injury.

MDA principle: 'Minimum movement, maximum stability' (#heb[תנועה מינימלית, יציבות מקסימלית]). The best immobilisation for non-professionals is immobilisation to the body itself or to a stable surface. If you are not a certified medic, do not try to build complex splints.

Is the injury to an arm or to a leg?

YES -> ARM: Support the injured arm using the child's healthy hand. Create an improvised sling using a shirt, scarf, sweater, or fabric folded into a triangle. Place the injured arm inside and tie around the child's neck. Secure the sling to the chest to prevent swinging.

NO -> LEG: Ask the child not to move the leg. Place pillows, rolled blankets, towels, or clothing on both sides to prevent movement. Gently bandage the injured leg to the healthy leg with padding between them. For suspected hip fractures: do NOT move the child; call for ambulance immediately.

|
V

Step 6: If using an improvised splint, apply it correctly.

Improvised splints can be made from cardboard, rolled newspapers, magazines, rulers, or any rigid material. The splint must extend beyond the joints above and below the fracture. Pad the splint with soft material (clothing, towels). Tie in place with fabric strips, scarves, or bandages -- firm but not tight enough to restrict circulation.

WARNING: Check fingers or toes below the splint for warmth, colour, and sensation. If they turn blue, cold, or numb, loosen the splint immediately.

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V

Step 7: Apply cold to reduce pain and swelling.

Apply an ice pack or cold compress wrapped in a towel -- never apply ice directly to skin. Apply for 15-minute intervals, then remove for 15 minutes before reapplying. Be especially gentle with children and explain that it may feel cold but will help. In Israel's hot climate, cold packs or a bag of frozen vegetables wrapped in cloth can be used.

WARNING: Never apply ice directly to the child's skin. Always wrap in cloth to prevent cold burns.

|
V

Step 8: Manage bleeding if this is an open (compound) fracture.

If bone is protruding through the skin, this is a medical emergency. Control bleeding through indirect pressure around (not on) the wound. Cover the exposed bone with a sterile dressing or the cleanest available cloth. Do NOT attempt to push the bone back in. Keep the MDA dispatcher on speaker for continuous guidance.

WARNING: Do NOT push protruding bone back into the wound. Do NOT apply pressure directly on the wound site. Apply a tourniquet only if haemorrhage is severe and uncontrollable.

Is the bleeding severe and uncontrollable despite indirect pressure?

YES -> Apply a tourniquet if available (MDA protocol for severe uncontrollable haemorrhage). Maintain contact with MDA dispatcher.

NO -> Continue indirect pressure and cover the wound. Keep the limb immobilised and wait for emergency services.

|
V

Step 9: Remove constrictive items from the injured area.

Remove jewellery, watches, bracelets, and rings from the affected hand or fingers immediately. Remove shoes from an injured foot or ankle. Swelling occurs rapidly after a fracture, and constrictive items can cut off circulation.

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V

Step 10: Prevent shock.

Keep the child warm with a blanket or clothing (but avoid overheating in Israel's climate). Keep the child lying down if possible. Monitor breathing and consciousness.

Is there suspected spinal injury or is the child vomiting?

YES -> Do NOT elevate the legs. Keep the child in the position found (spinal injury) or on their side (vomiting). Continue monitoring.

NO -> Elevate the legs slightly to help prevent shock. Continue monitoring breathing and consciousness.

|
V

Step 11: Do not give the child food or drink.

Avoid giving the child food or drink, as surgery under general anaesthesia may be required. If the child is very thirsty, moisten their lips with a wet cloth.

Step 12: Monitor the child until emergency services arrive or you reach the hospital.

Continue to reassure the child. Monitor for signs of shock (pale skin, rapid breathing, confusion). Keep the injured limb immobilised. If transporting to hospital yourself (for less severe injuries), ensure the limb remains immobilised during transport. Any suspected sprain in a child should be evaluated by a doctor to rule out growth plate injury.

DO NOT:

X Do NOT attempt to straighten, pull, or realign a broken bone -- this can damage arteries, nerves, and growth plates, potentially affecting future bone development.
X Do NOT move the child unnecessarily, especially if spinal injury is suspected.
X Do NOT apply heat to the injury (no hot compresses, no warm water).
X Do NOT apply ice directly to the child's skin -- always wrap in cloth first.
X Do NOT tie splints or bandages too tightly -- monitor circulation (warmth, colour, sensation) below the injury.
X Do NOT allow the child to bear weight on an injured leg.
X Do NOT remove clothing by pulling over the injury -- cut it away if necessary.
X Do NOT give aspirin to children (risk of Reye's syndrome); use paracetamol (Acamol) or ibuprofen (Nurofen for Children) if needed for pain.
X Do NOT push a protruding bone back into the wound in an open fracture.
X Do NOT give the child food or drink -- surgery under anaesthesia may be required.

Equipment Needed:

- Improvised sling material (shirt, scarf, sweater, or triangular bandage)
- Improvised splint material (cardboard, rolled newspapers, magazines, rulers, or rigid material)
- Padding material (clothing, towels, blankets) for splint and leg immobilisation
- Ice pack or cold compress (or bag of frozen vegetables)
- Cloth or towel for wrapping ice pack
- Sterile dressings or clean cloth (for open fractures)
- Bandages or fabric strips for securing splints
- Blanket for warmth and shock prevention
- Tourniquet (for severe uncontrollable bleeding in open fractures, if available)

HEAD & SPINAL INJURY — CHILD

Israel — Child

Suspected Head or Spinal Cord Injury in a Child

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א) **Police (100 :משטרה)** **Fire (102 :כיבוי)** **Hatzalah (1221 :הצלה)**

When to apply: Falls from height exceeding 1 m (or more than twice the child's height), vehicle/bicycle/scooter accidents, falls from playground equipment, struck by heavy object, blast/shrapnel exposure, any fall onto hard surface with direct head impact in infant under 1 year, penetrating head wound. Also when the child shows: loss of consciousness, seizures, repeated vomiting, unequal pupils, confusion/irritability/excessive drowsiness, clear fluid or blood from nose/ears, limb weakness/numbness, or bulging fontanelle in infants.

Step 1: Ensure scene safety.

Check that the area is safe for you and the child before approaching. Wear gloves if available. In blast or security events, confirm the scene is clear before entering.

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Step 2: Assess the child's responsiveness without moving them.

Gently tap the child and call their name. For infants, tap the sole of the foot. Use the AVPU scale: A (Alert), V (responds to Voice), P (responds to Pain), U (Unresponsive).

WARNING: Do NOT shake the child. Do NOT move the child unless they are in immediate danger (fire, traffic, structural collapse).

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Is the child responsive?

YES: Calmly instruct the child to stay completely still. Reassure them with eye contact and a calm voice. Proceed to Step 3.

NO: Check for breathing using the jaw thrust technique to open the airway (do NOT use head-tilt chin-lift). Proceed to Step 3 and call 101 immediately.

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Step 3: Call 101 (Magen David Adom) immediately.

Always call 101 for any suspected head or spinal injury in a child. Provide your location, the child's approximate age, the mechanism of injury, current symptoms, and level of consciousness (AVPU). Even if the child appears alert and well, call 101 for any high-risk mechanism.

WARNING: Do not delay calling 101. Children can deteriorate rapidly after head injury. A child who appears fine initially may have a developing intracranial bleed.

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Step 4: Keep the child completely still -- do NOT move them.

This is the single most important instruction. If the child is lying down, do not sit them up. If the child is sitting, do not lay them down unless they become unconscious and need airway management. Maintain the child in the position found.

WARNING: Moving a child with a spinal injury can cause permanent neurological damage or paralysis. Standing or walking ability does NOT exclude spinal cord injury.

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Is the child in immediate danger (fire, traffic, structural collapse)?

YES: Move the child only the minimum distance necessary to reach safety, keeping the head, neck, and spine aligned as much as possible.

NO: Do not move the child at all. Keep them in the position found and proceed to Step 5.

Source: Magen David Adom (MDA) | MDA BLS Guide (2016), MDA ALS Professional Manual (2024), and Israeli Standard-of-Care Paediatric Head Injury Guidance
Imported: 2026-03-15 · Last verified: 2026-03-15
Personal reference only – not medical advice. Always call 101 in an emergency.

HEAD & SPINAL INJURY — STABILISATION & AIRWAY

Israel — Child

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א) **Police (100 :משטרה)** **Fire (102 :כיבוי)** **Hatzalah (1221 :הצלה)**

Step 5: Stabilise the head and neck manually (manual in-line stabilisation).

Place your hands on either side of the child's head and hold it steady, aligned with the spine. Do not twist, bend, or rotate the head or neck. Maintain this position continuously until MDA paramedics arrive and take over. Two providers are ideal if available.

WARNING: Do NOT attempt to straighten the neck if it is in an abnormal position. Hold it in the position found.

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Step 6: Manage the airway if the child is unconscious, using the jaw thrust manoeuvre.

If the child is unresponsive, open the airway using jaw thrust only -- place your fingers behind the angles of the lower jaw and lift the jaw forward. Do NOT use head-tilt chin-lift, as this can worsen a cervical spine injury. Position yourself near the child's face.

WARNING: Never use head-tilt chin-lift on a child with suspected spinal injury. Jaw thrust is the only safe airway manoeuvre.

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Is the child breathing after jaw thrust?

YES: Maintain the jaw thrust and manual cervical stabilisation. Monitor breathing continuously. Proceed to Step 7.

NO: Begin CPR per MDA protocol (see IL-CHILD-CPR-001), maintaining spinal alignment as much as possible. Continue until MDA arrives or the child resumes breathing.

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Step 7: Control any visible scalp bleeding.

If the scalp is bleeding, apply gentle pressure with a clean cloth or sterile bandage. Scalp wounds can bleed heavily. In children, even moderate scalp bleeding can cause shock due to their smaller blood volume (an infant has only approx. 240 mL total, a one-year-old approx. 800 mL).

WARNING: Do NOT apply direct pressure if you suspect a skull fracture (a depressed or unusually soft area of the skull). Do NOT push back any protruding matter or insert anything into a wound.

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Do you suspect a skull fracture (depressed area, visible bone, clear fluid from ears or nose)?

YES: Do not apply pressure to the wound. Loosely cover the wound with a sterile, non-adherent dressing. Do not attempt to stop the flow of clear fluid from ears or nose – this may be cerebrospinal fluid.

NO: Apply gentle, direct pressure with a clean cloth or sterile bandage to control bleeding.

Source: Magen David Adom (MDA) | MDA BLS Guide (2016), MDA ALS Professional Manual (2024), and Israeli Standard-of-Care Paediatric Head Injury Guidance
Imported: 2026-03-15 · Last verified: 2026-03-15
Personal reference only – not medical advice. Always call 101 in an emergency.

HEAD & SPINAL INJURY — MONITORING & RED FLAGS

Israel — Child

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א) **Police (100 :משטרה)** **Fire (102 :כיבוי)** **Hatzalah (1221 :הצלה)**

Step 8: Assess for red flag warning signs and report to 101 dispatcher or arriving MDA crew.

Check for and note: loss of consciousness (even briefly), seizures or convulsions, repeated vomiting (more than once), unequal pupils (one larger than the other), clear fluid or blood draining from nose or ears, weakness or inability to move any limb, altered behaviour (unusual aggression, lethargy, confusion), bulging fontanelle in infants, and large or rapidly expanding scalp swelling.

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Are any red flag signs present (loss of consciousness, seizures, repeated vomiting, unequal pupils, fluid from ears/nose, limb weakness)?

YES: Communicate these findings urgently to the 101 dispatcher or arriving MDA crew. These signs suggest serious intracranial or spinal cord injury requiring immediate hospital treatment.

NO: Continue monitoring. Even without red flags, maintain immobilisation and await MDA assessment. Children can deteriorate after a delay.

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Step 9: Prevent hypothermia -- cover the child to maintain warmth.

Cover the child with a blanket, coat, or any available covering. Children lose body heat faster than adults due to their higher surface-area-to-body-mass ratio. Hypothermia worsens outcomes in trauma patients. In hot weather, provide shade instead.

WARNING: Do not move the child to cover them. Place the covering over them in their current position.

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Step 10: Monitor continuously until MDA arrives.

Monitor breathing, level of consciousness (AVPU), and any changes in symptoms continuously. Note the time of injury and record any changes. Do not leave the child alone at any point. Keep the child calm and reassured.

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Does the child vomit?

YES: Carefully log-roll the child onto their side while a second person supports head and neck alignment (keeping head, neck, and body moving as one unit). This prevents aspiration. Do the best you can to maintain spinal alignment.

NO: Maintain the child in their current position. Continue monitoring.

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Step 11: Do NOT remove a helmet if the child is wearing one.

If the child was wearing a bicycle, scooter, or other helmet at the time of injury, leave the helmet in place. Helmet removal requires two trained rescuers using a specific technique to avoid worsening a cervical spine injury. Leave this for MDA paramedics.

WARNING: Improper helmet removal can cause catastrophic cervical spine injury. Never attempt to remove a helmet alone.

Source: Magen David Adom (MDA) | MDA BLS Guide (2016), MDA ALS Professional Manual (2024), and Israeli Standard-of-Care Paediatric Head Injury Guidance
Imported: 2026-03-15 · Last verified: 2026-03-15
Personal reference only – not medical advice. Always call 101 in an emergency.

HEAD & SPINAL INJURY — CRITICAL REMINDERS

Israel — Child

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א)	**Police** (100 :משטרה)	**Fire** (102 :כיבוי)	**Hatzalah** (1221 :הצלה)

DO NOT:

X Move the child unless they are in immediate life-threatening danger (fire, traffic, structural collapse)
X Remove a helmet -- leave this for MDA paramedics (requires two trained rescuers)
X Allow the child to stand up or walk, even if they want to -- standing or walking does NOT exclude spinal injury
X Give the child food or drink (they may need surgery or imaging under anaesthesia)
X Apply pressure to a depressed skull fracture or push back any protruding matter
X Insert anything into a wound
X Attempt to straighten the neck if it is in an abnormal position
X Use head-tilt chin-lift to open the airway -- use jaw thrust only
X Shake the child to assess responsiveness
X Leave the child alone -- continuous monitoring is essential
X Stop the flow of clear fluid from ears or nose (may be cerebrospinal fluid)

Equipment needed:

— Disposable gloves
— Clean cloth or sterile bandages for wound control
— Blanket or coat for warmth

Source: Magen David Adom (MDA) | MDA BLS Guide (2016), MDA ALS Professional Manual (2024), and Israeli Standard-of-Care Paediatric Head Injury Guidance | https://www.mdais.org/101/first-aid
Imported: 2026-03-15 · Last verified: 2026-03-15
Personal reference only – not medical advice. Always call 101 in an emergency.
Protocol ID: IL-CHILD-HEAD-SPINAL-INJURY-001

HEART ATTACK / CHEST PAIN

Israel — Child

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א) **Police (100 :משטרה)** **Fire (102 :כיבוי)** **Hatzalah (1221 :הצלה)**

When to apply this protocol:

Persistent, severe, or worsening chest pain (especially during/after activity) – syncope or near-fainting during activity – palpitations – unexplained shortness of breath – sudden collapse during sports – cyanosis (bluish lips/fingertips) – chest pain with fever (possible myocarditis/pericarditis) – any new cardiac symptoms in a child with a known heart condition.

Step 1: Stay calm and reassure the child. Assess for cardiac warning signs.

Look for: persistent or severe chest pain, pain during exertion, syncope or near-syncope, palpitations, unexplained breathlessness, cyanosis, sudden collapse. Red flags: pain during exertion (not at rest), associated syncope/dizziness, known congenital heart condition, family history of sudden cardiac death at a young age, recent viral illness followed by new chest pain/fatigue/breathlessness (possible myocarditis), or direct blow to the chest immediately before collapse (commotio cordis).

Step 2: Call 101 (MDA) immediately.

Describe the child's symptoms, age, and any known medical conditions to the dispatcher. Follow all instructions from the MDA dispatcher. If a bystander is present, have them make the call while you stay with the child.

Step 3: Assess the child's level of consciousness and breathing.

Is the child conscious and breathing?

YES → Position the child comfortably. Proceed to Step 4.

NO → If unconscious but breathing: recovery position (on their side). If not breathing or no pulse: proceed immediately to Step 7 (CPR).

Step 4: Position the conscious child in a comfortable position -- typically sitting up or semi-reclined.

A sitting or semi-reclined position eases breathing. Allow the child to adopt whatever position feels most comfortable.

Step 5: Keep the child calm and monitor continuously.

Loosen tight clothing around the chest and neck. Keep the child warm but not overheated. Do NOT allow the child to eat or drink (in case hospital procedures are needed). Continuously monitor breathing and level of consciousness. Note the time symptoms began and relay this to the ambulance crew.

WARNING: Do NOT give aspirin to the child. Unlike the adult heart attack protocol, aspirin is not indicated for pediatric chest pain and carries a risk of Reye's syndrome. Do NOT give any medications unless specifically instructed by the 101 dispatcher or a physician.

Step 6: Assess whether the situation involves commotio cordis.

Did the child collapse immediately after a direct blow to the chest during sports or physical activity?

YES → Likely commotio cordis – a time-critical emergency. Check responsiveness immediately. If unresponsive and not breathing, begin CPR (Step 7) and use an AED as fast as possible. Defibrillation within 3 minutes gives the best chance of survival.

NO → Continue monitoring the child and wait for emergency services. Keep the child calm and in a comfortable position.

Step 7: If the child becomes unresponsive and is not breathing normally, begin pediatric CPR immediately.

Single rescuer: 30:2 compression-to-ventilation ratio. Two rescuers: 15:2. Compression rate: 100-120 per minute. Compression depth: approximately 5 cm (one-third of chest depth) for children aged 1 year to puberty. After 5 cycles (approximately 2 minutes), call 101 if not already done.

WARNING: Do NOT delay CPR while waiting for an AED. Begin compressions immediately.

Step 8: Use an AED as soon as one is available.

Use pediatric pads or a dose attenuator if available for children under 8 years or under 25 kg. If only adult pads are available, use them -- do not delay defibrillation. MDA has deployed over 4,000 public-access defibrillators across Israel; their locations are available via the MDA app and website.

Are pediatric AED pads available?

YES → Use pediatric pads or dose attenuator. Follow AED voice prompts.

NO → Use adult pads. If pads overlap on a small child, place one on front of chest and one between shoulder blades. Follow AED voice prompts.

Step 9: Continue CPR and AED use until emergency services arrive, the child recovers, or you are too exhausted to continue.

Continue CPR cycles between AED analysis and shocks. If multiple rescuers are present, rotate the compressor role every 2 minutes to maintain quality compressions.

DO NOT:

X Do NOT give aspirin to a child -- carries a risk of Reye's syndrome.
X Do NOT give nitroglycerin or any cardiac medication unless prescribed and directed by a physician.
X Do NOT dismiss chest pain in a child who has recently been ill, has a known heart condition, collapsed during activity, or has a family history of sudden cardiac death.
X Do NOT attempt to drive the child to hospital yourself if cardiac arrest is suspected -- call 101 for an ambulance.
X Do NOT leave the child unattended while waiting for the ambulance.
X Do NOT give the child food or drink (in case hospital procedures are needed).
X Do NOT give any medications unless specifically instructed by the 101 dispatcher or a physician.

Equipment needed:

— AED with pediatric pads or dose attenuator (if available)
— Phone to call 101 (MDA) or 1221 (United Hatzalah)
— MDA app (for AED locator and emergency calling)

HEATSTROKE & HEAT EXHAUSTION IN CHILDREN

Israel — Child

ID: IL-CHILD-HEATSTROKE-001

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

When to apply: When a child shows signs of heat-related illness: intense thirst, headache, profuse sweating, pale clammy skin, nausea, vomiting, dizziness, muscle cramps, floppiness or unusual sleepiness (young children), or more severe signs such as hot dry red skin, confusion, seizures, or loss of consciousness.

Step 1: Assess the child's condition to determine severity.

Check for key signs: Is the child sweating (heat exhaustion) or is the skin hot, dry, and red (heat stroke)? Is the child conscious and responsive? Children aged 0-4 are at highest risk. Young children may become floppy or unusually sleepy rather than showing typical adult symptoms.

Does the child have hot, dry, red skin, body temperature above 40 C, confusion, seizures, or loss of consciousness?

YES -> HEAT STROKE – life-threatening emergency. Call MDA 101 immediately and begin aggressive cooling. Follow all steps in parallel.

NO -> Likely heat exhaustion. Move to shade and begin cooling. Call 101 if no improvement within 30 minutes.

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Step 2: Call MDA at 101 or use the My MDA app.

For heat stroke, call immediately. For heat exhaustion, call if the child does not improve within 30 minutes of cooling. Follow dispatcher instructions until the ambulance arrives.

Is the child showing signs of heat stroke (hot dry skin, temperature above 40 C, altered consciousness, seizures)?

YES -> Call 101 immediately. Do not delay – heat stroke can cause organ damage and death within minutes to hours.

NO -> Begin cooling measures. Monitor the child and call 101 if no improvement within 30 minutes.

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Step 3: Move the child to a cool environment immediately.

Remove the child from the heat source. Move to shade, an air-conditioned room, or the coolest available location. If the child was found in a vehicle, remove them from the vehicle immediately.

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Step 4: Remove excess and tight clothing.

Remove clothing to allow heat to dissipate from the body. Leave only minimal loose clothing.

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Step 5: Begin active cooling.

Pour or spray tepid to cool water (not ice-cold) over the child's body. Fan the child to promote evaporative cooling. Place ice packs or cold wet cloths in the armpits and groin areas, where major blood vessels are close to the skin surface. If available, direct air conditioning or a fan toward the child.

WARNING: Do not use ice-cold water immersion for young children without medical supervision.

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Step 6: Assess the child's level of consciousness.

Determine whether the child is conscious and able to swallow safely.

Is the child conscious, alert, and able to swallow without vomiting?

YES -> Offer small, frequent sips of cool water or oral rehydration solution (ORS). Proceed to Step 7.

NO -> Do NOT give any fluids. If unconscious but breathing, place in recovery position to maintain the airway. Proceed to Step 8.

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Step 7: Provide oral rehydration (conscious children only).

Give small, frequent sips of cool water or oral rehydration solution. Do not force fluids. Stop if the child begins vomiting.

WARNING: Do NOT force fluids if the child is vomiting or has altered consciousness. A vomiting child should not drink due to risk of aspiration.

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Step 8: Maintain the airway.

If the child is unconscious, ensure the airway is open. Place the child in the recovery position if unconscious but breathing. Continue to monitor breathing.

Is the child breathing?

YES -> Maintain the recovery position, continue cooling, and wait for MDA.

NO -> Begin CPR according to paediatric protocol (see IL-CHILD-CPR protocol) and continue until MDA arrives.

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Step 9: Continue cooling and monitoring until MDA arrives.

Do not stop cooling efforts. Continue fanning and applying cool water. Monitor the child's consciousness, breathing, and skin colour. Note the time cooling began to report to the paramedics.

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Step 10: If the child was found in a vehicle, note the circumstances.

Temperatures inside a locked car in Israeli summer can reach 70 C. If a child is found locked in a vehicle, break the window if necessary to extract the child. Report to MDA that the child was found in a vehicle, including estimated time of exposure if known.

WARNING: A child left in a vehicle in Israeli summer conditions is a critical emergency. Every minute counts.

DO NOT:

X Do NOT give fever-reducing medications (antipyretics) such as acetaminophen (Acamol), ibuprofen (Nurofen), or naproxen. These drugs do not reduce heat stroke temperature and may cause harm.
X Do NOT use ice-cold water immersion for young children without medical supervision.
X Do NOT force fluids if the child is vomiting or has altered consciousness.
X Do NOT leave the child unattended at any point.
X Do NOT give fluids to an unconscious child.
X Do NOT delay calling 101 if heat stroke is suspected.
X Do NOT leave a child unattended in a vehicle, even for a moment, even with windows open or air conditioning running.

Equipment needed:

- Cool water (for pouring or spraying over the body)
- Fan or means of fanning
- Ice packs or cold wet cloths
- Oral rehydration solution (ORS) or cool drinking water
- Thermometer (if available)

Source: Magen David Adom (MDA) (2024-04-01) · Last verified: 2026-03-15
Document: MDA Heat Stroke, Dehydration, and Children in Vehicles Guidelines; MDA Summer Emergency Guidelines; MDA ALS Manual 2024
Personal reference only – not medical advice. Always call 101 in an emergency.

HYPOTHERMIA / COLD EXPOSURE — CHILD

Israel — Child (including Infants)

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :(מד"א	**Police (100 :(משטרה**	**Fire (102 :(כיבוי**	**Hatzalah (1221 :(הצלה**

When to call: Call MDA at 101 immediately for any child showing signs of moderate or severe hypothermia (confusion, loss of consciousness, severe shivering or paradoxical cessation of shivering, muscle rigidity). Call for any infant with suspected hypothermia. Alternatively, call United Hatzalah at 1221.

Step 1: Remove the child from the cold environment immediately.

Move the child to a warm, sheltered location. Handle the child very gently -- rough movement can trigger cardiac arrhythmias in a cold heart.

WARNING: Minimise all movement and jostling. A cold heart is extremely sensitive to movement and can develop fatal arrhythmias.

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Step 2: Remove any wet clothing.

Carefully cut or remove wet clothing. Wet clothing accelerates heat loss. Replace with dry clothing or blankets.

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Step 3: Dry the child and wrap in warm blankets.

Dry the child thoroughly. Position the child horizontally. Cover with warmed blankets, focusing on the head, neck, chest, and groin areas where major blood vessels are close to the surface.

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Step 4: Perform ABC assessment (Airway, Breathing, Circulation).

Check airway patency, breathing, and pulse. For infants, maintain nasal patency as they are obligate nasal breathers. Take up to 60 seconds to assess pulse and breathing in hypothermic patients, as vital signs may be very slow.

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Is the child breathing and has a pulse?

YES: Continue rewarming. Monitor breathing and pulse continuously. Place in recovery position if unconscious but breathing.

NO: Begin CPR immediately. Continue CPR and rewarming until emergency services arrive. A hypothermic patient may appear dead but can recover with proper rewarming.

HYPOTHERMIA — CHILD (continued)

Rewarming, Severity Assessment & Transport

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (מד"א): 101 | **Police (משטרה):** 100 | **Fire (כיבוי):** 102 | **Hatzalah (הצלה):** 1221

Step 5: Apply passive and active rewarming to the core body areas.

Apply warmed blankets or heat packs (wrapped in cloth to avoid burns) to the head, neck, chest, and groin. Warm the core first, not the extremities. For infants, ensure thorough coverage of the entire body.

WARNING: Do NOT apply direct heat sources (hot water bottles, heating pads) directly against skin -- risk of burns. Always wrap heat sources in cloth.

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Step 6: Assess severity of hypothermia.

Stage 1 (37-35C): shivering, normal judgement. Stage 2 (35-32C): severe shivering, confusion, slurred speech, pallor. Stage 3 (32-28C): deteriorating consciousness, muscle rigidity, weakening reflexes. Stage 4 (below 28C): unconsciousness, absent reflexes, severely slowed or absent breathing.

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Is the child conscious and able to swallow safely?

YES: Offer warm (not hot) sweet drinks. No alcohol or caffeine.

NO: Do not give any fluids by mouth. Maintain airway and continue rewarming. Transport urgently.

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Step 7: Call 101 (MDA) and transport urgently to hospital.

All children with suspected hypothermia should be evaluated at hospital. During transport, continue to minimise movement and maintain rewarming. Keep the child horizontal during transport.

WARNING: Minimise jostling during transport -- may trigger cardiac arrhythmias.

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Step 8: Monitor continuously until handover to emergency services.

Monitor breathing, pulse, and level of consciousness. Be prepared to perform CPR if the child deteriorates. Note: infants may NOT shiver even when hypothermic -- absence of shivering does not rule out hypothermia in infants.

DO NOT:

X Give alcohol, caffeine, or allow smoking -- these worsen hypothermia.
X Make excessive or rough movements -- risk of cardiac arrhythmias in a cold heart.
X Apply direct heat to skin without a cloth barrier -- risk of burns.
X Rewarm extremities before the core -- can cause dangerous drop in core temperature as cold blood returns to the heart.
X Give fluids by mouth to an unconscious or semi-conscious child -- aspiration risk.
X Assume a severely hypothermic child is dead -- 'no one is dead until they are warm and dead.'
X Use hot water immersion for rewarming in the field -- risk of burns and cardiac arrhythmias.
X Jostle or move the child unnecessarily during transport.

Equipment needed:

— Warm blankets or sleeping bags
— Dry clothing
— Heat packs or warm water bottles (wrapped in cloth)
— Phone (to call 101)

POISONING -- FIRST AID (CHILD)

Israel – Child

ID: IL-CHILD-POISONING-001

CALL 101 (MDA) – CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א)	Police (:משטרה)	Fire (102 :כיבוי)	Hatzalah (:הצלה)	Poison Center:
	100		1221	04-7771900

Call 101 immediately if:

Child is unconscious or losing consciousness; difficulty breathing or respiratory distress; burns visible in or around the mouth; seizures or convulsions; severe vomiting; cyanosis (blue skin colouration); known ingestion of a highly toxic substance (drain cleaner, large quantity of medications).

For non-symptomatic cases: Call the Israel Poison Information Center at **04-7771900** (24/7) for guidance.

Summary: First aid for poisoning in children, covering ingestion, inhalation, skin contact, and eye exposure. Core principle: never induce vomiting, preserve the substance container, and call for professional guidance immediately.

Step 1: Ensure scene safety.

Ensure your own safety before approaching. For inhalation poisoning, do not enter a confined space filled with toxic fumes without protection. If hazardous gases, smoke, or flammable materials are suspected, contact the fire department at 102.

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Step 2: Assess the child's consciousness, breathing, and airway.

Is the child unconscious or not breathing?

YES -> Begin CPR immediately and call 101 (MDA). Follow the pediatric CPR protocol (IL-CHILD-CPR-001).

NO -> Proceed to Step 3.

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Step 3: Determine the route of poisoning exposure.

Identify whether exposure was through ingestion (most common in children), inhalation, skin contact, or eye contact. This determines which treatment path to follow.

Was the substance ingested (swallowed)?

YES -> Proceed to Step 4 (Ingestion protocol).

NO -> Proceed to Step 9 for inhalation, Step 10 for skin contact, or Step 11 for eye contact.

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Step 4: Identify the substance and preserve evidence.

Look for the container, packaging, or remnants of the substance. If substance is still in the child's mouth, attempt to remove it manually by wiping out with a cloth. Collect and preserve the container, packaging, remaining substance, and any vomit for medical teams. Note the estimated amount ingested and the time of ingestion if possible.

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Step 5: Do NOT induce vomiting.

Previous guidelines recommended inducing vomiting, but current Israeli medical protocol explicitly prohibits it. The substance passing back through the oesophagus causes additional chemical burns (for caustic agents), there is risk of aspiration (inhaling vomit into the lungs), and research shows uncontrolled vomiting removes only a partial amount of poison and is ineffective.

WARNING: This is the most critical rule. Do NOT induce vomiting regardless of the substance ingested. Do NOT use finger insertion, soapy water, or salt water to trigger vomiting.

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Step 6: Determine if the substance is caustic (acid or base).

Caustic substances include drain cleaners (acids), toilet bowl cleaners (acids), oven cleaners (bases/alkalis), bleach (economica), degreasers, and limescale removers.

Is the ingested substance a caustic agent (acid, base, bleach, or similar corrosive cleaning product)?

YES -> If the child is conscious and breathing normally: give water or milk to dilute. Toddlers: 60 ml (1/4 cup). Older children: 120 ml (1/2 cup). Adolescents: 240 ml (1 cup). Proceed to Step 7.

NO -> Do NOT give food or liquids unless specifically instructed by the Poison Center. Proceed to Step 7.

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Step 7: Call for help based on symptoms.

Are symptoms present? (altered consciousness, breathing difficulty, mouth burns, vomiting, abdominal pain, seizures, cyanosis)

YES -> **Call 101 (MDA) immediately.** Describe the substance, estimated quantity, time of ingestion, and child's current condition.

NO -> Call the Israel Poison Information Center at **04-7771900** for guidance on whether hospital evaluation is needed. Follow their instructions.

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Step 8: Prepare for transport to hospital.

All poisoning cases require hospital assessment, even if symptoms seem mild. Some toxic substances (e.g., paracetamol) cause no noticeable signs initially but cause severe organ damage (liver, kidneys) that appears later. Always bring the substance container and packaging to the hospital.

WARNING: Do not wait for symptoms before seeking professional guidance. Delayed toxicity is common with many household substances and medications.

ALTERNATIVE EXPOSURE ROUTES

Step 9: Inhalation poisoning: move the child to fresh air.

Move the child to a well-ventilated area or fresh air immediately. If the child is not breathing, begin CPR. Call 101 (MDA). Contact the fire department at 102 if hazardous gases or flammable materials are suspected. For carbon monoxide exposure: signs include headaches, weakness, dizziness, nausea, vomiting, possible loss of consciousness, and facial redness. For organophosphate (pesticide) exposure: signs include excessive secretions, pinpoint pupils, muscle contractions, and breathing difficulty.

WARNING: Never enter a confined space with toxic fumes without proper protection. Never mix bleach-based cleaners with other cleaning products (especially acids) -- this releases toxic chlorine gas causing severe lung irritation and can trigger asthma attacks.

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Step 10: Skin contact: remove contaminated clothing and flush with water.

Remove contaminated clothing carefully. Flush the affected skin area with large amounts of running water for 15-20 minutes. Do not apply creams, ointments, or other substances. Seek medical evaluation.

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Step 11: Eye contact: flush with running water for at least 15-20 minutes.

Flush the eye(s) with large amounts of running water at room temperature for at least 15-20 minutes. Gently lift the eyelids to ensure thorough flushing. Do not rub the eyes. Seek immediate medical evaluation.

DO NOT:

X Do NOT induce vomiting -- regardless of the substance ingested. This is outdated practice and causes additional harm (oesophageal burns from caustic agents, aspiration risk, ineffective removal).
X Do NOT give activated charcoal at home unless specifically instructed by the Poison Center or a physician.
X Do NOT give food to the poisoned child. Water or milk for dilution purposes only, and only for caustic substances when the child is conscious.
X Do NOT attempt home remedies (raw eggs, olive oil, etc.).
X Do NOT wait for symptoms before calling for guidance -- some toxic substances cause no noticeable signs initially but cause severe organ damage (liver, kidneys) later.
X Do NOT discard the container or packaging -- always bring it to the hospital for identification by medical teams.
X Do NOT mix cleaning products -- especially bleach with acids, which releases toxic chlorine gas.
X Do NOT enter a toxic fume environment without proper protection.

Equipment Needed:

- Cloth (to wipe substance from child's mouth if present)
- Water or milk (for dilution of caustic substances only, age-appropriate amounts)
- Running water source (for skin and eye decontamination, 15-20 minutes flushing)
- Container or bag (to preserve substance packaging and any vomit for medical teams)

RECOVERY POSITION — CHILD

Israel — Child (including Infants)

Unconscious, Breathing Casualty

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א) **Police** (100 :משטרה) **Fire** (102 :כיבוי) **Hatzalah** (1221 :הצלה)

When to call: Call MDA at 101 for any unconscious child. Place the child in the recovery position while waiting for emergency services. Alternatively, call United Hatzalah at 1221.

Step 1: Confirm the child is unconscious but breathing normally.

Check responsiveness by speaking loudly and tapping the child's shoulders. Check for normal breathing by looking for chest rise and fall, listening for breath sounds, and feeling for air on your cheek for up to 10 seconds.

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Is the child breathing normally?

YES: Proceed with the recovery position. Call 101 or have someone else call.

NO: If the child is not breathing or only gasping, begin CPR immediately. Do NOT place in recovery position.

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Step 2: Check for suspected spinal injury.

Assess the mechanism of injury. Spinal injury should be suspected if the child fell from a height, was in a vehicle collision, has head or neck trauma, or complains of neck/back pain.

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Is spinal injury suspected?

YES: Do NOT move the child into recovery position. Stabilise the head and spine in the position found. Maintain airway using jaw thrust manoeuvre without tilting the head. Wait for emergency services.

NO: Proceed with the recovery position appropriate for the child's age.

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Step 3: Determine the appropriate recovery position technique based on age.

For children aged 1 to 8 years: use the standard lateral recovery position (same as for adults, performed more gently). For infants under 1 year: use the cradle/arm position.

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Is the child under 1 year of age (infant)?

YES: Use the infant recovery position: hold the infant face-down on your forearm with the head lower than the body, supporting the head and jaw with one hand. Proceed to Step 7 (next page).

NO: Use the standard lateral recovery position for children aged 1-8 years. Proceed to Step 4 (next page).

Personal reference only — not medical advice. Always call 101 in an emergency.

RECOVERY POSITION — CHILD (continued)

Positioning Techniques & Monitoring

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א) **Police (100 :משטרה)** **Fire (102 :כיבוי)** **Hatzalah (1221 :הצלה)**

Step 4: For children aged 1-8: kneel beside the child and position the near arm.

Kneel beside the child. Place the arm nearest to you at a right angle to the body, with the elbow bent and the palm facing upward.

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Step 5: Bring the far arm across the chest and roll the child towards you.

Take the child's far arm and bring it across the chest. Hold the back of their hand against the cheek nearest to you. With your other hand, bend the child's far knee. Roll the child towards you by pulling on the bent knee. Perform this movement gently -- children require less force than adults.

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Step 6: Adjust the position and ensure airway patency.

Adjust the upper leg so that the hip and knee are both at right angles. Tilt the head back gently to keep the airway open. Children have proportionally larger heads -- position carefully to avoid neck flexion which can obstruct the airway. The child should be stable on their side with the mouth angled downward to allow drainage.

WARNING: Children have proportionally larger heads than adults. Ensure the head position does not flex the neck forward, as this can obstruct the airway.

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Step 7: For infants under 1 year: use the cradle/arm recovery position.

Pick up the infant and hold face-down along your forearm, with the head supported by your hand and the head lower than the body. This allows gravity-assisted drainage of fluids from the airway. Support the jaw with your fingers. Do NOT use the standard lateral recovery position for small infants.

WARNING: Do NOT place infants in the standard lateral (side-lying) recovery position. Use the forearm cradle position only.

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Step 8: Monitor breathing continuously until emergency services arrive.

Check breathing regularly (every minute). Ensure the airway remains open. Cover the child with a blanket to prevent hypothermia while in the recovery position. Be prepared to begin CPR if breathing stops.

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Has the child stopped breathing?

YES: Roll the child onto their back and begin CPR immediately.

NO: Continue monitoring in the recovery position. Maintain body warmth and airway patency.

DO NOT:

X Use the recovery position if the child is not breathing -- begin CPR instead.
X Use the recovery position if spinal injury is suspected -- stabilise the spine.
X Place infants under 1 year in the standard lateral recovery position -- use the forearm cradle position.
X Leave the child unattended in the recovery position -- monitor breathing continuously.
X Allow the child's head to flex forward (especially in children with larger heads) -- this can obstruct the airway.
X Forget to cover the child to prevent hypothermia.

Equipment needed:

— Blanket or coat (to prevent hypothermia)
— Phone (to call 101)

Source: Magen David Adom (MDA) (2016-01-01) · Last verified: 2026-03-16
Personal reference only — not medical advice. Always call 101 in an emergency.
Protocol ID: IL-CHILD-RECOVERY-POSITION-001

SCORPION STING — CHILD

Israel — Child

Israel Poison Information Center: 04-7771900 (Rambam Medical Center)

CALL 101 (MDA) — CALL IMMEDIATELY FOR EVERY SCORPION STING IN A CHILD

MDA (101 :(מד"א | **Police (100 :(משטרה** | **Fire (102 :(כיבוי** | **Hatzalah (1221 :(הצלה**

WARNING: Children are at significantly higher risk from scorpion stings due to lower body weight and faster systemic venom progression. The deathstalker scorpion (Leiurus quinquestriatus) can be deadly for young children. All scorpion stings in children require emergency medical evaluation – do not "wait and see." The common belief that black scorpions are harmless is incorrect – black scorpions can also be dangerous and potentially lethal.

Step 1: Ensure scene safety. Move the child away from the scorpion immediately.

Do not attempt to catch or handle the scorpion. If safe to do so, photograph or note the scorpion's appearance (colour, size) for identification at hospital.

WARNING: Do NOT attempt to catch or handle the scorpion. This risks additional stings.

Step 2: Call 101 (MDA) immediately.

Request an ambulance. All scorpion stings in children require emergency medical evaluation -- do not 'wait and see.' Report the child's age, approximate weight, time of sting, sting location on the body, and any symptoms. Also consider calling the Israel Poison Information Center at 04-7771900 for real-time guidance.

WARNING: Do NOT delay calling 101 for children. Children deteriorate faster than adults due to higher venom-to-body-weight ratio.

Step 3: Keep the child calm and still. Have the child lie down or sit still.

Reassure the child by speaking calmly and confidently. Minimise all movement. Movement accelerates venom distribution through the lymphatic and circulatory systems. Anxiety and crying can increase heart rate, which further accelerates venom spread.

WARNING: Immobility is critical. Any movement accelerates venom distribution.

Step 4: Cool the sting area with an ice pack or cold compress wrapped in cloth.

Apply ice or a cold compress wrapped in cloth to the sting site. Cooling relieves pain and slows venom absorption via vasoconstriction. Apply for 10 minutes on, 10 minutes off. Most effective during the first 2 hours following the sting.

WARNING: Do NOT apply ice directly to the child's skin -- always wrap in cloth. This is different from snakebite protocol, where ice is NOT recommended.

Is ice or a cold compress available?

YES Wrap the ice or cold compress in cloth and apply to the sting site. Apply for 10 minutes on, 10 minutes off.

NO Skip cooling. Focus on keeping the child calm, still, and the limb immobilised. Proceed to next step.

Step 5: Immobilise the stung limb. Keep it still and below heart level.

Immobilise the stung limb against the body as you would for a fracture. Keeping it below heart level slows venom distribution.

Is a splint or rigid object available to immobilise the limb?

YES Apply the splint to immobilise the stung limb, keeping it below heart level.

NO Immobilise the limb by positioning it against the child’s body. Use clothing or fabric to secure it in place if possible. Keep it below heart level.

Step 6: Monitor the child closely for signs of systemic envenomation.

Watch for: Neurological -- restlessness, agitation, altered consciousness, drowsiness, confusion, seizures, numbness/tingling, breathing difficulties. Cardiovascular -- rapid heartbeat, irregular rhythms, chest pain. Autonomic -- excessive sweating, salivation/drooling, tearing, vomiting, nausea, abdominal pain, muscle twitching. Delayed reactions may occur -- even if the child initially appears well, hospital observation is essential.

WARNING: Systemic symptoms can progress rapidly in children. Do not assume the child is fine based on initial appearance.

Is the child showing systemic symptoms? (breathing difficulty, altered consciousness, seizures, rapid heartbeat, excessive sweating/salivation, vomiting)

YES Relay all symptoms to the 101 dispatcher immediately. If the child loses consciousness and stops breathing, begin CPR. Continue monitoring and prepare for rapid transport.

NO Continue monitoring closely. Symptoms can develop with delay. Maintain calm, immobilisation, and cooling. Hospital evaluation is still required.

Step 7: Prepare for and facilitate rapid transport to hospital.

All pediatric scorpion stings require hospital evaluation regardless of symptom severity. At hospital, the child may receive: pain relief, supportive treatment for cardiovascular and respiratory systems, monitoring for arrhythmias and delayed reactions, and scorpion antivenom in cases with significant systemic symptoms. Minimum observation period at hospital is 6 hours. If the scorpion was photographed, have the photo ready for medical staff.

Has the ambulance arrived?

YES Hand over to paramedics. Provide the child's age, weight, time of sting, sting location, any symptoms observed, and scorpion photo if available.

NO Continue monitoring, cooling, and keeping the child calm and still. If the child's condition deteriorates rapidly and no ambulance is imminent, consider rapid private transport to the nearest hospital emergency department.

DO NOT:

X Do NOT apply a tourniquet -- concentrates venom in the limb and causes tissue damage.
X Do NOT cut or incise the sting site -- causes additional injury and does not remove venom.
X Do NOT attempt to suck out the venom -- ineffective and risks infection.
X Do NOT apply alcohol or antiseptic to the wound -- interferes with medical assessment.
X Do NOT give the child medication without medical advice -- may interact with venom effects or mask symptoms.
X Do NOT underestimate a sting based on scorpion colour -- black scorpions can be as dangerous as yellow ones and can be potentially lethal.
X Do NOT delay calling 101 for children -- children deteriorate faster than adults due to higher venom-to-body-weight ratio.
X Do NOT apply ice directly to the child's skin -- always wrap in cloth.
X Do NOT allow the child to walk or move -- movement accelerates venom distribution.
X Do NOT 'wait and see' with children -- all scorpion stings in children require emergency medical evaluation.

Equipment needed:

— Ice pack or cold compress (wrapped in cloth)
— Splint or rigid object for limb immobilisation
— Phone (to call 101 / 04-7771900 and photograph the scorpion)
— Cloth or towel (to wrap ice pack)
— Blanket or clothing (to keep child comfortable while lying still)

SEIZURES IN CHILDREN

Including Febrile Seizures

Israel — Child

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :(מד"א | **Police (100 :(משטרה** | **Fire (102 :(כיבוי** | **Hatzalah (1221 :(הצלה**

CALL 101 IMMEDIATELY IF:

- This is the child's **first seizure**
- Seizure lasts **more than 5 minutes**
- Difficulty breathing, choking, or skin discolouration (cyanosis/pallor)
- Child does **not regain consciousness** after seizure ends
- Multiple seizures in a short period
- Anti-seizure medication given but seizure persists

Step 1: Ensure safety. Remove dangerous objects from the child's vicinity.

Move furniture, sharp objects, and hard items away. Pad the area with pillows or blankets if possible.

Step 2: Lay the child on their side with head supported and tilted to the side or slightly downward.

Keeps the airway clear and prevents choking if the child vomits. Do not move the child unless in immediate danger (e.g., near traffic or a ledge).

Step 3: Loosen tight clothing, especially around the neck.

Undo buttons, ties, or anything constricting the airway or chest.

Step 4: Time the seizure. Note when it started and how long it lasts.

Seizure duration is critical for emergency responders. A seizure lasting more than 5 minutes is a medical emergency (status epilepticus).

WARNING: A seizure lasting 5 or more minutes, or multiple consecutive seizures without recovery of consciousness, constitutes status epilepticus. Untreated cases beyond 30 minutes risk permanent brain damage.

Is this the child's FIRST seizure?

YES -> Call 101 (MDA) immediately. A first seizure always requires medical evaluation. MDA will transport to hospital.

NO -> Continue to assess seizure duration and severity (Step 6).

Has the seizure lasted more than 5 minutes, or does the child show difficulty breathing, choking, or skin discolouration (blue/pale)?

YES -> Call 101 (MDA) immediately. This is a medical emergency.

NO -> Continue monitoring. Stay with the child. Proceed to Step 7.

Step 7: If the child has a prescribed seizure-stopping medication (e.g., rectal diazepam), administer it according to the doctor's instructions.

Only administer medication specifically prescribed for this child. Do NOT give any medication by mouth during the seizure.

Has medication been administered but the seizure persists?

YES -> Call 101 (MDA) immediately.

NO -> Continue monitoring until the seizure stops.

Step 8: For febrile seizures: wrap the child in a damp (lukewarm) towel to help reduce fever.

Use lukewarm water only. Do NOT put the child in a bath or shower during the seizure.

WARNING: Do NOT attempt to cool the child rapidly with cold water, ice, or cold baths. Use only lukewarm damp towels.

Step 9: If in a vehicle: keep the child secured in their safety seat.

Pull over safely if driving. Do not remove the child from the safety seat during the seizure.

Step 10: After the seizure stops: place the child in the recovery position (on their side) and monitor breathing.

The child may be drowsy, confused, or sleepy (post-ictal state). This is normal and can last minutes to hours. Stay with the child and provide reassurance.

Does the child regain consciousness and begin to recover?

YES -> Continue to monitor. After a febrile seizure, the child should be examined by a paediatrician. For a first seizure, seek immediate medical evaluation.

NO -> Call 101 (MDA) immediately if not already called. Failure to regain consciousness is a medical emergency.

Step 12: Once recovered (febrile seizure): manage the fever to reduce recurrence risk.

Administer age- and weight-appropriate fever-reducing medication (paracetamol or ibuprofen) as directed by the family doctor. Dress in light clothing. Ventilate the room. Give plenty of fluids. A lukewarm bath (~20 min) may be given only when the child is NOT actively seizing.

WARNING: Antipyretics may improve comfort but have NOT been proven to prevent febrile seizures (per Israeli Ministry of Health). Dosage should be determined by the family doctor.

DO NOT:

X Force the child's mouth open -- the child will not swallow their tongue.
X Insert anything into the mouth -- this can injure teeth or jaw.
X Attempt resuscitation movements (CPR) during the seizure -- this can only cause harm.
X Put the child in a bath or shower during the seizure.
X Splash water on the child during the seizure.
X Try to wake the child or stop the seizure by restraining or holding them down.
X Give medications by mouth during the seizure.
X Attempt to cool the child rapidly with cold water or ice.
X Insert an airway device during an active seizure.
X Attempt forced airway opening during an active seizure.

Equipment needed:

– Lukewarm damp towel (for febrile seizures)
– Pillows or blankets (to pad surroundings)
– Prescribed seizure-stopping medication if available (e.g., rectal diazepam)
– Fever-reducing medication for post-seizure fever management (paracetamol or ibuprofen, as directed by doctor)

Personal reference only — not medical advice. Always call 101 in an emergency.

SEVERE BLEEDING (HEMORRHAGE CONTROL) — CHILD

Israel — Child (approx. 1-12 years)

Protocol ID: IL-CHILD-SEVERE-BLEEDING-001

CALL 101 (MDA) — CALL IMMEDIATELY

When to apply: Severe or uncontrolled external bleeding in a child (approx. 1–12 years). Signs: rapid spurting or steady flow of blood, pooling blood, blood-soaked clothing, or a severed/partially severed limb. Children have significantly smaller blood volumes (a 3-year-old has only 1,200 mL total) – what appears moderate in an adult may be life-threatening in a child.

Step 1: Ensure scene safety and wear disposable gloves if available.

Do not approach until the scene is safe. If a parent or caregiver is present, involve them immediately -- they can help keep the child calm and assist with treatment.

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Step 2: Call 101 (Magen David Adom) immediately.

Provide location, number of casualties, nature of injuries. Emphasise the patient is a child -- give approximate age and weight if known. If alone and bleeding is immediately life-threatening, begin hemorrhage control first and call as soon as possible.

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Step 3: Calm the child using age-appropriate communication.

Speak softly using simple language. Involve the parent/caregiver. Explain what you are doing simply. Acknowledge fear and pain. Shield the child from the sight of blood if possible.

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Step 4: Check for embedded or impaled objects in the wound.

Is there an embedded or impaled object in the wound?

YES -> Do NOT remove the object. Stabilise it with padding around it. Apply pressure around the object, not on it. Gently restrain child's hands if they try to pull at it. Skip to Step 10 (monitor for shock).

NO -> Proceed to Step 5 (direct pressure).

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Step 5: Apply strong, direct pressure to the source of bleeding.

Press firmly directly on the wound with gloved hand or sterile gauze. Maintain continuous firm pressure without lifting to check -- this allows clot formation. On a small child, one adult hand may cover the entire wound.

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Step 6: If dressing soaks through, add layers on top -- do NOT remove the original dressing.

Has direct pressure controlled the bleeding?

YES -> Maintain pressure. Elevate the limb above heart level if possible (Step 7). Go to Step 10 (monitor for shock).

NO -> Proceed to Step 8 (Israeli bandage / pressure dressing).

SEVERE BLEEDING (HEMORRHAGE CONTROL) — CHILD

CALL 101 (MDA) — CALL IMMEDIATELY

Step 7: If possible, elevate the injured limb above heart level.

Elevation is an adjunct to direct pressure, not a replacement. It reduces blood flow to the injured area.

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Step 8: Apply an Israeli Emergency Bandage (pressure dressing).

1) Apply white sterile pad directly over the wound. 2) Wrap elastic bandage around the area, passing through the pressure bar. 3) Reverse direction and continue wrapping for circumferential pressure. 4) Secure using the built-in closure bar. On smaller children, the elastic wrap may need to be folded or doubled.

WARNING: The elastic component can more easily occlude arteries in smaller limbs. Verify distal pulse after application -- check for warmth, colour, and pulse below the bandage. If pulse is absent, loosen slightly and re-secure.

Has the Israeli bandage controlled the bleeding?

YES -> Maintain dressing in place. Go to Step 10 (monitor for shock).

NO -> Assess wound location. Limb: go to Step 9b (tourniquet). Junctional area (groin, neck, axilla, shoulder): go to Step 9a (wound packing).

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Step 9: Escalate hemorrhage control based on wound location.

Is the wound on a limb (arm or leg)?

YES -> Assess child's age and limb size, then apply a tourniquet (Step 9b / Step 11).

NO -> Apply wound packing for junctional wounds (Step 9a / Step 10a).

Step 9a: Wound packing for junctional areas (groin, neck, axilla, shoulder).

1) Insert hemostatic or plain gauze finger-by-finger into the wound. 2) Fill internally in a sunburst pattern to create internal pressure against the bleeding vessel. 3) Apply the attached dressing with direct pressure for 1-2 minutes. 4) Tighten bandage straps to maintain pressure. Use smaller amounts of gauze proportional to the child's wound size.

WARNING: Do NOT use wound packing for penetrating stab wounds. Do NOT enlarge wound openings to facilitate packing.

Step 9b: Apply a tourniquet for uncontrolled limb bleeding.

1) Place the tourniquet 5-10 cm above the source of bleeding (proximal, toward the heart). 2) Tighten the windlass until bleeding completely stops. 3) Secure the windlass in place. 4) Record the time of application in 4-digit format (e.g., 'TQ 1430').

WARNING: A tourniquet will cause significant pain. A frightened child in pain will likely scream and struggle -- this is expected. Do NOT loosen the tourniquet because the child is in distress. Enlist a caregiver to comfort the child. Do NOT remove or loosen unless instructed by a paramedic or physician.

Is the child aged 2 or older with a limb circumference of at least 13 cm?

YES -> Apply a CAT Gen 7 tourniquet. Ages 6+ with limb circumference 16 cm+: both upper and lower extremity application is reliable. Ages 2-5: upper extremity has 100% success rate; lower extremity 94.5%.

NO -> For infants/very small children under 2: no evidence for manufactured tourniquets. Use sustained direct pressure as primary method. Consider improvised narrow pressure devices only under medical direction.

SEVERE BLEEDING (HEMORRHAGE CONTROL) — CHILD

CALL 101 (MDA) — CALL IMMEDIATELY

Step 10: Monitor the child for signs of hypovolemic shock.

Children enter hypovolemic shock faster than adults. Tachycardia (rapid heart rate) is the earliest and most reliable sign. Also watch for: pale, cool, clammy, or mottled skin; bluish discolouration around lips and fingernails; fast shallow breathing; weakness, lethargy, or confusion. Hypotension is a LATE sign in children -- do not wait for it.

WARNING: Children compensate longer then decompensate rapidly. Do NOT wait for low blood pressure to suspect shock.

Is the child showing signs of shock? (tachycardia, pale/cool/mottled skin, confusion, rapid breathing)

YES -> Keep the child lying down. If conscious, elevate legs above the head. If unconscious, place in recovery position. Cover with a blanket even in warm weather. Moisten lips with water but do NOT give food or drink. Urgently update the 101 dispatcher on deterioration.

NO -> Continue monitoring closely. Maintain hemorrhage control. Keep the child warm, calm, and reassured until MDA arrives. Reassess frequently – children can decompensate suddenly.

DO NOT:

X Do not remove an embedded or impaled object from the wound.
X Do not lift or remove a blood-soaked dressing to check the wound -- add layers on top.
X Do not remove or loosen a tourniquet once applied unless instructed by a paramedic or physician.
X Do not loosen a tourniquet because the child is screaming or in distress -- pain is expected.
X Do not give food or drink to a child with severe bleeding or suspected shock.
X Do not use wound packing for penetrating stab wounds.
X Do not enlarge wound openings to facilitate wound packing.
X Do not assume a rapid pulse (tachycardia) is caused only by anxiety, fear, or pain -- suspect significant blood loss.
X Do not wait for hypotension to suspect shock in a child -- hypotension is a late, pre-arrest sign.
X Do not delay calling 101 for severe or uncontrolled bleeding in a child.
X Do not apply a tourniquet directly over a joint -- place it 5-10 cm above the wound on the limb.
X Do not attempt to use a manufactured tourniquet on a child under 2 years old -- use sustained direct pressure.
X Do not lie to the child about pain -- if something will hurt, say so honestly in age-appropriate language.

Equipment Needed:

- Disposable gloves
- Sterile gauze or clean cloth
- Israeli Emergency Bandage (pressure dressing)
- Tourniquet (CAT Gen 7 – evidence-supported for children aged 2+ with limb circumference 13 cm+)
- Wound packing material (hemostatic gauze preferred)
- Blanket or coat (for shock management and warmth)
- Comfort item for the child (toy, stuffed animal) if available

SHOCK (HELEM) — FIRST AID (CHILD)

Israel — Child

ID: IL-CHILD-SHOCK-001

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א) **Police (100 :משטרה)** **Fire (102 :כיבוי)** **Hatzalah (1221 :הצלה)**

Call 101 immediately when you see:

Tachycardia (earliest sign), pale or cool skin, prolonged capillary refill (>2 seconds), cold clammy extremities, anxiety or irritability, rapid shallow breathing, mottled skin, altered mental status, or hypotension (a late sign). Common pediatric causes: hemorrhage (trauma), dehydration (vomiting/diarrhea), burns, anaphylaxis, severe infection (sepsis). Children have much lower total blood volumes than adults – small fluid losses can be life-threatening.

Summary: Recognise and manage shock in a child by calling 101 immediately, controlling bleeding, positioning appropriately, maintaining warmth, and monitoring for sudden deterioration.

Step 1: Call 101 (Magen David Adom) immediately.

Provide your location, the child's approximate age, and describe the signs you are observing (rapid pulse, pale skin, altered behaviour). Shock in children is a time-critical emergency -- early hospital treatment with IV fluids and medications is essential.

WARNING: Do not delay calling 101. Children can appear deceptively stable during compensated shock and then deteriorate suddenly.

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Step 2: Assess and manage life threats -- check airway, breathing, and circulation (ABC).

Is the child breathing?

YES -> Continue to Step 3.

NO -> Begin CPR immediately (see IL-CHILD-CPR protocol). Continue CPR until EMS arrives or the child resumes breathing.

Step 3: Control any visible bleeding.

Apply direct pressure to wounds with a sterile bandage, clean cloth, or your gloved hand. Hemorrhage is the most common cause of hypovolemic shock in pediatric trauma. In children, even small volumes of blood loss can be life-threatening due to their low total blood volume.

Is there visible external bleeding?

YES -> Apply firm, direct pressure to the wound. Use sterile gauze or clean cloth. Do not remove blood-soaked dressings – add layers on top. See IL-CHILD-SEVERE-BLEEDING protocol for escalation if bleeding is uncontrolled.

NO -> Proceed to Step 4. Note: shock may result from internal bleeding, dehydration, anaphylaxis, or infection – the absence of visible bleeding does not exclude shock.

Step 4: Assess the child's level of consciousness and determine positioning.

Is the child conscious?

YES -> Lay the child flat on their back. Gently elevate the legs above the level of the head (e.g., rest legs on a chair, pillow, or folded blanket) to increase blood flow to the brain and vital organs. Proceed to Step 6.

NO -> Proceed to Step 5 to assess for spinal injury before positioning.

Step 5: Position an unconscious child appropriately.

Is spinal injury suspected (fall from height, car accident, diving injury)?

YES -> Do NOT move the child. Stabilise the head and neck in the position found. Maintain an open airway using a jaw-thrust manoeuvre without tilting the head. Wait for EMS.

NO -> Place the child in the recovery position (lateral/side-lying position) to protect the airway from vomit or secretions. Continue to Step 6.

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Step 6: Keep the child warm -- cover with a blanket, coat, or clothing.

When perfusion is inadequate, the body loses heat rapidly. Cover the child and insulate from cold ground as well (place a blanket or clothing beneath them). Even in warm weather, a child in shock can become hypothermic due to poor perfusion.

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V

Step 7: Remove or loosen any restrictive clothing.

Loosen belts, tight waistbands, collars, or other clothing that may impair breathing or circulation.

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V

Step 8: Calm and reassure the child.

Anxiety and distress worsen the physiological response to shock by increasing heart rate and oxygen demand. Speak gently and stay with the child. If a parent or caregiver is present, involve them -- position them where the child can see and hear them.

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V

Step 9: Moisten the child's lips with water if they are thirsty.

Use a damp cloth or a small amount of water on the lips only.

WARNING: Do NOT give the child anything to eat or drink. The child may need surgery or anaesthesia, and a full stomach increases the risk of aspiration.

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Step 10: Monitor the child continuously until EMS arrives.

Track breathing rate, pulse (check for tachycardia or bradycardia), skin colour (pale, mottled, or cyanotic), capillary refill time, and level of consciousness. Reassess frequently -- children can transition from compensated to decompensated shock suddenly and without warning.

WARNING: Be prepared for sudden deterioration. Tachycardia is the earliest sign of shock. Bradycardia (abnormally slow heart rate) is a pre-arrest sign in children -- if the child develops bradycardia, be ready to begin CPR.

Is the child's condition worsening (increasing heart rate, falling consciousness, mottled skin, weak pulses)?

YES -> Urgently update the 101 dispatcher on the child's deterioration. Ensure legs remain elevated (if conscious) or airway is protected (if unconscious). Re-check bleeding control. Prepare for CPR if cardiac arrest occurs.

NO -> Continue monitoring and maintaining warmth, positioning, and reassurance until MDA arrives.

DO NOT:

X Do not sit the child upright -- this reduces blood flow to the brain and worsens shock.
X Do not give the child anything to eat or drink -- moisten lips only.
X Do not leave the child unattended -- continuous monitoring is essential due to risk of sudden deterioration.
X Do not delay calling 101 -- early hospital treatment with IV fluids and medications is critical for survival.
X Do not assume a child with normal blood pressure is not in shock -- children maintain blood pressure through strong compensatory mechanisms until they suddenly decompensate.
X Do not attribute tachycardia solely to anxiety, fear, or pain -- suspect shock until proven otherwise.
X Do not move a child with suspected spinal injury -- stabilise and wait for EMS.
X Do not remove blood-soaked dressings from wounds -- add additional layers on top.

Equipment Needed:

- Blanket or coat (for warmth and insulation from ground)
- Sterile gauze or clean cloth (for bleeding control)
- Disposable gloves
- Pillow or folded blanket (for leg elevation)

SMOKE / TEAR GAS INHALATION — CHILD

Israel — Child

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :(מד"א	**Police (100 :(משטרה**	**Fire (102 :(כיבוי**	**Hatzalah (1221 :(הצלה**

When to call: Call MDA at 101 immediately for any child with smoke or toxic inhalation exposure. Also call Fire Department at 102 if the fire is ongoing. Children should be assessed even with light smoke exposure due to their vulnerability. Alternatively, call United Hatzalah at 1221.

Step 1: Ensure your own safety first.

Use protective equipment if available. Do NOT re-enter a burning building or hazardous area to rescue the child. Maintain distance from the exposure source. If the area is filled with smoke or gas, wait for Fire Department (102) rescue teams with breathing apparatus.

WARNING: Do NOT enter a hazardous environment without proper protective equipment. More rescuers die from inhalation than from any other fire-related cause.

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Step 2: Remove the child from the exposure source to fresh air immediately.

Move the child as far from the smoke, gas, or fumes as possible. Move to an open, well-ventilated area. If indoors, move outdoors. Remove any contaminated clothing. If tear gas exposure, move upwind from the gas source.

WARNING: Heavier gases (including some combustion byproducts) concentrate at ground level where children breathe. Move the child to higher ground or a ventilated area.

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Step 3: Call 101 (MDA) and 102 (Fire Department).

Report the type of exposure (smoke, tear gas, chemical fumes), the child's age and condition, and whether the exposure was in an enclosed space. Enclosed space exposure is more dangerous due to higher gas concentrations.

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Step 4: Position the child upright to ease breathing.

Seat the child in a comfortable upright position. This position maximises lung expansion and eases breathing effort. If the child is unconscious but breathing, place in the recovery position.

Is the child conscious?

YES: Seat the child upright. Calm and reassure them. Proceed to Step 5.

NO: If breathing: place in recovery position (see recovery position protocol). If not breathing: begin CPR immediately.

Step 5: Administer high-concentration oxygen if available.

If supplemental oxygen is available, administer at maximum concentration via face mask. This is critical for carbon monoxide poisoning (CO displaces oxygen from haemoglobin) and cyanide exposure. High-flow oxygen is the primary field treatment for inhalation injuries.

Is supplemental oxygen available?

YES: Administer high-concentration oxygen via face mask. Continue until handover to emergency services.

NO: Ensure the child is in the freshest air possible. Keep them calm to reduce oxygen demand. Monitor breathing closely.

SMOKE / TEAR GAS INHALATION — CHILD (continued)

Stridor, Poisoning Signs, Tear Gas & Monitoring

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א) **Police (100 :משטרה)** **Fire (102 :כיבוי)** **Hatzalah (1221 :הצלה)**

Step 6: Assess for stridor and airway compromise.

Listen for stridor (high-pitched noisy breathing) which indicates upper airway swelling. In children, stridor is common between ages 3 months to 5 years and indicates significant airway compromise. Stridor after smoke exposure is urgent and indicates potential thermal airway injury. Croup-like symptoms (barking cough, stridor) after smoke exposure require urgent attention.

WARNING: Stridor in a child after inhalation injury is a danger sign -- the airway may swell further and close. This child needs advanced airway management from emergency services.

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Does the child have stridor or severe breathing difficulty?

YES: This is urgent. Keep the child calm and upright. Do not examine the throat. Ensure 101 has been called. Cool mist humidifier if available. Be prepared for CPR if airway closes.

NO: Continue monitoring. Watch for delayed onset of respiratory distress which can develop hours after exposure.

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Step 7: Monitor for signs of specific poisoning.

Carbon monoxide (CO): pink/reddish lips, headache, confusion, dizziness -- from incomplete combustion in enclosed spaces. Cyanide: altered consciousness, seizures -- from burning plastics, foam, paints. Both are colourless. Note any chemical containers or materials that were burning to report to emergency services.

WARNING: Symptoms of inhalation injury can be delayed by hours. A child who appears well immediately after exposure may deteriorate later. All exposed children should be evaluated at hospital.

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Step 8: For tear gas exposure: irrigate eyes and exposed skin.

If tear gas (CS/OC) exposure: irrigate the child's eyes with clean flowing water for 15 minutes. Wash exposed skin with soap and water. Remove contaminated clothing. Move upwind from the gas source. Fresh air is the primary treatment.

WARNING: Do NOT rub the eyes after tear gas exposure -- this worsens irritation. In enclosed spaces, tear gas concentration can become dangerous rather than merely irritating.

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Step 9: Prevent hypothermia and monitor continuously until emergency services arrive.

Cover the child with a blanket -- children lose heat rapidly. Monitor breathing continuously. Be prepared to perform CPR if breathing stops. Note the duration of exposure and any substances involved for medical staff. Ensure adequate hydration if the child is alert and able to swallow.

DO NOT:

X Re-enter a burning building or hazardous area -- wait for Fire Department rescue teams.
X Ignore light smoke exposure in a child -- children should be assessed even with mild exposure.
X Assume a well-appearing child is uninjured -- respiratory deterioration can be delayed by hours.
X Rub the eyes after tear gas exposure -- this worsens irritation.
X Give food or drink to a child with stridor or significant breathing difficulty -- aspiration risk.
X Examine the throat of a child with stridor -- this can worsen airway swelling.
X Leave the child unattended -- monitor breathing continuously.
X Position a child with breathing difficulty lying flat -- keep upright.

Equipment needed:

— Supplemental oxygen and face mask (if available -- typically first responder equipment)
— Clean water (for eye irrigation after tear gas)
— Blanket (to prevent hypothermia)
— Cool mist humidifier (if available, for stridor management)
— Phone (to call 101 and 102)

SNAKEBITE FIRST AID (CHILD)

Israel — Child

Primary venomous species: Palestine viper (Daboia palaestinae)

CALL 101 (MDA) — CALL IMMEDIATELY FOR EVERY SNAKEBITE

MDA (101 :מד"א) **Police (100 :משטרה)** **Fire (102 :כיבוי)** **Hatzalah (1221 :הצלה)**

When to apply: When a child has been bitten or suspected bitten by a snake. Treat every snakebite as venomous until proven otherwise – do not wait for symptoms to appear. Common bite locations in Israeli children: 63% lower limbs, 33% upper limbs, 4% head/neck.

Children are at higher risk due to lower body weight and faster symptom progression.

Step 1: Ensure scene safety. Move the child away from the snake.

Maintain at least 2 metres distance from the snake. Do not attempt to catch, kill, or handle the snake. If safe to do so from a distance, photograph the snake for hospital identification.

WARNING: Do NOT approach, chase, or attempt to catch the snake. This risks additional bites.

Step 2: Call 101 (MDA) immediately.

Request an ambulance. Report the child's age, approximate weight, bite location, time of bite, and any symptoms. Treat every snakebite as venomous until proven otherwise.

WARNING: Do NOT wait for symptoms before calling. Children can deteriorate rapidly due to lower body weight.

Step 3: Keep the child calm and still. Have the child lie down.

Reassure the child by speaking calmly and confidently. Minimise all movement. Snake venom travels through the lymphatic system, which is activated by body movement, not heartbeat. Children may be frightened and want to run -- gently but firmly prevent this.

WARNING: Immobility is critical. Any movement accelerates venom distribution through the lymphatic system.

Step 4: Position the bitten limb below heart level and immobilise it.

Keep the bitten limb lower than the heart to slow venom distribution. Immobilise the limb against the body as you would for a fracture.

Is a splint or rigid object available to immobilise the limb?

YES -> Apply the splint to immobilise the bitten limb as for a fracture, keeping it below heart level.

NO -> Immobilise the limb by positioning it against the child's body. Use clothing or fabric to secure it in place if possible. Keep it below heart level.

Step 5: Remove all constrictive items from the affected limb.

Remove rings, bracelets, watches, anklets, and tight clothing from the bitten limb. Swelling can develop rapidly and constrictive items can cause secondary injury by cutting off circulation.

Step 6: Clean the bite wound gently if materials are available.

Clean the bite site with water and soap only. Do not apply antiseptic, alcohol, or any other substance to the wound.

WARNING: Do NOT apply alcohol, antiseptic, or any other substance to the bite site.

Are water and soap available?

YES -> Gently clean the bite site with water and soap only.

NO -> Leave the wound as is. Do not apply any substances. Cleaning can be done at hospital.

Step 7: Monitor the child for signs of systemic envenomation.

Watch for: swelling spreading beyond the bite site, rapid heartbeat (tachycardia), vomiting, nausea, abdominal pain, difficulty breathing or swallowing, altered consciousness, pallor, hypotension (dizziness, fainting), rash or itching. In children, systemic signs may develop more rapidly than in adults.

Has the child lost consciousness and stopped breathing?

YES -> Begin CPR immediately. Continue until emergency services arrive.

NO -> Continue monitoring. Keep the child calm, still, and lying down with the bitten limb below heart level. Wait for emergency services.

Step 8: Prepare for transport to hospital.

All snakebite victims require urgent medical evaluation, even if no symptoms are present. Antivenom (monovalent equine anti-V. palaestinae) is stocked at Israeli hospitals. If the snake was photographed, have the photo ready to show medical staff.

DO NOT:

X Do NOT apply a tourniquet -- concentrates venom and causes tissue damage.
X Do NOT cut the bite site -- causes additional injury and does not remove venom.
X Do NOT attempt to suck out the venom -- ineffective and risks infection.
X Do NOT apply ice or cold compresses -- can cause tissue damage, does not slow venom absorption.
X Do NOT apply alcohol, antiseptic, or any substance to the wound.
X Do NOT give the child food or drink -- risk of vomiting, child may need anaesthesia at hospital.
X Do NOT give the child hot liquids or alcohol.
X Do NOT induce vomiting.
X Do NOT chase, catch, kill, or handle the snake -- risk of additional bites.
X Do NOT use a pressure immobilisation bandage -- not recommended for viper bites in the Israeli protocol.
X Do NOT allow the child to walk or move -- movement accelerates venom spread.
X Do NOT wait for symptoms before seeking emergency care -- treat every snakebite as venomous.

Equipment needed:

— Splint or rigid object for limb immobilisation
— Water and soap (for wound cleaning)
— Phone (to call 101 and photograph the snake)
— Blanket or clothing (to keep child warm and comfortable while lying still)

Source: Magen David Adom (MDA) (2026-01-01) · Last verified: 2026-03-15
Protocol ID: IL-CHILD-SNAKEBITE-001
Personal reference only — not medical advice. Always call 101 in an emergency.

SPIDER BITES — CHILD

Israel — Child (including Infants)

Brown Recluse & Black Widow

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א) **Police (100 :משטרה)** **Fire (102 :כיבוי)** **Hatzalah (1221 :הצלה)**

When to call: Call MDA at 101 if the child develops systemic symptoms: severe pain, abdominal cramps, difficulty breathing, chest tightness, excessive sweating, altered consciousness, or if a black widow bite is suspected. Call for any spider bite in an infant or toddler. Alternatively, call United Hatzalah at 1221.

Step 1: Move the child away from the spider. If safely possible, capture or photograph the spider for identification.

Do not attempt to handle the spider directly. If the spider can be safely trapped under a glass or container, do so for hospital identification. A photograph from a safe distance is also helpful. Knowing the species guides hospital treatment decisions (antivenom for black widow).

WARNING: Do NOT attempt to catch the spider if there is any risk of additional bites.

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Step 2: Wash the bite area with water and soap.

Clean the bite site gently with water and soap to reduce infection risk. Do not apply antiseptic, alcohol, or any other substance.

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Step 3: Apply a cold compress to reduce pain and swelling.

Apply ice or a cold compress wrapped in cloth to the bite area for 10-15 minutes. This helps reduce pain and local swelling. Remove for 10 minutes, then reapply.

WARNING: Wrap ice in cloth -- do not apply ice directly to skin, especially on a child's sensitive skin.

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Step 4: Keep the bitten limb immobilised and at heart level.

Immobilise the bitten limb to slow venom distribution. Keep the limb at approximately heart level. Have the child lie down or sit still.

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Step 5: Monitor for systemic symptoms.

Watch closely for: severe or spreading pain, redness spreading along lymph pathways, abdominal cramps, difficulty breathing or swallowing, excessive sweating, muscle cramps (spreading to abdomen, back, chest), nausea/vomiting, elevated blood pressure, altered consciousness. In children, systemic symptoms develop faster due to lower body weight. Infants and toddlers cannot describe symptoms -- watch for irritability, unusual crying, poor feeding, and changes in behaviour.

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Is the child developing systemic symptoms (severe pain, abdominal cramps, breathing difficulty, sweating, muscle cramps)?

YES: Call 101 immediately if not already done. This may be a black widow bite requiring antivenom. Keep the child calm and still. Prepare for transport to hospital.

NO: Continue monitoring closely. Seek medical evaluation for all spider bites in children, even if no systemic symptoms are present.

SPIDER BITES — CHILD (continued)

Species Identification & Hospital Transport

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א) **Police (100 :משטרה)** **Fire (102 :כיבוי)** **Hatzalah (1221 :הצלה)**

Step 6: Identify the spider species if possible.

Brown Recluse (shshan chum): small, brown, long legs, lives indoors. Bite barely felt initially, effects appear hours later. Causes local tissue damage and potentially necrotic ulcer. Black Widow (almana shchora): black with markings, found outdoors in gardens and stone walls. Bite causes immediate pain, burning, and rapid systemic symptoms. Female bite is the dangerous one.

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Is the spider identified as or suspected to be a Black Widow?

YES: Call 101 immediately. Black widow antivenom is available at Israeli hospitals. Transport urgently. Black widow venom effects can be more severe in children, including respiratory distress.

NO: Seek medical evaluation. Brown recluse bites require monitoring for delayed necrotic wound development. Hospital observation may be recommended for young children even with mild bites.

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Step 7: Transport to hospital for medical evaluation.

All spider bites in children should receive medical evaluation due to the greater venom-to-body-weight ratio. Bring the captured/photographed spider if available. Report the time of bite, symptoms observed, and any changes in the child's condition. Hospital may check tetanus vaccination status. The first 6-8 hours are critical for determining recovery and preventing permanent tissue damage.

DO NOT:

X Apply a tourniquet -- tourniquets worsen envenomation outcomes by concentrating venom in the limb.
X Cut the bite site -- causes additional injury and does not remove venom.
X Attempt to suck out the venom -- ineffective and risks infection.
X Apply antiseptic, alcohol, or any substance other than water and soap to the bite.
X Bandage tightly over the bite site.
X Delay medical evaluation for any spider bite in a child -- children are more vulnerable to venom effects.
X Ignore a painless bite -- brown recluse bites are often painless initially but can develop serious necrotic wounds hours later.
X Give the child medications (painkillers, antihistamines) without medical direction.

Equipment needed:

— Water and soap (for wound cleaning)
— Ice or cold compress wrapped in cloth
— Container or glass (to safely capture the spider for identification)
— Phone (to call 101 and photograph the spider)

STROKE IN CHILDREN

Cerebrovascular Event — Pediatric

Israel — Child

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א) **Police (100 :משטרה)** **Fire (102 :כיבוי)** **Hatzalah (1221 :הצלה)**

SUSPECT STROKE IF THE CHILD SHOWS ANY OF:

- Facial drooping or asymmetry
- Arm weakness or drift
- Slurred or incomprehensible speech
- Sudden severe headache with vomiting
- New-onset seizure (especially one-sided)
- Sudden altered mental status or confusion
- Sudden loss of balance or coordination
- Sudden vision loss or sudden behavioural changes
- In very young children: irritability, feeding difficulty, or lethargy

Step 1: Apply the FAST assessment to check for stroke signs.

F (Face): Ask the child to smile — check for facial drooping or asymmetry. A (Arms): Ask the child to raise both arms — check if one drifts downward or cannot be raised. S (Speech): Ask the child to repeat a simple phrase — check for slurred, confused, or incomprehensible speech. T (Time): Note the exact time symptoms began.

WARNING: In children, stroke may present differently from adults. Be alert for seizures at onset, sudden severe headache with vomiting, gradual onset of symptoms, altered mental status, ataxia, or subtle signs in very young children (irritability, feeding difficulty, lethargy). FAST is not validated for the pediatric population and may miss atypical presentations.

Is any FAST sign present, or are there other neurological signs suggestive of stroke?

YES -> Call 101 immediately and continue with the remaining steps.

NO -> Continue to monitor the child. If symptoms develop or you remain concerned, call 101.

Step 2: Call 101 (MDA) immediately if any stroke signs are present.

State clearly that you suspect a child is having a stroke. Describe the symptoms observed. MDA dispatchers will guide you while sending an ambulance to the appropriate stroke centre.

Step 3: Note the exact time symptoms began.

This is critical for the medical team. Clot-dissolving medication (alteplase) is only effective within approximately 4.5 hours for children aged 28 days to 18 years. Mechanical clot removal may be effective for children aged 6 and older within 6--24 hours in certain cases. If you did not witness onset, note the last time the child was seen acting normally.

Step 4: Keep the child calm and still. Position the child appropriately.

Lay the child down in a comfortable position. Loosen any tight clothing around the neck and chest.

Is the child conscious?

YES -> Lay the child down with head and shoulders slightly elevated. Keep them calm and still.

NO -> Proceed to Step 5 to assess breathing and position accordingly.

Step 5: If the child is unconscious, assess breathing and act accordingly.

Is the unconscious child breathing?

YES -> Place the child in the recovery position (on their side) and monitor continuously.

NO -> Begin CPR immediately and follow MDA dispatcher instructions.

Step 6: If the child has a seizure, manage it safely.

Do not restrain the child. Do not put anything in the child's mouth. Protect the child from injury by clearing the surrounding area. After the seizure stops, place the child on their side. Note the duration and type of seizure for medical personnel.

Is the child having a seizure?

YES -> Clear the area around the child to prevent injury. Do not restrain or insert anything into the mouth. After the seizure stops, place on their side. Note duration and type.

NO -> Continue monitoring the child for changes in consciousness, breathing, and symptoms.

Step 7: Monitor the child continuously until emergency services arrive.

Watch for changes in consciousness, breathing, and symptoms. Be prepared to report any progression to MDA paramedics on arrival. Be ready to begin CPR if the child becomes unresponsive and stops breathing.

DO NOT:

X Delay calling 101 to wait and see if symptoms resolve.
X Give the child any food or drink -- stroke may impair the swallowing reflex, creating a risk of aspiration and choking.
X Give any medications, especially not aspirin -- if the stroke is haemorrhagic (bleeding), aspirin will worsen it.
X Attempt to drive the child to hospital yourself unless MDA explicitly advises it. MDA ambulances can begin treatment en route and will transport to the appropriate stroke centre.
X Assume a child cannot have a stroke -- children can have strokes at any age.
X Restrain the child during a seizure.
X Put anything in the child's mouth during a seizure.
X Rely solely on FAST to rule out stroke in children -- adult stroke screening tools are not validated for the pediatric population and may miss atypical presentations.

Source: Magen David Adom (MDA) (2024-11-18) · Last verified: 2026-03-15
Protocol ID: IL-CHILD-STROKE-001
Personal reference only — not medical advice. Always call 101 in an emergency.

TOURNIQUET APPLICATION — CHILD

Israel — Child (including Infants)

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :(מד"א	**Police (100 :(משטרה**	**Fire (102 :(כיבוי**	**Hatzalah (1221 :(הצלה**

When to call: Call MDA at 101 immediately for any child with life-threatening bleeding. Apply the tourniquet while waiting for emergency services – do not delay haemorrhage control to make the call. Alternatively, call United Hatzalah at 1221.

Step 1: Identify life-threatening limb haemorrhage requiring a tourniquet.

A tourniquet is needed when bleeding from a limb is massive and uncontrollable with direct pressure, when the wound involves an amputation or near-amputation, or when direct pressure is impractical (multiple casualties, other injuries requiring attention).

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Is the bleeding from a limb and cannot be controlled with direct pressure?

YES: Apply a tourniquet immediately. Proceed to Step 2.

NO: Use direct pressure with sterile gauze or clean cloth to control bleeding. If bleeding is from the torso, neck, or junctional area, pack the wound and apply maximum direct pressure.

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Step 2: Select the appropriate tourniquet for the child's limb size.

CAT (Combat Application Tourniquet) Gen 7: effective for most children aged approximately 2 years and above, with limb circumference of approximately 13-16 cm or greater. SWAT-T (elastic wrap) or RATS (ratcheting) tourniquet: effective across all pediatric sizes including very small limbs. For infants or very small toddlers with limbs under 13 cm circumference: use direct pressure and wound packing as primary method, or SWAT-T/RATS if available.

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Is the child's limb large enough for a CAT tourniquet (approx. 13 cm circumference or greater, typically age 2+)?

YES: Use the CAT tourniquet. Proceed to Step 3.

NO: Use a SWAT-T or RATS tourniquet if available. If no pediatric tourniquet is available, apply maximum direct pressure with wound packing, or improvise a tourniquet using a triangular bandage and windlass.

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Step 3: Apply the tourniquet 5-7 cm (2-3 inches) proximal to the wound.

Place the tourniquet on the limb between the wound and the heart, approximately 5-7 cm (2-3 inches) above the wound. Apply it over a single bone (upper arm or upper leg) when possible, not over a joint. Do not apply directly over the wound.

WARNING: Do NOT apply the tourniquet over a joint (elbow, knee). Place it on the upper arm or upper thigh for best effectiveness.

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Step 4: Tighten the tourniquet until bleeding stops.

For CAT tourniquet: pull the strap tight, then turn the windlass rod until bleeding stops. Secure the windlass in the clip. For SWAT-T: stretch and wrap the elastic band tightly around the limb, overlapping each layer, until bleeding stops. Check that distal bleeding has stopped completely.

WARNING: Tourniquet application is extremely painful. Warn the child and parent. Pain is expected and necessary -- an insufficiently tight tourniquet is worse than no tourniquet (venous bleeding without arterial occlusion).

TOURNIQUET — CHILD (continued)

Recording, Monitoring & Transport

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA (101 :מד"א) **Police (100 :משטרה)** **Fire (102 :כיבוי)** **Hatzalah (1221 :הצלה)**

Step 5: Record the time of tourniquet application.

Write the time of application clearly on the tourniquet, on the child's skin near the tourniquet, or on a piece of tape attached to the child. Use 24-hour format. This information is critical for hospital staff.

WARNING: Recording the time is essential. Prolonged tourniquet application can cause tissue damage, and hospital staff need this information to plan care.

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Step 6: Do NOT remove the tourniquet once applied.

Only hospital personnel should remove a tourniquet. Premature removal can cause recurrence of life-threatening bleeding and release of accumulated toxins into the bloodstream.

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Step 7: Re-assess and monitor the child frequently.

Check that bleeding has stopped. Check the tourniquet has not loosened. Monitor the child for signs of shock (rapid weak pulse, pallor, cool mottled skin, altered consciousness). Children decompensate rapidly from blood loss. Keep the child warm to prevent hypothermia.

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Has the bleeding stopped after tourniquet application?

YES: Maintain the tourniquet in place. Monitor for shock. Transport urgently.

NO: Apply a second tourniquet proximal (closer to the body) to the first one. Tighten both. If bleeding still continues, apply maximum direct pressure in addition to the tourniquets.

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Step 8: Transport urgently to hospital.

All tourniquet patients require urgent hospital care. Transport as quickly as possible. Report the time of tourniquet application, the device used, and the child's condition to arriving emergency services or hospital staff.

DO NOT:

X Remove a tourniquet once applied -- only hospital personnel should remove it.
X Apply a tourniquet for venomous animal bites (snakes, scorpions, spiders) -- tourniquets worsen envenomation outcomes.
X Apply a tourniquet over a joint (elbow, knee) -- place on upper arm or upper thigh.
X Apply a tourniquet too loosely -- an insufficiently tight tourniquet causes venous bleeding without arterial occlusion, worsening blood loss.
X Use a standard CAT tourniquet on very small infant limbs (under approx. 13 cm circumference) -- it will not be effective.
X Delay tourniquet application to attempt prolonged direct pressure on massive haemorrhage -- apply the tourniquet immediately.
X Forget to record the time of application.

Equipment needed:

— CAT (Combat Application Tourniquet) Gen 7 -- for children aged approx. 2+ with sufficient limb size
— SWAT-T tourniquet -- for all pediatric sizes including very small limbs
— RATS tourniquet -- alternative for all pediatric sizes
— Improvised tourniquet materials (triangular bandage + rigid windlass object) -- if no manufactured device available
— Marker or pen (to record time of application)
— Sterile gauze (for wound packing if needed)
— Phone (to call 101)